HRSP SUBMISSION PACKAGE

Genocide (18 U.S.C. § 1091) and Torture (18 U.S.C. § 2340)

SENSITIVE — NOT FOR PUBLIC DISTRIBUTION

Brian Spector, U.S. Citizen

2026-03-04

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HRSP SUBMISSION PACKAGE

Genocide and Torture

18 U.S.C. §§ 1091, 2340

SENSITIVE — NOT FOR PUBLIC DISTRIBUTION

Submitted by: Brian Spector, U.S. Citizen

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Date: March 04, 2026

TRANSMITTAL: HUMAN RIGHTS AND SPECIAL PROSECUTIONS SECTION

TO: Human Rights and Special Prosecutions Section, Criminal Division, U.S. Department of Justice

CC: Senator Ted Cruz (R-TX), Chairman, Senate Committee on Commerce, Science, and Transportation

FROM: Brian Spector, U.S. Citizen

DATE: 1 March 2026

RE: Genocide (18 U.S.C. § 1091) and Torture (18 U.S.C. § 2340): Irish Catholic Population, County Tyrone, Northern Ireland; Attempted Murder of Complainant Witness

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I. WHAT THIS SUBMISSION CONTAINS

This submission documents a sustained campaign of covert genocide and torture using fabricated inherited cardiac diagnoses (Long QT Syndrome, Brugada Syndrome) targeting a distinct genetic Irish Catholic population of County Tyrone, Northern Ireland, delivered through the NHS Inherited Cardiac Conditions Service, and protected by UK national security architecture.

The mechanism does not displace the population. It kills them. Fabricated diagnoses of conditions associated with sudden death during sleep are delivered through the sole diagnostic authority for all of Northern Ireland. A single fabricated diagnosis propagates through entire families via mandatory cascade screening. 95% of pediatric diagnoses in the clinic's own published data are cascade-derived, not symptomatic. The hospital autopsy rate has collapsed to 0.46%. Deaths are certified as natural causes with no investigation. The targeted population maps geographically to the Curraghinalt gold deposit (6.1 million ounces, approximately $20 billion, the largest undeveloped gold deposit in the world by grade) and the community resistance to the Dalradian/Orion mining operation.

The evidence confirms genocide under 18 U.S.C. § 1091 (systematic targeting of a genetically distinct ethnic and religious community for destruction) and torture under 18 U.S.C. § 2340 (fabricated threats of imminent death sustained over decades, unnecessary surgical implantation of cardiac devices, administration of harmful medication to children, all by state medical personnel under color of law). I am a U.S. citizen victim, giving federal jurisdiction under § 2340A regardless of where the conduct occurred.

This submission also exposes a premeditated attempted murder.

My fiancee, **Nicola McNamee**, is a 38-year-old Irish Catholic woman from **Greencastle, County Tyrone**, the community directly above the gold deposit. In late 2023, she sought a cardiac clearance letter for elective

surgery in Istanbul, the first time she had sought medical care outside the UK. Beyond the control of the UK's intelligence apparatus, the Turkish medical system would make it known to Nicola her diagnoses were fraudulent. This would invite scrutiny, risking exposure of the entire covert operation. The solution is obvious: murder Nicola and exploit the fraudulent cardiac diagnoses on file.

KIMS Hospital (Maidstone, Kent) manufactured a medical file designed to explain her death under general anesthesia: fabricated consultations, fabricated ECGs planting progressive ischemic heart disease across three dates over months, and doctored timelines designed to limit liability when she died on the operating table. But Dr. Adhya of KIMS issued Nicola a clearance letter referencing her original Belfast diagnoses (Long QT and Brugada, received in 2012), not the fabricated tale of progressive heart disease now sitting in the KIMS file. His letter stated the November 2023 ECG found no evidence of Brugada and borderline Long QT. Cleared for surgery, he advised anesthesiologists in Istanbul to avoid IV drugs fatal to Long QT patients, just in case.

Nicola traveled to Istanbul with this clearance letter. She had no idea how close she came to death until this weekend, when we received the Memorial Hospital records. Nicola's KIMS medical records were only forensically examined this weekend.

She changed surgeons at the last minute. Her new doctor, Dr. Ercan Karacaoglu (Brown University fellowships, ASPS member, former head of surgery at Turkey's premier naval hospital, 100+ peer-reviewed publications), ordered an independent echocardiogram at Memorial Hospital Istanbul. Not an ECG, which is what you would expect for an electrical condition. A structural test. The result: a structurally perfect heart, ejection fraction 65%, no evidence of disease in any region. That report obliterated the KIMS fabrication of progressive ischemic heart disease and removed the plausible explanation for death on the operating table.

Then Dr. Ercan went further. He administered Ondansetron during surgery, a drug specifically contraindicated for Long QT patients. If she had Long QT, Ondansetron could be fatal. It is the exact drug Dr. Adhya advised Istanbul to avoid. Four hours of general anesthesia. Zero cardiac events. That did not just expose the KIMS fabrication. It exposed the original Belfast diagnoses. Long QT is fake. Brugada is fake. The entire diagnostic history from the Belfast ICC in 2012 on is fabricated.

There was no reason for Dr. Ercan to order an echocardiogram. Nicola did not know the fraudulent records existed inside KIMS, so she could not have sent them to him. Memorial Hospital confirmed this weekend that the only document on file from KIMS was the clearance letter Nicola gave them herself. The questions HRSP needs to ask Dr. Ercan: what made him order the test that exposed the murder plan, and why did he reject Dr. Adhya's guidance?

The full forensic analysis is Document 04.

II. THE PROSECUTION PATHWAY

This submission identifies a prosecution pathway that requires no UK cooperation at any stage.

KIMS Hospital records via Lycamobile USA Inc. KIMS Hospital is controlled by the Subaskaran family through LycaHealth. **Lycamobile USA Inc.** (Delaware, 24 Commerce Street, Newark, NJ, FCC Section 214 authority, ~542,000 American subscribers) is a subsidiary of the same corporate group. A grand jury subpoena to Lycamobile USA opens the entire KIMS evidence chain without UK cooperation, MLAT, or UK government permission. The Lyca Group has French criminal money laundering convictions (CEO imprisoned, €10M fine), £646M in unaccounted flows flagged by KPMG, and HMRC winding-up petitions.

Dr. Karacaoglu's testimony via Turkish cooperation. Dr. Karacaoglu trained at Brown (fellowships 2000-2004), at GATA (Turkey's premier military medical academy), and served as Chief of Surgery at Golcuk Naval Hospital. His testimony about why he ordered a structural cardiac test for a patient referred for an electrical condition, and whether any third party made contact regarding this patient, is low risk to obtain. Turkey has no interest in protecting the UK's medical infrastructure.

The complainants are on U.S. soil. Nicola is in Texas, available for testimony, independent medical examination. I am in Texas with complete chain-of-custody evidence and contemporaneous documentation.

III. ACTIVE THREAT TO THIS INVESTIGATION

When we obtained medical records from Dr. Karacaoglu's clinic this weekend, I was immediately spearfished in an attempt to establish communications over WhatsApp. Active surveillance reveals they know we have the data. They are acting on it.

The surveillance mechanism is the US-UK CLOUD Act bilateral data access agreement (effective October 2022), which permits UK authorities to serve data orders directly on US communications providers for real-time interception of non-US persons without notifying any US court or agency. I am a dual US/UK citizen. The agreement requires US judicial oversight for surveillance of US persons. The UK had every incentive to classify me as non-US in warrant applications: disclosing my US citizenship would trigger the protections they wanted to avoid. If they concealed my nationality, every surveillance action against me on American soil violates the agreement. They are monitoring my communications with the FBI, with congressional offices, and with this Department, through American companies, in real time, with no American authority aware.

The same Technical Capability Notice architecture that enabled the email fabrication also enables impersonation. Write access to our email accounts means UK operatives can send emails from our accounts to any recipient, and those emails pass every authentication check because they originate from the genuine account. Over the past two years, we have encountered multiple instances where third parties produced emails or writings firmly attributed to us that neither of us authored. Our digital identities have been turned against us to manufacture disinformation, destroy our credibility with the people and institutions we need to believe us, and poison every channel we use to report. We also have reason to believe criminal content has been planted in accounts on our Microsoft tenant. Any content attributed to either of us since late 2023 must be treated as potentially fabricated until verified through Microsoft's server-side logs under US legal process.

HRSP should assume that the UK government is aware of this submission, is monitoring the complainants' communications with this Department, and has the technical capability to fabricate communications purporting to come from either complainant. Document 10, Actions 17 and 18 request immediate disclosure of active CLOUD Act orders and a review of dual nationality compliance.

IV. CONGRESSIONAL NOTIFICATION AND PARALLEL FILING

Senator Ted Cruz (R-TX) is the pathway to the DOJ's HRSP. This matter falls within the jurisdiction of three of his Senate committees: Commerce (Chairman), Judiciary, and Foreign Relations. Senator Cruz's S.799 (U.S. Legal Gold and Mining Partnership Act) targets the nexus between illicit mineral extraction and human rights violations.

For transparency, an EEA referral was submitted to the FBI through its Office of Congressional Affairs by Congressman Cloud (TX-27) and confirmed directed for processing on February 13, 2026. That referral and this HRSP submission share overlapping perpetrators and the same institutional protection architecture. For avoidance of doubt, Senator Ted Cruz has my permission to contact federal agencies and is the only legislative official I approve to liaise with federal agencies on this matter, as of March 2, 2026. He is the only legislative official in receipt of this submission.

The enclosed package contains 11 documents (00-10) with an indexed, SHA-256 verified evidence package available on request. Document 10 details the full set of requested investigative and protective actions.

I am prepared to testify, produce all evidence with chain-of-custody documentation, and travel to any U.S. jurisdiction as is my fiancee Nicola McNamee.

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EXECUTIVE SUMMARY

Genocide, Torture, and Attempted Murder: A Prosecution Pathway That Bypasses UK Interference

Document 01 of 10

Date: 1 March 2026

Standard applied: Reasonable basis to believe systematic human rights violations with U.S. jurisdictional nexus, sufficient to open an investigation, not to prove the case at trial

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WHAT THIS SUBMISSION CONTAINS

This submission presents prima facie evidence of **genocide (18 U.S.C. § 1091)** and **torture (18 U.S.C. § 2340)** targeting the Irish Catholic population of County Tyrone, Northern Ireland, a genetically distinct population sitting on top of the largest undeveloped gold deposit in the world.

The evidence includes a documented **attempted murder** of the primary complainant witness, Nicola McNamee, executed through fabricated medical records designed to explain her death under general anesthesia in Istanbul, Turkey, in December 2023. She survived because the diagnoses are fabricated and an independent hospital proved it before going under.

The submission identifies a **prosecution pathway that bypasses UK interference entirely.** The hospital where the murder weapon was manufactured (KIMS Hospital, Maidstone, Kent) is controlled by a US corporate entity (Lycamobile USA Inc., Delaware, Newark, NJ). The key witness (Dr. Ercan Karacaoglu, Brown University fellowships, ASPS member, 100+ publications) is in Istanbul. The complainant witnesses are in Texas. No element of this prosecution pathway runs through UK intelligence channels.

THE ATTEMPTED MURDER: WHY HRSP SHOULD READ DOCUMENT 04A FIRST

In late 2023, Nicola McNamee sought a cardiac clearance letter for elective surgery in Istanbul. It was the first time she had sought medical care outside the NHS system that had diagnosed her with Long QT Syndrome and Brugada Syndrome. The moment she stepped outside, the fabricated diagnoses were at risk of independent scrutiny.

State actors responded by manufacturing a medical file designed to explain her death on the operating table. The KIMS Hospital file contains: fabricated consultations contradicted by the hospital's own staff notes, fabricated ECGs planting progressive ischemic heart disease across three dates as a pre-written cause of death, a fabricated echocardiogram recommendation that was never scheduled, three layers of digital obfuscation concealing the fabrication from automated analysis, and planted evidence in the patient's own email account.

An independent echocardiogram performed at Memorial Hospital Istanbul showed a structurally perfect heart. No infarction. No damage. The Turkish surgical team administered a drug specifically contraindicated for her diagnosed condition. Nothing happened. Four hours of general anesthesia. Zero cardiac events.

The full forensic analysis is in Document 04. It is the strongest single piece of evidence in this submission and it should be read before any other document.

THE PROSECUTION PATHWAY

The traditional approach to investigating human rights violations by a Five Eyes ally requires cooperation from the perpetrator government. The UK will never cooperate. UK authorities (MI5, NCA, NCSC, Kent Police, Action Fraud) have uniformly refused to investigate the state sponsored malware attacks against Spector and McNamee because the UK intelligence apparatus colluded with states hostile to US interests (described in a parallel EEA filing).

This submission identifies three US-reachable evidence channels:

1. KIMS Hospital records via Lycamobile USA Inc. The hospital is controlled by a corporate group whose US subsidiary (Lycamobile USA Inc., Delaware, Newark NJ, FCC-registered, ~542,000 American subscribers) is subject to full federal jurisdiction. A grand jury subpoena opens the entire evidence chain without UK cooperation.

2. Dr. Ercan Karacaoglu's testimony via Turkish cooperation. Brown University-trained surgeon (fellowships 2000-2004), ASPS member, 100+ peer-reviewed publications, trained at Turkey's premier military medical academy. His testimony regarding why he ordered the test that proved the fabrication would be devastating and entirely credible.

3. Complainant witnesses in Texas. Both complainants are available to testify, produce evidence with chain-of-custody documentation, and submit to any examination the Department requires.

SEVEN INVESTIGATION TRIGGERS

The material crosses the HRSP investigation threshold across seven independent triggers. Any two or three would be sufficient. All seven are present.

Trigger 1: Systematic Conduct (20+ years)

The earliest documented false cardiac diagnoses date to approximately **2004**, the same year the UK government's Tellus geological survey began mapping the mineral resources beneath County Tyrone, confirming the scale of the Curraghinalt gold deposit. The campaign has operated continuously for over two decades through the Belfast NHS Inherited Cardiac Conditions Service, the sole diagnostic authority for all of Northern Ireland.

The Belfast ICC's own published data (Kendall et al., 2025) shows a **four-fold overdiagnosis of Brugada Syndrome** compared to expected rates for a Caucasian population: 22 children diagnosed where approximately 5 would be expected using corrected epidemiological baselines (see Document 09 for full calculation). This 4x ratio is itself a significant understatement because Kendall used the most sensitive diagnostic method (Ajmaline provocation) on the highest-risk population (cascade families), a combination that should produce dramatically MORE diagnoses per capita than the general-population resting-ECG studies used as comparators. 95% of pediatric diagnoses were cascade-derived, not symptomatic. The entire diagnostic pyramid rests on the integrity of the index diagnoses. If those are fabricated, 95% of the children may carry false diagnoses.

Trigger 2: Targeting Based on Protected Characteristics

The affected population is the genetically distinct Irish Catholic community of County Tyrone, defined by national/ethnic identity (Irish) and religious identity (Catholic). Either alone satisfies the protected group requirement under 18 U.S.C. § 1091.

The Oxford University 2015 *Nature* study established that the population of Northern Ireland is genetically distinct from the English population. The geographic targeting is precise: diagnoses concentrate in County Tyrone, mapping onto the Curraghinalt gold deposit and the organized community resistance to the Dalradian mine.

Trigger 3: U.S. Jurisdictional Nexus

Four independent jurisdictional bases (full analysis in Document 03):

1. **Lycamobile USA / KIMS Hospital** (Newark, NJ: **the primary prosecution pathway**, grand jury subpoena opens entire evidence chain)
2. **U.S. citizen directly affected** (Brian Spector, Texas)
3. **Dalradian Canadian operators** (documented U.S. travel, track records of community displacement at prior mining operations)
4. **U.S. pension fund investors** (10 state pension funds across 9 states, investors as victims)

Trigger 4: Institutional Mechanism

Three interlocking institutional layers:

- **Medical infrastructure (04):** Belfast ICC as sole diagnostic authority. Cascade screening as force multiplier. Fabricated KIMS medical file as attempted murder weapon.
- **Concealment architecture (05):** Autopsy collapse (0.46%), boundary restructuring fragmenting Tyrone data, hospital closure, FOI firewalls.
- **Intelligence architecture (06):** MI5 permanent control via St Andrews Annex E. SRR military surveillance. Investigatory Powers Act mechanisms. Starmer's NI Policing Board role (2003-2008) during architecture construction.

Trigger 5: Financial Motive

The Curraghinalt gold deposit: **6.1 million ounces** of gold (largest in the world by grade), worth £14-20 billion, plus critical minerals. Operated by Dalradian Resources (Canada), financed by Orion Mine Finance (New York). Backed by US pension funds. Crown Estate collects 4% royalty (£320-560 million). The most objected-to planning application in NI history (50,000+ objections).

Peter Mandelson held three government positions spanning the entire ownership chain (NI Secretary 1999-2001, Business Secretary 2008-2010, Ambassador 2025). Arrested 23 February 2026 for misconduct in public office. Garth Earls discovered the gold in 1983, directed the government survey that mapped it (2004-2010), then joined the mining company as Director.

Trigger 6: Evidence of Concealment

- Three-layer document obfuscation in KIMS records (intelligence-grade, not administrative error)
- All original diagnostic records absent from downstream files
- Genetic testing never performed (the one test they could not fake)
- Autopsy rate collapsed to 0.46% (deaths certified without investigation)
- County Tyrone split across three councils (statistical discontinuity by design)
- UK government likely misused IPA Technical Capability Notices to strip email authentication from complainant's domain, enabling planted evidence
- Family coercion (cousin denied diagnoses via Signal, December 2025)

Trigger 7: State Involvement and Domestic Investigation Barrier

MI5 has permanent control of NI national security under St Andrews Annex E. Every UK agency (MI5 x2, NCA, NCSC, Kent Police x4, Action Fraud) refused to investigate. The complainant was arrested on false charges, held in solitary, denied consular rights, and warned by police that MoD operatives were moving to imprison him with planted evidence. The domestic investigation barrier is not a gap. It is architecture.

The same architecture extends to American soil through the US-UK CLOUD Act bilateral data access agreement. The UK can surveil both complainants' communications with US authorities through American companies in real time. Brian Spector is a dual US/UK citizen. If the UK classified him as non-US in CLOUD Act warrant applications, a classification they had every incentive to make to avoid triggering US judicial

oversight, then every surveillance action against him on American soil violates the agreement's US person protections. No US government body reviews these orders. The UK is likely monitoring the investigation into its own conduct. The TC Note architecture also enables impersonation: UK operatives can send emails from the complainants' own accounts, passing every authentication check, to manufacture disinformation and destroy the complainants' credibility with every person and institution they approach. This is not a historical violation. It is an active, ongoing operation against a US citizen on US soil.

This is the jurisdictional gap HRSP was created to fill.

BOTTOM LINE

Seven investigation triggers, all present. A documented attempted murder with prima facie forensic evidence. A prosecution pathway that bypasses UK interference through a US corporate entity in Newark, New Jersey. A Brown-trained surgeon in Istanbul ready to testify. Complainant witnesses in Texas with 30 years of forensic expertise and complete chain-of-custody documentation.

The evidence in this submission does not ask HRSP to extrapolate a pattern from fragments. It presents a 60-event convergence timeline spanning 22 years (Document 09), a 36-point evidentiary checklist (Document 04), nine identified perpetrators (Document 07), and a prosecution pathway that runs entirely through US federal jurisdiction.

HRSP does not need to prove this case to open an investigation. HRSP needs to determine whether the pattern is real. The evidence reviewed here provides more than sufficient basis for that determination.

Document 01 of 10. Prepared for HRSP submission package, 1 March 2026.

SENSITIVE: NOT FOR PUBLIC DISTRIBUTION

HUMAN RIGHTS FRAMEWORK, STATUTORY ANALYSIS

Legal Framing of Systematic Medical Targeting Under Federal Law

Document 02 of 10

Date: 1 March 2026

Purpose: To map the evidentiary record onto the statutory elements of 18 U.S.C. §§ 1091–1093 (Genocide), 18 U.S.C. §§ 2340–2340B (Torture), 18 U.S.C. § 2441 (War Crimes), and the immigration persecutor bar provisions of 8 U.S.C. §§ 1182(a)(3)(E) and 1227(a)(4)(D)

INTRODUCTORY NOTE

This document does not argue that the case is proved. It argues that the evidentiary record maps onto the statutory elements of four independent federal frameworks with sufficient specificity to warrant HRSP investigation.

The standard at this stage is not proof beyond reasonable doubt. It is not probable cause. It is whether the evidence, taken as a whole, provides a reasonable basis to believe that the conduct described may constitute violations of one or more of these statutes, and that the U.S. has jurisdiction to investigate.

Every statutory element below is addressed against the evidence presented in Documents 03 through 07.

I. GENOCIDE, 18 U.S.C. §§ 1091–1093

The Statute

Section 1091 prohibits acts committed with the **specific intent to destroy, in whole or in substantial part**, a national, ethnic, racial, or religious group. The statute enumerates six prohibited acts. Three are potentially applicable to this case:

- **§ 1091(a)(1):** Killing members of the group
- **§ 1091(a)(2):** Causing serious bodily injury to members of the group
- **§ 1091(a)(3):** Causing the permanent impairment of the mental faculties of members of the group through drugs, torture, or similar techniques

Element 1, Protected Group

Statutory requirement: The victims must belong to a national, ethnic, racial, or religious group.

Evidence:

The affected population is a genetically distinct group, the Irish Catholic community of County Tyrone, Northern Ireland. This population satisfies the protected group requirement on two independent grounds:

- **National/ethnic group:** The Irish Catholic population of County Tyrone constitutes a national and ethnic group, Irish, with a distinct cultural identity, historical continuity, and territorial connection to the land predating the Plantation of Ulster (1609). The Oxford University 2015 *Nature* study confirmed that the Northern Irish population is genetically distinct from the English population, forming a separate genetic cluster. This is not a political classification. It is a biological, cultural, and historical one.

  IMAGE NOT FOUND: `GREEN_TRIANGLES.jpg` – Check EVIDENCE folder

  IMAGE NOT FOUND: `GREEN_TRIANGLES_CLOSE_UP.jpg` – Check EVIDENCE folder

- **Religious group:** The affected population is Catholic. In Northern Ireland, Catholic identity is not merely denominational, it is a defining marker of the community's national and ethnic identity, inseparable from its Irish heritage. The Plantation of Ulster explicitly targeted Catholic landholders for dispossession. The targeting pattern identified in this submission falls along the same sectarian line.

Either ground alone satisfies § 1091. Both are present.

Assessment: The protected group is clearly defined, historically documented, and genetically distinct.

Element 2, Prohibited Acts

§ 1091(a)(1), Killing members of the group

The evidence documents a **prima facie attempted murder** of Nicola McNamee (Document 04): a medical file fabricated to explain her death under general anesthesia in Istanbul in December 2023, containing fabricated ECGs planting progressive ischemic heart disease as a pre-written cause of death, a fabricated echocardiogram recommendation never scheduled (a liability shield), and three layers of digital obfuscation concealing the fabrication until post-mortem. An independent echocardiogram at Memorial Hospital Istanbul proved the infarction findings are fabricated: structurally perfect heart, ejection fraction 65%, no wall motion abnormality.

Beyond this documented attempt, the evidence shows a mechanism capable of producing undetectable deaths at the population level:

- False diagnoses of Long QT Syndrome and Brugada Syndrome, conditions associated with sudden cardiac death during sleep, label the affected population with a medical narrative that explains away unexpected deaths as "natural causes"
- Northern Ireland's hospital autopsy rate has collapsed to approximately 0.46%, ensuring deaths can be certified without independent investigation
- No systematic molecular autopsy program exists in Northern Ireland, meaning the genetic basis of a SADS death cannot be verified or falsified post-mortem

Assessment: An attempted killing is documented with prima facie forensic evidence. The mechanism for population-level killing is established. Investigation is required to determine the full scope.

§ 1091(a)(2), Causing serious bodily injury

Section 1093(1) defines "serious bodily injury" to include injury involving substantial risk of death, extreme physical pain, protracted and obvious disfigurement, or protracted loss or impairment of the function of a bodily member, organ, or mental faculty.

The evidence documents:

- **Unnecessary pacemaker implantation:** The complainant witness's mother was fitted with a permanent pacemaker on the basis of a cascaded cardiac diagnosis. If the originating diagnosis is false, and a cardiologist outside the UK declared the hereditary diagnoses flatly wrong in January 2024, then the pacemaker is an unnecessary invasive surgical intervention. Pacemaker implantation requires general anesthesia, surgical incision, lead placement into the heart, and lifelong device management including generator replacements every 5–10 years. This constitutes serious bodily injury under § 1093(1). It is widely documented these devices can theoretically be remotely triggered to provoke cardiac arrest. This leaves her at the mercy and under the control of the same people operating the genocide.
- **Unnecessary cardiac medication administered to children:** Patients diagnosed with Long QT Syndrome and Brugada Syndrome are placed on restrictive medication regimens and advised to avoid entire categories of common drugs. If the underlying diagnosis is false, these restrictions constitute an ongoing impairment of bodily function imposed without medical justification. Children, including the complainant's brother and sister, were administered beta blockers as toddlers that have caused adverse reactions throughout their administration. As documented in the Kendall study, 216 children at a median age of 18 months have been continually administered beta blockers through the Belfast ICC program. Administering beta blockers to children who do not need them causes measurable physiological harm, bradycardia, hypotension, fatigue, exercise intolerance, and sleep disturbance, as well as documented developmental and mental health effects including depressive symptoms and cognitive blunting.
- **Propofol administration against contraindication:** The complainant witness's KIMS Hospital records document that propofol, a drug she had specifically identified as causing adverse reactions, was administered as an anesthetic induction agent over her documented objection. Three anesthetists are recorded in the file; the patient states she met none of them. The ASA grade was upgraded from 2 to 3 between the anesthetic review and the procedure without documented justification.

§ 1091(a)(3), Permanent impairment of mental faculties through drugs, torture, or similar techniques

The evidence documents:

- **Psychological impact of false terminal diagnosis:** Victims receiving these misdiagnoses, particularly Brugada Syndrome, are disadvantaged in several life-degrading ways. They struggle to obtain life insurance. As a result, obtaining a mortgage becomes difficult if not impossible. This poverty-inducing stigma erodes mental health. Victims are required to disclose to employers, partners, and family that they may carry a genetic mutation posing a real probability of early death. Patients told they carry an inherited condition associated with sudden cardiac death, and that their children may carry it too, suffer prolonged psychological harm. The cascade screening mechanism ensures this psychological burden propagates through entire families.
- **Family coercion:** The complainant witness's mother refuses to communicate unless it is over WhatsApp, despite knowing her daughter has been surveilled through WhatsApp and remains targeted. The complainant witness's cousin denied family diagnoses via Signal messaging, evidence of pressure to maintain the false narrative within the family structure. Coercion designed to prevent family members from seeking independent medical evaluation constitutes a technique intended to impair the victim's capacity to understand and respond to the targeting.

Element 3, Specific Intent (*Dolus Specialis*)

Statutory requirement: The acts must be committed with the **specific intent to destroy, in whole or in substantial part**, the protected group.

Evidence supporting intent inference:

1. **Selectivity of targeting:** The fraudulent diagnoses concentrate in County Tyrone's Irish Catholic population, not in Belfast, not in Antrim. The geographic precision maps onto the Curraghinalt gold deposit, the largest confirmed unmined gold vein globally. Radiating outward, the diagnostic boundaries conform to the optioned parcels and prospecting licenses that cover most of Tyrone, where community resistance to extraction is fierce. Random medical error would not produce this geographic and demographic pattern.

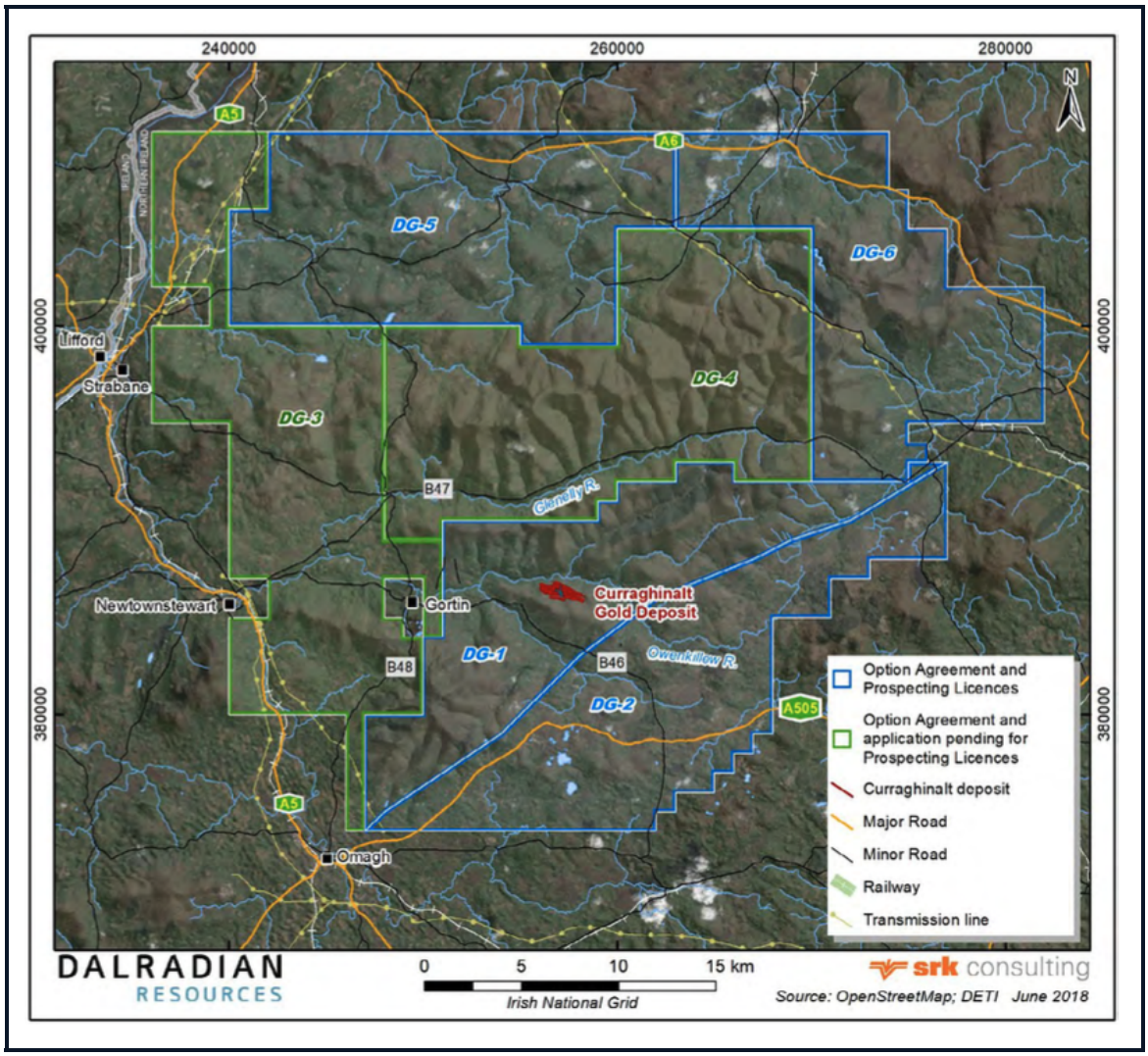

Figure 1: Boundaries of the Dalradian Gold 'Belt' covering County Tyrone

2. **Duration and persistence:** The campaign has operated continuously from approximately 2004 to the present, two decades. A twenty-year pattern is not an oversight. It is a program.

3. **Institutional investment in concealment:** The five-element cover-up architecture documented in Document 05, autopsy collapse, boundary restructuring, hospital closure, FOI firewalls, and molecular autopsy gap, represents substantial institutional investment in preventing detection. Entities do not invest this level of effort in concealing accidents.

4. **Financial motive aligned with group destruction:** The Curraghinalt gold deposit (£14-20 billion) can only be developed if community resistance is overcome. The Crown Estate collects a 4% royalty on all gold extracted. The financial motive aligns precisely with the destruction of the community's capacity to resist.

5. **The "substantial part" analysis:** The term "substantial part" in § 1091 has been interpreted by tribunals and federal courts to include qualitative significance, a group's leadership, its cultural core, or its capacity for self-governance. The Irish Catholic community of Greencastle, Rouskey, and Gortin represents the organized resistance to the mine. Targeting this population targets the community's

capacity to defend its territory. This satisfies the "substantial part" requirement under the qualitative interpretation adopted by the ICTY in *Prosecutor v. Krstić* and recognized in U.S. case law.

Element 4, Jurisdiction

Statutory requirement under § 1091(d): The offense is committed in the United States; the alleged offender is a U.S. national; the alleged offender is present in the United States; or the alleged offender is a stateless person habitually resident in the United States.

Evidence:

Four independent jurisdictional bases are documented in full in Document 03. The strongest are summarized here:

- **U.S. corporate financing of the mine:** Orion Mine Finance (New York City), a Delaware-registered private equity firm with $4.5 billion in assets under management, acquired a large equity stake and finances the Curraghinalt gold deposit through Dalradian Resources, a Canadian mine operator in September 2018. Orion's senior executives are based in New York and are present in the United States.
- **U.S. public pension fund exposure:** Ten confirmed state pension funds across nine states, including Florida SBA ($210B AUM), Washington State Investment Board ($125B), Texas Municipal Retirement System ($28B), and LACERA (Los Angeles County, $250M committed to Fund IV), are limited partners in Orion Mine Finance funds. American public retirement money is flowing into an enterprise whose development pathway may depend on systematic ethnic targeting. The potential losses to American retirees may be substantial.
- **U.S. citizen directly affected through connected operation:** Brian Spector is a United States citizen whose company 2REAL, (Delaware C-Corp) he co-founded with Nicola McNamee, the complainant witness, was the target of economic espionage by the same institutional apparatus that operates the suspected medical targeting campaign. He was arrested on false charges in the United Kingdom, held in solitary confinement, and denied U.S. Embassy notification in violation of the Vienna Convention on Consular Relations. Kent Police twice warned him that MoD operatives were attempting to imprison him and his partner with planted evidence. He fled the UK. His connection to the human rights violation is through his partner, Nicola McNamee, the primary complainant witness, Irish Catholic, County Tyrone, and through the documented fact that the same MoD operatives and intelligence apparatus appear in both the economic espionage operation and this HRSP submission.
- **U.S. corporate control of the hospital where the murder weapon was manufactured (PRIMARY PROSECUTION PATHWAY):** KIMS Hospital is controlled by the Subaskaran family through LycaHealth, acquired for £140M from Magnetar Capital (Evanston, IL) in October 2021. **Lycamobile USA Inc.** (Delaware corp, 24 Commerce Street, Newark, NJ, FCC Section 214 authority, ~542,000 American subscribers, $50-100M annual revenue) is a subsidiary of the same corporate group. A federal grand jury subpoena to Lycamobile USA opens the entire KIMS evidence chain without UK cooperation. Lycamobile's parent group has French criminal money laundering convictions (€10M fine, CEO imprisoned), £646M in unaccounted flows flagged by KPMG, and HMRC winding-up petitions. This is the jurisdictional lever that bypasses UK intelligence interference completely.
- **FTO activity on U.S. soil:** On 15 October 2025, an IRGC-designated operative conducted surveillance against the complainants in Washington, D.C., days after Brian Spector met with FBI agents at the Manassas field office. Spector's EEA referral provides evidence of the allegiance between UK MoD, Islamist factions and Chinese interests hostile to the US. The IRGC is a designated Foreign Terrorist

Organization since April 2019. Operational activity on U.S. soil against a U.S. citizen complainant is an automatic federal jurisdictional trigger.

- **U.S. diplomatic engagement:** U.S. Ambassador to the UK Warren Stephens publicly pressured Northern Ireland authorities to approve the mine in November 2025, framing it as part of the US-UK Economic Prosperity Deal.

II. TORTURE, 18 U.S.C. §§ 2340–2340B

The Statute

Section 2340A makes it a federal crime for any person to commit or attempt to commit torture outside the United States, where the alleged offender is a U.S. national or is present in the United States.

Section 2340 defines torture as an act committed by a person acting under the **color of law** specifically intended to inflict **severe physical or mental pain or suffering** upon another person within his custody or physical control.

Element 1, Color of Law

Statutory requirement: The perpetrator must act under the color of law, i.e., in an official or quasi-official capacity.

Evidence:

The conduct is carried out through multiple layers of state authority:

- **NHS as state healthcare institution:** The NHS Inherited Cardiac Conditions Service is operated under the authority of the Belfast Health and Social Care Trust, a public body funded by and accountable to the Northern Ireland Department of Health. The physicians, nurses, and technicians operating the cardiac screening program act under state authority.
- **MI5 permanent operational control:** The St Andrews Agreement, Annex E (2006), transferred permanent control of national security intelligence in Northern Ireland to MI5. This was implemented during a period of direct rule when the Northern Ireland Assembly (Stormont) was prorogued between 2002 and 2007. MI5's accountability runs to Westminster, not to the devolved Northern Ireland government, not to the Police Service of Northern Ireland, and not to the Northern Ireland Policing Board.
- **The Special Reconnaissance Regiment (SRR):** Annex E also established the operational framework under which British Army special forces, specifically the SRR, operate under MI5 direction in Northern Ireland. The Committee on the Administration of Justice (CAJ) described this arrangement as a **"parallel police force"** with **"minimal or non-existent"** accountability to devolved NI institutions. The NI Department of Justice has no access to pre-devolution national security records. The Police Ombudsman must report national security issues to the Secretary of State at Westminster, not to NI authorities. Misconduct panels must defer to the UK government on whether they may access national security information. The result is an opaque security apparatus operating within Northern Ireland's borders with no oversight from any Northern Ireland institution, a parallel policing structure with no democratic accountability to the population it operates against.

- **Statutory shielding framework:** The UK government's Official Secrets Act and the Justice and Security Act 2013 (Closed Material Procedures) provide the legal framework through which these operations are shielded from domestic scrutiny.

The conduct is carried out through state institutions (NHS), by or at the direction of state agents (MI5/MoD/SRR), under statutory authority (St Andrews Agreement, Annex E), and shielded by state legal mechanisms (OSA, CMP). This satisfies the color of law requirement comprehensively.

Element 2, Severe Physical Pain or Suffering

Evidence:

- **Unnecessary pacemaker implantation**, a permanent surgical device implanted on the basis of a cascaded false diagnosis. The procedure involves general anesthesia, surgical incision, and lead placement into the cardiac chambers. If the diagnosis is fabricated, the entirety of this procedure constitutes severe physical pain inflicted without medical justification.
- **Unnecessary beta blocker administration to children**, The Kendall et al. (2025) study documents 216 children diagnosed through the Belfast ICC program at a median age of 18 months. These children have been continually administered beta blockers, cardiac medications that suppress heart rate and blood pressure. In children who do not have the cardiac conditions for which beta blockers are indicated, chronic administration causes: bradycardia (dangerously low heart rate), hypotension, chronic fatigue, exercise intolerance, sleep disturbance, and documented neurodevelopmental effects including depressive symptoms, cognitive blunting, and behavioural changes. The complainant witness's brother and sister were administered these medications as toddlers and have experienced adverse reactions throughout their course of treatment. If the underlying diagnoses are false, as the evidence in Document 03 supports, then the systematic administration of cardiac drugs to children who do not need them constitutes severe physical suffering imposed through medical authority.
- **Propofol administration over patient objection**, the complainant witness's documented drug sensitivity was overridden without her consent during a procedure at KIMS Hospital. Three anesthetists are recorded; the patient met none. The ASA classification was changed from grade 2 to grade 3 between the anesthetic review and the procedure without explanation.
- **Restrictive medication regimens**, patients carrying false Long QT/Brugada diagnoses are placed on lifelong drug restrictions that constrain their daily lives, their ability to receive emergency medical care, and their autonomy. They are barred from entire categories of commonly prescribed medications, affecting treatment options for unrelated conditions throughout their lives.

Assessment: The pacemaker implantation and the systematic administration of beta blockers to children meet the severity threshold. The propofol override and medication restrictions provide supporting evidence of a pattern of state-administered medical harm.

Element 3, Severe Mental Pain or Suffering

Section 2340(2) defines "severe mental pain or suffering" as the prolonged mental harm caused by or resulting from specific predicate acts. Two predicate acts are applicable:

§ 2340(2)(A), Intentional infliction or threatened infliction of severe physical pain or suffering:

- A false diagnosis of Long QT Syndrome or Brugada Syndrome tells the patient she carries a condition that can cause sudden death during sleep. This is a threat of death imposed through medical authority. The patient is told this condition is inherited, meaning her children are at risk. The psychological burden of believing oneself to carry a lethal genetic condition, and of believing one has passed that condition to one's children, constitutes prolonged mental harm of the most severe character.

§ 2340(2)(D), Threat that another person will imminently be subjected to death or severe physical pain:

- The cascade screening mechanism ensures that family members are diagnosed after the index patient. The complainant witness was told her condition was "the worst in the family", a statement that simultaneously threatened her life and communicated that her family members were also at risk. Her mother was subsequently fitted with a pacemaker. Her brother and sister were diagnosed. The family-wide dimension of the campaign transforms each individual false diagnosis into a threat against the patient's loved ones.

Assessment: Strong. The psychological architecture of the false diagnosis, a lethal genetic condition transmitted to one's children, is precisely the kind of prolonged mental harm § 2340 was enacted to address.

Element 4, Specific Intent

Statutory requirement: The act must be **specifically intended** to inflict severe physical or mental pain or suffering.

The intent analysis parallels the genocide framework. The evidence of concealment, three-layer document obfuscation, absent original records, the deliberate non-performance of genetic testing, demonstrates that the diagnosticians knew the diagnoses were false. One does not invest in sophisticated concealment of legitimate medical practice. The absence of the one test that would confirm or exclude the diagnosis (genetic testing) is the most powerful evidence of intent: it is the test they could not fake.

Assessment: The concealment evidence provides the strongest basis for the intent inference.

Element 5, Jurisdiction

Statutory requirement under § 2340A: Jurisdiction attaches when the alleged offender is a U.S. national or is present in the United States, regardless of nationality.

Evidence:

The torture statute's jurisdictional reach covers any perpetrator who enters U.S. soil. Multiple perpetrators and their corporate vehicles have documented U.S. presence:

- **Lycamobile USA Inc. (Newark, NJ).** The corporate group that controls KIMS Hospital, where the fabricated medical file was manufactured, operates a U.S. subsidiary at 24 Commerce Street, Newark, NJ. The former CEO of Lycamobile USA, **Christopher Tooley**, received a three-year prison sentence from a Paris criminal court in October 2023 for money laundering, along with a €250,000 fine and a five-year ban from business management. The conviction involved €17 million laundered through shell companies and false invoices. Tooley was also a governor (officer) of Lycamobile USA per

Washington State and Delaware filings. Federal subpoena authority over Lycamobile USA opens the KIMS evidence chain. Revenue from U.S. telecommunications operations flows through the Lyca Group's intercompany network to LycaHealth, the KIMS Hospital owner. The full Lycamobile criminal history is documented in the appendix to this submission.

- **Dalradian Resources executives (Canadian operators of the mine).** Dalradian is the Canadian company that operates the Curraghinalt mine. Its executives have documented U.S. travel and a track record that is directly relevant to the conduct alleged in this submission. Dalradian's CEO **Patrick Anderson** would have traveled routinely to New York for meetings with Orion Mine Finance, the project's financial backer. Dalradian's senior management includes **Brian Kelly** (Managing Director since 2014, previously ran CEMEX's operations in Ireland), **Keith McKay** (CFO, previously CFO at Continental Gold in Colombia where three staff were killed and the project faced massive community resistance over forced displacement of local populations), and **Eric Tremblay** (COO, previously General Manager at Canadian Malartic, Canada's largest open-pit gold mine, subject to class action lawsuits from residents over damage from dust, noise, and explosions). All three also became directors of **Minco PLC**, connected to mining projects worldwide with documented environmental devastation, indigenous displacement, and worker deaths. Board member **Sean Roosen** (Osisko Gold Royalties, $30M+ CAD invested) has extensive U.S. connections. The PSNI invoiced Dalradian approximately £400,000 for escorting the company's explosives, blurring the line between public policing and private security. Dalradian compiled a 92-page surveillance dossier on community leader Cormac McAleer. Any Dalradian executive who entered U.S. soil while participating in an operation that required the systematic targeting of the community opposing the mine is subject to § 2340A jurisdiction. Their operational history at other sites demonstrates a pattern of engagement with environments where community opposition is crushed rather than accommodated.
- **Peter Mandelson.** Appointed UK Ambassador to the United States in 2025. Present on U.S. soil in an official capacity until his arrest on February 23, 2026. Mandelson held three government positions spanning the entire ownership chain of the gold deposit (Document 07). His presence in the United States as Ambassador is an automatic jurisdictional trigger under § 2340A.
- **UK-based medical perpetrators.** Any physician, administrator, or intelligence operative identified in the Belfast ICC targeting campaign who has traveled to the United States for any purpose (conferences, personal travel, professional exchange) is subject to § 2340A jurisdiction upon entry. HRSP should request CBP entry records for all individuals identified in Document 07.
- Immigration consequences under 8 U.S.C. § 1182(a)(3)(E) apply to any identified perpetrator who seeks entry to the United States.

Note on Orion Mine Finance (New York). Orion is not a perpetrator. Orion is a New York-based private equity firm that invested over $400 million in the Curraghinalt deposit through Dalradian Resources. The evidence does not suggest Orion had knowledge of the systematic targeting. Orion and its U.S. pension fund investors (ten state pension funds across nine states) are potential victims whose retirement capital is at risk when this evidence enters the public domain. They did not know. They are not complicit. However, Orion's records regarding its due diligence on Dalradian, its awareness of community opposition, and any communications with UK government entities may contain evidence relevant to the investigation and are subpoenable through standard federal process.

Assessment: The torture framework has multiple independent jurisdictional bases. Lycamobile USA's former CEO is a convicted money launderer. Dalradian's Canadian executives have documented U.S. travel, documented histories at mining operations where communities were forcibly displaced, and a documented pattern of corporate surveillance against community opposition in Tyrone itself. Peter Mandelson was

physically present in the United States as Ambassador until twelve days ago. Any UK-based perpetrator who has ever entered the United States is reachable through CBP records.

III. WAR CRIMES, 18 U.S.C. § 2441

The Statute

Section 2441 makes it a federal crime for a U.S. national or member of the U.S. Armed Forces, or any person with respect to a victim who is a U.S. national or Armed Forces member, to commit a war crime, defined to include grave breaches of the Geneva Conventions and other specified violations.

The Armed Conflict Threshold

Section 2441 requires that the conduct occur in the context of and in association with an **armed conflict**. This is the threshold question.

Northern Ireland is not currently in a state of armed conflict as conventionally understood. The Good Friday Agreement (1998) ended the formal conflict, and no international or non-international armed conflict is recognized by international bodies.

However, three considerations warrant preserving this statutory framework in the analysis:

1. **The St Andrews Agreement (2006) preserved a wartime intelligence architecture.** Annex E transferred permanent MI5 control of national security intelligence in Northern Ireland during a period of direct rule, when the Northern Ireland Assembly (Stormont) was prorogued between 2002 and 2007. This arrangement was designed for counter-terrorism during the Troubles. The continuation of this wartime architecture into peacetime, including the SRR operating as a parallel policing structure with no accountability to devolved institutions, and its apparent deployment against a civilian population for resource extraction purposes, raises the question of whether the legal framework governing intelligence operations in Northern Ireland has itself failed to transition to peacetime.
2. **Common Article 3 of the Geneva Conventions** applies to situations of internal armed conflict and prohibits violence to life and person, cruel treatment, torture, and outrages upon personal dignity against persons taking no active part in hostilities. If the intelligence architecture established during the Troubles is being used to target civilians during peacetime, there is an argument, not yet established but worth investigating, that the state itself has not ended the conflict with respect to this population.
3. **The Military Commissions Act of 2006** amended § 2441 to include specific offenses, murder, torture, cruel or inhumane treatment, taking hostages, intentionally causing great suffering, that do not require a full armed conflict determination if committed by or against U.S. nationals.

Applicable Provisions (If Armed Conflict Threshold Met)

- **§ 2441(d)(1)(A), Torture:** Identical in substance to the § 2340 analysis above
- **§ 2441(d)(1)(B), Cruel or inhumane treatment:** A broader standard than torture, encompassing treatment that causes serious physical or mental pain but may fall short of the "severe" threshold under § 2340
- **§ 2441(d)(1)(D), Intentionally causing great suffering or serious injury to body or health:** The unnecessary pacemaker implantation, the chronic administration of beta blockers to children, and the psychological burden of false terminal diagnoses fall within this provision

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IV. IMMIGRATION CONSEQUENCES, PERSECUTOR BAR AND DEPORTABILITY

8 U.S.C. § 1182(a)(3)(E), Inadmissibility

Any alien who has **ordered, incited, assisted, or otherwise participated in** persecution on account of race, religion, nationality, membership in a particular social group, or political opinion is **inadmissible** to the United States.

The persecutor bar does not require a criminal conviction. It does not require proof beyond reasonable doubt. It requires a reasonable basis to believe the individual participated in persecution based on a protected characteristic.

Application:

Every medical professional, intelligence operative, and government official identified in Document 07 (Perpetrator Profiles) who is not a U.S. citizen is subject to the persecutor bar if the evidence supports their participation in the systematic medical targeting of the Irish Catholic population.

The persecutor bar is particularly powerful in this context because:

- It operates independently of criminal prosecution, HRSP can refer individuals for visa revocation even if criminal proceedings are not initiated
- It captures a broad range of conduct, not just the physicians who issued the diagnoses, but administrators who facilitated the cascade screening, intelligence operatives who directed the concealment, and officials who prevented investigation
- The "persecution" standard under immigration law is lower than the statutory elements of genocide or torture, it requires discriminatory conduct causing harm on account of a protected characteristic, without the "severe pain or suffering" threshold of § 2340 or the "intent to destroy" requirement of § 1091

8 U.S.C. § 1227(a)(4)(D), Deportability

Any alien present in the United States who has participated in persecution under § 1182(a)(3)(E) is **deportable**.

Application:

If any identified perpetrator is currently present in the United States, as a visa holder, legal permanent resident, or otherwise, they are deportable upon a showing that they participated in the persecution. HRSP has coordinated denaturalization proceedings in analogous cases.

V. STATUTORY FRAMEWORK SUMMARY

Statute	Elements Met	Elements Requiring Development	Threshold
18 U.S.C. §§ 1091–1093 (Genocide)	Protected group; serious bodily injury (pacemaker, beta blockers to children); institutional mechanism; jurisdiction (four independent bases)	Specific intent (circumstantial but convergent); killing (mechanism documented, deaths require investigation)	Specific intent to destroy in whole or substantial part
18 U.S.C. §§ 2340–2340B (Torture)	Color of law (NHS, MI5, SRR, MoD, CMP); severe physical pain (pacemaker, beta blockers, propofol); severe mental pain (false terminal diagnosis, family cascade); concealment evidence supporting intent	Additional perpetrator identification	Specific intent to inflict severe pain or suffering under color of law
18 U.S.C. § 2441 (War Crimes)	Conduct consistent with prohibited acts; state actors identified; wartime architecture preserved post-conflict	Armed conflict threshold	Conduct in context of armed conflict

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VI. COORDINATION WITH PARALLEL AUTHORITIES

The human rights framework does not operate in isolation. This submission identifies conduct that also falls within the jurisdiction of:

- **18 U.S.C. §§ 1961–1968 (RICO):** The pattern of racketeering activity across multiple entities, jurisdictions, and predicate offenses may support a RICO prosecution. The enterprise spans healthcare institutions, intelligence agencies, legal firms, and corporate entities.
- **The Global Magnitsky Human Rights Accountability Act:** Provides for targeted sanctions, asset freezes and visa bans, against individuals responsible for significant human rights abuses. This is a complementary tool that HRSP can coordinate through the Office of Foreign Assets Control (OFAC).

CONCLUSION

Three independent federal statutory frameworks provide legal authority for HRSP to investigate this matter. Presented are a protected population systematically targeted through institutional mechanisms, for an identifiable financial motive, with state-level concealment, precisely the category of conduct these statutes were enacted to address.

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U.S. JURISDICTIONAL NEXUS

Four Independent Bases for Federal Jurisdiction

Document 03 of 10

Date: 1 March 2026

PURPOSE

This document establishes that HRSP possesses federal jurisdiction over the suspected systematic targeting of the Irish Catholic population of County Tyrone, Northern Ireland. The analysis identifies four independent jurisdictional bases. Any one is sufficient. Together, they provide HRSP with a prosecution pathway that runs entirely through U.S. federal jurisdiction and requires no cooperation from the UK government at any stage.

NEXUS 1: LYCAMOBILE USA AND THE KIMS HOSPITAL EVIDENCE CHAIN

This is the prosecution pathway. It bypasses UK intelligence interference completely.

KIMS Hospital (Maidstone, Kent) is where the fabricated medical file was manufactured: the fabricated ECGs, the fabricated consultations, the three-layer digital obfuscation, and complicit staff. This hospital is not an independent UK institution beyond federal reach. It is controlled by a corporate group with a U.S. subsidiary subject to full federal jurisdiction.

Corporate Chain

Level	Entity	Detail
Hospital	KIMS Hospital Limited (UK 07525422)	Maidstone, Kent. Where the fabricated medical file was manufactured
Holding	KIMS Kent Holdings Limited (09911624)	Directed by Prema Subaskaran (appointed Oct 2021)
Acquirer	Lyca Health Limited (08519935)	**Dormant company** at Companies House. Files no trading accounts. PSC: Allirajah Subaskaran

Level	Entity	Detail
Parent	Lyca Group	60+ entities, 23 countries. Centralized financial control under Allirajah Subaskaran
US Entity	**Lycamobile USA Inc.**	Delaware corp (29 Nov 2012). 24 Commerce Street, Newark, NJ. FCC Section 214 authority. ~542,000 subscribers. $50-100M annual revenue
Prior owner	**Magnetar Capital**	Evanston, Illinois. US hedge fund. Sold KIMS to LycaHealth Oct 2021 for up to £140M

The Prosecution Pathway

A federal grand jury subpoena to **Lycamobile USA Inc.** (Newark, NJ) for all records relating to KIMS Hospital, LycaHealth, medical records production protocols, and the identity, direction, and communications of records staff and executives:

- Does not require UK cooperation
- Does not require an MLAT request
- Does not require any UK government permission
- Does not pass through MI5, PSNI, or any UK intelligence channel
- Is enforceable through standard federal contempt process

A parallel subpoena to **Magnetar Capital** (Evanston, IL) for all records relating to KIMS Hospital operations, compliance status, and records management at point of sale provides a second US-reachable evidence channel.

White & Case LLP (New York) has active litigation against Lycamobile USA for $7.8 million in unpaid legal fees (NY County Supreme Court), confirming that US courts already have personal jurisdiction over this entity.

Financial Chain (Lycamobile USA Revenue to KIMS)

Lycamobile UK's own audited accounts state: "the majority of the cash flows associated with investing and financing activities are ultimately with related parties under the control of Mr A Subaskaran." The acquisition of KIMS for £140M was funded through the Lyca Group intercompany network. US revenue enters through Lycamobile USA, routes through the Madeira IP structure (Pettigo Comercio Internacional Lda, 90.5% owned by Prema Subaskaran, 5% corporate tax), and converges with global revenues under centralized family control.

Criminal History of the Parent Group

Jurisdiction	Finding
France	Criminal conviction for money laundering (Oct 2023). €10M fine. CEO Christopher Tooley: 3-year prison sentence
Germany	€70M unpaid sales taxes. Subsidiary renamed and vanished
United Kingdom	£51M VAT ruling (Jul 2024). HMRC winding-up petitions (Aug 2024). 90% UK workforce cut (Dec 2024)
Auditors	EY resigned (2014). KPMG resigned (2017): **£646M unaccounted**. PKF Littlejohn: disclaimer of opinion
BuzzFeed	Filmed cash rucksacks (£240K each, twice daily) at London post offices
HMRC	Refused French raid request citing Conservative Party donations
US (California)	CPUC revoked wireless license (2016). Operated without authority 10 months

The hospital where the murder weapon was manufactured is controlled by a documented criminal enterprise whose US subsidiary is a Delaware corporation with FCC registration, hundreds of thousands of American subscribers, and active litigation in New York courts.

NEXUS 2: U.S. CITIZEN DIRECTLY AFFECTED

Brian Spector is a United States citizen. He is the fiance of the primary complainant witness (Nicola McNamee), a direct target of a connected foreign intelligence operation documented in the parallel EEA referral, and a victim of the same institutional apparatus that operates the suspected medical targeting campaign.

Element	Detail
Citizenship	United States
Current location	Texas
Partner	Nicola McNamee, primary complainant witness, Irish Catholic, County Tyrone
Company	2REAL, Delaware C-Corp, Texas domicile
Persecution	Fled to US after warnings of imminent false arrest orchestrated by MoD

A U.S. citizen directly affected by conduct connected to suspected human rights violations is the statutory basis for jurisdiction under both 18 U.S.C. § 1091 and 18 U.S.C. § 2340A.

CLOUD Act Dual Nationality Violation

Brian Spector holds dual US/UK citizenship. This creates a separate legal violation that compounds the jurisdictional basis.

The US-UK CLOUD Act bilateral data access agreement (effective October 2022) permits UK authorities to serve data orders directly on US communications providers for real-time interception and stored data of non-US persons, without notifying any US court or agency. The agreement contains a critical safeguard: if the target is a US person, the UK must notify the US and obtain US judicial authorization.

No US government body independently reviews UK CLOUD Act orders. No one verifies the UK's nationality classification of the target. The UK had direct incentive to classify Brian Spector as non-US in warrant applications: disclosing his US citizenship would trigger the agreement's mandatory US judicial oversight, oversight that would expose the surveillance to American authorities.

If the UK concealed Brian Spector's US citizenship:

- Every surveillance action against him on American soil violated the agreement's US person protections
- The UK monitored a US citizen's communications with US federal law enforcement (FBI, congressional offices, this submission) without any American court or agency knowing
- The bilateral agreement was breached by deliberate misrepresentation
- The CLOUD Act's central safeguard failed because no US authority verifies UK nationality classifications

Nicola McNamee, as a UK citizen, has no CLOUD Act protections at all. The UK can surveil her communications on American soil through American companies with zero US oversight.

The CLOUD Act violation is not a historical event. It is an ongoing operation against a US citizen on US soil, conducted through American technology companies, while that citizen attempts to alert American authorities to genocide, attempted murder, and economic espionage against US interests.

NEXUS 3: DALRADIAN RESOURCES OPERATORS WITH U.S. PRESENCE

Dalradian Resources is the Canadian mining company that operates the Curraghinalt mine. Its executives have documented U.S. travel and track records at mining operations associated with community displacement, violence, and environmental devastation.

Executive	Role	US Presence	Prior Operations
Patrick Anderson	CEO, Founder	Routine NYC travel for Orion meetings	Founded Dalradian
Brian Kelly	Managing Director (since 2014)	Routine NYC travel for Orion meetings	Previously ran CEMEX's Ireland operations
Keith McKay	CFO	Routine NYC travel for Orion meetings	Previously CFO at Continental Gold, Colombia (3 staff killed, forced displacement, massive community resistance)

Executive	Role	US Presence	Prior Operations
Eric Tremblay	COO	Routine NYC travel for Orion meetings	Previously GM at Canadian Malartic (class action lawsuits from residents for dust, noise, explosions)
Sean Roosen	Board, Osisko Gold Royalties	Extensive US connections, $30M+ CAD invested	Osisko: royalties from mines linked to indigenous displacement (Kenya), toxic waste (Macedonia)

All three senior managers (Kelly, McKay, Tremblay) also became directors of **Minco PLC**, connected to mining projects worldwide with documented environmental devastation, indigenous displacement, and worker deaths across Ireland, Colombia, Canada, Mexico, Chile, Ecuador, and Kenya.

Dalradian's conduct in Tyrone is consistent with their operational pattern. The PSNI invoiced Dalradian approximately £400,000 for escorting the company's explosives, blurring the line between public policing and private security. Dalradian compiled a 92-page surveillance dossier on community leader Cormac McAleer. Mine opponents received death threats (PSNI confirmed, June 2019, no prosecution). McAleer was attacked with a nailed bar.

Any Dalradian executive who entered U.S. soil while participating in an operation that required the systematic targeting of the community opposing the mine is subject to § 2340A jurisdiction. HRSP should request CBP entry records for all Dalradian executives.

NEXUS 4: U.S. PENSION FUND EXPOSURE

American public pension funds, managing retirement money for teachers, firefighters, police officers, and state employees, are limited partners in Orion Mine Finance funds. Orion is the New York-based private equity firm that finances the Curraghinalt mine through its controlling stake in Dalradian Resources.

Confirmed Orion Fund Investors

Pension Fund	State	AUM	Fund
State Board of Administration of Florida	FL	$210B	Fund II
Washington State Investment Board	WA	$125B	Fund II
Texas Municipal Retirement System	TX	$28B	Fund II
Maine Public Employees Retirement System	ME	$14B	Fund II
Arizona Public Safety Personnel Retirement System	AZ	$10B	Fund II
LACERA (Los Angeles County Employees Retirement)	CA		Fund IV ($250M committed)
Oregon Public Employees Retirement Fund	OR		LP (confirmed)
North Carolina Retirement Systems	NC		LP (confirmed)
State of Michigan Retirement System	MI		LP (confirmed)
University of Michigan Endowment	MI	$11.3B	Fund II

Orion pitch documents cite **22 public plans** across strategy. Only those listed above have been publicly identified through pension fund disclosures and financial press reporting.

American Investors Are Victims

These pension funds did not know. They are not complicit. They are at risk.

American public money, retirement savings for millions of workers, is invested in a mine sitting on top of an active genocide. If this evidence enters the public domain without federal action, and it will, every pension fund listed above faces reputational devastation, fiduciary duty challenges, and potential divestment pressure. The federal government has an obligation to protect these investors by investigating the genocide, compensating the victims, offering resettlement in the USA, obtaining total control of the enterprise, ensuring the investment can proceed on legitimate terms.

Note on Orion Mine Finance. Orion is not identified as a perpetrator. Orion is a New York-based private equity firm (Delaware-registered, $4.5 billion AUM) that acquired a controlling stake in Dalradian for US$407 million in September 2018. The evidence does not suggest Orion had knowledge of the systematic targeting. However, Orion's records regarding due diligence on Dalradian, awareness of community opposition, and communications with UK government entities are subpoenable through standard federal process and may contain evidence relevant to the investigation.

NEXUS SUMMARY

#	Nexus	Entity	Jurisdictional Basis
1	**Lycamobile USA / KIMS Hospital**	Lycamobile USA Inc. (Newark, NJ)	Grand jury subpoena. Entire KIMS evidence chain. No UK cooperation required
2	**U.S. citizen victim**	Brian Spector (Texas)	Direct statutory basis, 18 U.S.C. §§ 1091, 2340
2A	**CLOUD Act dual nationality violation**	Brian Spector (dual US/UK)	UK likely concealed US citizenship in CLOUD Act warrants. Ongoing surveillance of US citizen on US soil through US companies without US oversight
3	**Dalradian operators**	Anderson, Kelly, McKay, Tremblay, Roosen (Canadian, documented US travel)	§ 2340A (present in US). CBP records. Pattern of community displacement
4	**U.S. pension fund investors**	10 state pension funds, 9 states	SEC coordination. Investors as victims. Policy urgency

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CONCLUSION

Four independent jurisdictional bases. The question is not whether jurisdiction exists. It is which basis HRSP leads with.

Lead with Nexus 1 (Lycamobile USA / KIMS Hospital). A grand jury subpoena to a Delaware corporation in Newark, New Jersey opens the entire evidence chain of the attempted murder. No MLAT. No UK cooperation. No MI5 obstruction. The UK government cannot block this pathway because it runs entirely through U.S. federal jurisdiction.

Support with Nexus 3 (Dalradian operators) for perpetrator-level targeting, **Nexus 4 (pension funds)** for policy urgency, and **Nexus 2 (U.S. citizen)** for the statutory basis.

American investors are victims, not perpetrators. The Department's role is to protect them by investigating the genocide and ensuring the Curraghinalt deposit serves American interests rather than funding a Five Eyes ally's campaign against a civilian population.

Document 03 of 10. Prepared for HRSP submission package, 1 March 2026.

SENSITIVE: NOT FOR PUBLIC DISTRIBUTION

04: INSTITUTIONAL ANALYSIS: MEDICAL INFRASTRUCTURE

Version 6.0, 1 March 2026

The Attempted Murder of Nicola McNamee

In late 2023, Nicola McNamee did something the institutional medical infrastructure that had controlled her care since childhood could not afford: she asked for a clearance letter.

Nicola, a 38-year-old Irish Catholic woman from County Tyrone, Northern Ireland, had carried two cardiac diagnoses since her twenties: Long QT Syndrome and Brugada Syndrome. Both had been delivered by the NHS Inherited Cardiac Conditions Service at Belfast City Hospital, the sole diagnostic authority for inherited channelopathies in Northern Ireland. She had been told she could die suddenly in her sleep. She had been told which drugs could kill her. She had been told her heart was a weapon pointed at itself. She had lived with that weight for over a decade.

That clearance letter was the tripwire.

The moment Nicola stepped outside the institutional architecture that had diagnosed her, the fabricated diagnoses were at risk of independent scrutiny.

They could not stop her from going to Istanbul. So they built a file designed to explain her death.

A timeline containing all content contained within the medical file is in document 04 Addendum Section 2. Fabricated consultations, fabricated ECGs planting progressive ischemic heart disease across three dates over months, and doctored timelines designed to limit liability when she died on the operating table. Every invented element serves a specific narrative function in the post-mortem story.

The Screenplay

What the timeline reveals is not a medical record. It is a screenplay, a document written for a single reader who was never supposed to be a living patient or her advocate. It was written for a pathologist, standing over a body, reconstructing why a young woman's heart stopped on an operating table in Istanbul.

Nicola's KIMS Hospital medical records have been constructed to provide the perfect cover for her murder.

Fabricated consultation (9 May 2023). A clinic appointment that never took place. The hospital's own internal staff notes contradict it, voicemails left the next morning trying to reach a patient who supposedly

attended the previous evening, follow-up calls a week later to a patient who remains unreachable. The booking ID does not match the date field of May, in fact it is copied from the genuine November appointment created six months later. The consultation is a fiction. But the letter it generated serves a precise function: it repositions Nicola's Brugada diagnosis from an institutional finding delivered by the Belfast ICC to a claim the patient herself reported to a consultant who then looked for it and could not find it.

The letter concludes: *"I do hope you found this evening's consultation helpful. I think it would be helpful to get some baseline tests including a 24-hour ECG and an echocardiogram and to get the results of your tests from St George's and Belfast. You are going to approach the St George's unit and Belfast to see if they can give us some information."* If Nicola died on the table, this letter would tell the pathologist: *the patient claimed she had Long QT and Brugada, but her own cardiologist couldn't confirm it. He recommended further testing, but the patient was told to chase her own records. The real problem was something else entirely.*

The fabricated ECGs. All ECGs in the KIMS file contain machine-generated findings of myocardial infarction, heart attack damage, planted across three separate dates. The findings construct a progressive ischemic heart disease narrative: anterior wall infarction from May, evolving lateral infarction by November, with a claim of active disease progression between two tracings taken 72 seconds apart. Long QT Syndrome does not produce infarction findings. Brugada Syndrome does not produce infarction findings. The fabricated ECGs describe coronary artery disease with progressive myocardial damage, a condition that kills people under general anesthesia. They are the pre-written cause of death.

The fabricated echocardiogram recommendation. The KIMS file contains a recommendation for a transthoracic echocardiogram, the one test that would definitively contradict the fabricated infarction findings by showing a structurally normal heart. No echocardiogram was ever scheduled. No appointment was ever made. The recommendation exists in the file as a liability shield, a line in the screenplay that says: *we recommended further testing, but the patient did not follow through.* When the pathologist reads the file, the story is: *we tried to help her. She didn't come back. If she had, we might have caught the ischemic heart disease that killed her.*

The three-layer obfuscation. The fabricated findings were visible in a PDF whose text layers had been stripped from 116 of 140 pages, whose 2,050 embedded objects had been systematically purged of metadata, and whose ECG data had been flattened to raster images. The infarction findings are printed on the tracings, visible to a human being who turns to those pages, but invisible to every text search, every AI system, every automated legal or insurance review tool. The findings were designed to survive in the file undetected during the patient's lifetime, discoverable only afterwards, by a pathologist examining paper records at post-mortem.

The planted email trail. The fabricated consultation letter was deposited in Nicola's own Office 365 sent items, three emails across two dates, all following an identical anomalous pattern: no subject line, no body text, just cardiac fabrication documents attached. Genuine emails from the same dates have subjects, body text, and recognizable forwarding context. The planted emails ensure that when Nicola's archives are produced in any proceeding, she looks like the source of the fabrication, not its victim.

Every element of this file points in the same direction: toward a death on an operating table in Istanbul, with a ready-made explanation that would survive any post-mortem inquiry.

What Happened in Istanbul

Nicola McNamee was not supposed to end up at Memorial Hospital. She was supposed to die at a different clinic.

The Surgeon Switch

Nicola was originally scheduled for surgery at the **Dr. Cem Berkay Sinaci Clinic** in Istanbul, approximately 1 December 2023, the day after Brian Spector's own procedure on 30 November.

At the Sinaci clinic, something clinically extraordinary occurred: **the anesthetist refused to meet Nicola for a pre-operative consultation.** For a patient carrying diagnoses of both Brugada Syndrome and Long QT Syndrome, conditions that make general anesthesia a high-risk procedure requiring specialized cardiac monitoring, restricted drug protocols, and immediate access to defibrillation, the refusal of an anesthetist to conduct a pre-operative assessment is not negligence. It is the removal of the single pre-operative step most likely to identify that the cardiac diagnoses were fabricated or required independent assessment. Instead of an anesthesia consultation, Ms. McNamee was directed to obtain a mammogram. The clinic's liaison representative, Ena Radoncic, facilitated communications throughout this period but could not arrange the anesthetist meeting.

Nicola arranged a list minute switch of surgeon to **Dr. Ercan Karacaoglu** of Este Istanbul, operating at Memorial Hospital. Surgery was rescheduled for **12 December 2023**.

Dr. Karacaoglu's credentials are directly relevant to his evidentiary value:

Field	Detail
Medical degree	GATA Gulhane Military Medical Academy, Ankara (1992), the Turkish armed forces' premier medical institution
US training	Aesthetic & Breast Surgery Fellowship, Brown University School of Medicine (2000-2001); Breast Surgery Fellowship, Brown University Alpert Medical School / Rhode Island Hospital (2003-2004)
US academic appointment	Adjunct Professor, Brown University
Military service	Chief of Plastic Surgery, Golcuk Naval Hospital (2002-2003)
Current academic position	Professor of Plastic Surgery, Bahcesehir University; Head of Department, Aesthetic, Plastic & Reconstructive Surgery, Memorial Atasehir Hospital
Board certifications	Fellow, European Board of Plastic, Reconstructive and Aesthetic Surgery (EBOPRAS); Member, American Society of Plastic Surgeons (ASPS); Member, International Society of Aesthetic Plastic Surgery (ISAPS)
Publication record	Over 100 peer-reviewed publications, 800+ academic citations, co-editor of Springer international textbook

This is a Brown-trained, ASPS-member, EBOPRAS-certified surgeon with military medical training and over 100 publications. A test (echocardiogram) for a patient whose referral letter described an electrical cardiac condition (channelopathy) was ordered. A surgeon with these credentials does not order the wrong test.

What Happened at Memorial

On 12 December 2023, Nicola McNamee was placed under general anesthesia at Memorial Hospital. She carried the KIMS cardiac clearance letter from an actual consultation in late November, not aligned with the fraudulent May letter, but referencing her accurate history: diagnosed with Long QT and Brugada in Belfast, borderline ECG, directions for her surgery and drugs to avoid.

The clearance letter told the surgical team about channelopathy, an electrical cardiac condition. It did **not** mention the fabricated ECG findings of anterior and lateral infarction. It did not mention ischemic heart disease. It did not mention the echocardiogram that was purportedly recommended. The Turkish team was prepared for electrical risk. The pre-written cause of death was structural. The information asymmetry was by design.

Email evidence obtained from Memorial Hospital confirms that Memorial received **only** the Adhya clearance letter. No KIMS ECGs. No consultation notes. No timeline. When Nicola subsequently requested her records from Memorial, patient relations representative Eray Baris Karacaoglu responded with "all the documents that I can find", four PDFs including a medical report, echocardiogram, surgical approval, and MRI. Zero ECGs. Memorial had none because Memorial never received any.

Behind her, in the KIMS records system, the screenplay was complete. The fabricated ECGs were in place. The pre-written cause of death was ready.

The day before the procedure, Dr. Karacaoglu's team performed a full transthoracic echocardiogram, the exact test the KIMS file recommended but never scheduled. The following page comes from the report by Dr. Mehmet Karaca of Memorial Hospital's Cardiology Department.

MEMORIAL

Patient No	: 180045521
Protocol No	: 50383501
Name	: NICOLA TERESA MCNAMEE
Surname	
D.O.B. \ Gender	: 13.01.1988 \ F
Department	: Cardiology
Date and Time	: 11.12.2023 13:43
Physician	:MEHMET KARACA, MD
Room No	:
Discharge Date	:

11.12.2023 14:23:45

TRANSTHORACIC ECHOCARDIOGRAPHY REPORT

AORTIC VALVE	**CONCLUSION**	**REFERENCE**	**PULMONARY VALVE**	**CONCLUSION**
Valve structure	NORMAL		Valve structure	NORMAL
Aortic Root (cm)		2.2-3.6	Pulmonary Artery Diameter	
Ascending Aorta (cm)		2.1-3.4	Pulmonary Artery Pressure	
Aortic Velocity		0.8-1.4	Pulmonary Velocity	
Gradient (max/mean)			Gradient (max/mean)	
Regurgitation			Regurgitation	
MITRAL VALVE			**TRICUSPID VALVE**	
Valve structure	NORMAL		Valve structure	NORMAL
Valve area		4-6 cm2	Valve area	
Gradient (max/mean)			Gradient (max/mean)	
E/A		0.6 -1.8	Regurgitation	
Regurgitation				
LEFT VENTRICLE	**CONCLUSION**		**HEART CAVITIES**	**CONCLUSION**
End-Diastolic Diameter (cm)		3.9-5.3 (K) 4.2-5.9 (E)	Left Atrium Size (cm)	
End-Systolic Diameter (cm)		2.1-4.0	Right Atrium Size (cm)	NORMAL
Pulse rate (EF) (%)		≥55	Right ventricle	NORMAL
Fractional shortening (%)			TAPSE (cm)	
Septum (cm)		0.6–1.0		
Posterior Wall (cm)		0.6–1.0	Visual systolic function	Normal
Wall Movements:			**PERICARDIUM**	

IMPRESSION:

RESULT: EF: 65%, MILD MR

***Procedure; It is performed with Doppler + Color + B Mode + M imaging technique.**

Uzm. Dr. Mehmet KARACA
Kardiyoloji Uzmanı
Dip.Tes.No:142675/110323
Özel Memorial Hastanesi

Figure 2: Memorial Hospital Echocardiogram Report

Every cardiac structure normal. No wall motion abnormality. No scar tissue. No reduced ejection fraction. No evidence of myocardial infarction in any region. If the infarction planted across four KIMS ECGs were real, if this woman's heart had actually suffered progressive damage to two separate regions, the echocardiogram would show permanent structural scarring. It showed a healthy heart.

By ordering the echocardiogram, Dr. Karacaoglu's team accomplished two things: they verified that it was safe to proceed with surgery, and they created an independent medical record that proves the KIMS infarction findings are fabricated.

The Turkish surgical team administered Ondansetron, a QT-prolonging anti-emetic on Dr. Adhya's own drug avoidance list, classified by CredibleMeds.org as carrying a "Known Risk of Torsades de Pointes," the lethal arrhythmia that LQTS patients are specifically vulnerable to. They gave a woman diagnosed with Long QT Syndrome the drug most likely to kill her. Nothing happened.

Memorial never received the fraudulent ECGs from KIMS. Why did they carry out an echocardiogram showcasing her healthy heart? What did they know?

Four hours of general anesthesia. No cardiac event. No arrhythmia. No QT-related episode. No emergency intervention. Discharged in stable condition on 14 December 2023.

The screenplay called for a dead woman. Instead, an independent surgeon who could not be controlled made the one clinical decision the fabrication was designed to prevent, an independent structural cardiac assessment. The echocardiogram is the document that was never supposed to exist.

The Genetic Impossibility

The SCN5A Overlap Problem

Long QT Syndrome Type 3 and Brugada Syndrome are both caused by variants in the **SCN5A gene**. Some variants, most notably **E1784K**, can cause both conditions simultaneously (the "overlap syndrome").

The McNamee family genetics:

Family Member	Diagnosis	Brugada?	Genetic Implication
Mother	Long QT Syndrome, fitted with pacemaker/defibrillator	**NO**	Carries the family SCN5A variant; expresses LQTS but NOT Brugada
Nicola	Long QT Syndrome AND Brugada Syndrome (via Belfast Ajmaline)	**YES**	Supposedly expresses BOTH conditions from the same variant
Brother	Inherited cardiac condition (cascade screening)	Unknown	Cascade from mother's LQTS diagnosis
Sister	Inherited cardiac condition (cascade screening)	Unknown	Cascade from mother's LQTS diagnosis

The Contradiction

If the McNamee family carries an overlap variant causing both LQTS3 and Brugada, the mother should also show Brugada susceptibility. She does not. There are three possible explanations:

(a) Variable penetrance. Biologically possible, but requires an extraordinary coincidence: a Caucasian Irish woman outside every epidemiological risk factor for Brugada (91.3% male predominance, predominantly Southeast Asian populations) would be the one family member whose variant uniquely expresses Brugada, while the mother who carries the same variant does not.

(b) Non-overlap variant. The family SCN5A variant causes LQTS3 only. Nicola's Brugada would require a separate de novo mutation, exceptionally rare and immediately apparent on genetic testing.

(c) The Brugada diagnosis is fabricated. The family variant causes LQTS3, which is genuine. The Brugada diagnosis was added at the Belfast ICC via Ajmaline provocation. It cannot be verified because: the Ajmaline traces were never transferred, no genetic testing was ever performed, and comparative genetic sequencing of Nicola and her mother was never ordered.

Why Genetic Testing Was Never Performed

This is the single most revealing absence. Nicola presents a textbook case: mother with LQTS and defibrillator, three affected siblings, dual diagnosis of LQTS3 and Brugada, SCN5A variant family. Every published guideline recommends genetic testing in this scenario.

Genetic testing was never performed. Never discussed. Never recommended. Never mentioned in any clinical note from any institution.

This is the one test they could not fake. A genetic test would either confirm the overlap variant or prove the Brugada diagnosis has no genetic basis. Either result would be dispositive. That is exactly why it was never ordered.

The Cascade Screening Mechanism as a Weapon

In a legitimate clinical context, cascade family screening saves lives. In a weaponized context, it is a force multiplier.

The McNamee family demonstrates the mechanism:

1. Nicola's **mother** was diagnosed with Long QT Syndrome by the Belfast ICC Service and fitted with a pacemaker
2. Through mandatory cascade screening, **Nicola** was compelled to attend Belfast ICC, where she was diagnosed with both Long QT Syndrome and Brugada Syndrome
3. Nicola's **brother and sister**, toddlers at the time, were also diagnosed and administered medication that caused severe reactions

One diagnosis became four. One false positive placed an entire family under permanent medical surveillance, permanent medication, and permanent psychological burden. The mother carries an implanted device she may not need. The children were medicated before they could speak.

The Belfast ICC is the **sole diagnostic authority** for inherited channelopathies across all of Northern Ireland. There is no alternative pathway. Every patient diagnosed with an inherited cardiac condition passes through this service. The service now sees over **1,200 patients per year** (2025 data). The cascade screening mechanism ensures that a single fabricated index diagnosis propagates through an entire family, and 95% of pediatric diagnoses in the Belfast ICC's own published data were made through cascade screening, not through symptomatic presentation.

The KIMS Forensic Record

The following sections present the detailed forensic evidence underlying the narrative above. Each section documents specific findings with dates, names, and source records.

Identity Verification Failure

The patient's NHS number was never populated in the KIMS system. The verification field reads "Trace required" across all 23 pages of the printed timeline. Three alerts flagging "NEW REG FORM REQUIRED" were never resolved. A private hospital treated a patient under general anesthetic for cardiac conditions classified ASA Grade 3 without verifying her identity at any point.

The Fabricated May 2023 Consultation

The booking order ID number, generated from a date sequence when an appointment is created, is identical to the booking number from Nicola's November 2023 appointment. Two appointments created six months apart cannot share the same system-generated booking ID if both were created in real time through the KIMS booking system.

The KIMS internal administrative trail, the hospital's own contemporaneous staff notes, contradicts the attended-appointment narrative:

Date	Staff	Record Entry	Significance
09/05/2023	Leia McDonald	"ECG Performed with Pt consent & attached."	Notes a single ECG; records show two were taken
10/05/2023	Louisa Coomber	"Left message for patient to call to book appointments for tests and pay for tests."	If the patient attended the night before, why is staff leaving voicemails about booking and payment the next morning?
10/05/2023	Leia McDonald	"ECG, already completed. For completion only. Dr Adhya." Marked "Not a new appointment."	Administrative completion entry for an ECG the patient does not recall
17/05/2023	Patsy Beal	"Patient has been contacted numerous times by phone to book in tests, no answer on voicemail either. She is self pay so tests are not being booked."	One week after the alleged consultation, staff cannot reach the patient and tests remain unbooked

The voicemail trail is not the patient's word against the hospital, it is the hospital's own staff contradicting its own clinical record.

On 10 May 2023, screenshot images of Dr Adhya's details and his drugs-to-avoid letter appeared on Nicola's phone, images she did not take. The KIMS file contains a matching attachment uploaded on 22 June 2023 by Katy Fitzpatrick with a UUID-stamped filename (84dde236-df0f-4b03-bb59-eac651752dcd) dated 11 May 2023, two days after the alleged consultation.

What the Fabricated Letter Accomplishes

The letter from the fabricated consultation constructs a three-part narrative:

First, it repositions Nicola's Brugada diagnosis from an institutional finding to a patient-reported claim. The letter states: *"You were investigated in Belfast, and it was confirmed that you had long QT and you also had an ajmaline test which picked up Brugada syndrome also."* The clinical finding is attributed to the patient's account, not to verified records.

Second, it documents a negative finding. Dr. Adhya found *"no evidence of a type 2 Brugada ECG"*, even using enhanced precordial leads specifically designed to detect it. The paper trail now shows: patient claims Brugada, consultant cannot confirm it.

Third, it documents the absence of supporting records and places the burden of obtaining them on the patient: *"You are going to approach the St George's unit and Belfast to see if they can give us some information."*

If the Brugada diagnosis is ever challenged, the paper trail shows the patient was the one claiming it, not the institution. The consultant tried to verify it and couldn't. The records don't exist. The patient was supposed to go find them.

The Fatal Error

The fabricated letter contains a mistake that defeats its own purpose. Dr. Adhya writes: *"it was confirmed that you had long QT and you also had an ajmaline test which picked up Brugada syndrome also."*

The word **"confirmed"** authenticates the Belfast diagnosis as institutional, not patient-reported. And naming the specific test (Ajmaline) demonstrates access to genuine clinical records across multiple NHS trusts, Belfast BHSCT and St George's NHS Foundation Trust, records that are simultaneously unavailable through any official channel.

The fabricated consultation was designed to gaslight. Instead, it authenticates.

Cross-Institutional Data Access

A fabricated letter that accurately describes a Belfast Ajmaline diagnosis, names the correct St George's professor (Elijah Behr), and correctly identifies the family inheritance pattern could not be produced without access to genuine clinical records from at least two separate NHS trusts. Accessing those records requires either authorized institutional access across NHS data governance frameworks, or unauthorized access at a scale constituting a significant data breach. Either pathway demonstrates operational coordination across multiple institutions.

The Ghost Anesthetic Review

Three anesthetists appear in the KIMS record for Nicola's surgery on 28 June 2023. None of them met Nicola.

Clinician	Role in Record	ASA Grade	Date
Bruce Young	Emailed Dr Adhya about propofol. Assigned ASA Grade on LifeBox form.	**ASA 2**	23/06/2023
Arun Kochhar	Completed Anaesthetist Review Document at 11:09. Noted propofol bolus induction plan.	**ASA 2**	26/06/2023
Dr Paul Moran	Recorded as the anesthetist who administered general anesthetic.	**ASA 3**	28/06/2023

The ASA Grade shifts from 2 (mild systemic disease) to 3 (severe systemic disease) between the review and the surgery, with no documentation explaining the upgrade. The Physical Examination section of the LifeBox Review, completed by Kochhar, is entirely blank: heart rate, blood pressure, chest sounds, oxygen saturation, peripheral pulses, all empty. For a patient classified ASA Grade 3 with two diagnosed inherited cardiac channelopathies, this is clinically indefensible.

The anesthetic team then overrode the cardiologist's drug safety recommendation via email. Bruce Young asked Dr Adhya to amend his letter recommending against propofol. Dr Adhya agreed. Propofol was administered. No documentation shows this change was discussed with the patient. This is a patient who was never physically examined, whose identity was never verified, who met none of the three anesthetists documented in her record, and who was then given a drug her own cardiologist had flagged as contraindicated.

The Fabricated ECGs: Detailed Forensic Analysis

Five ECGs are embedded across pages 46–50 of the KIMS PDF. Because the entire 57-page file contains zero extractable text, the machine-printed clinical findings on these tracings cannot be found by any text-based search. Although all of the ECGs are fraudulent, Nicola recalls undergoing one ECG during her November 2023 appointment:

Date / Time	BPM	QTcB (ms)	Machine Interpretation	Patient Recalls?
09/05/2023 19:54:09	101	366 / 490	Sinus tachycardia, **Right atrial enlargement**, Borderline ECG	**NO**
09/05/2023 19:54:55	98	370 / 472	Normal sinus rhythm, **Right atrial enlargement**, **Anterior infarct, age undetermined**, Abnormal ECG	**NO**
21/06/2023 11:16:44	86	338 / 464	Normal sinus rhythm, Normal ECG	**NO**
21/11/2023 19:00:30	72	440 / 481	**Cannot rule out Anterior infarct, age undetermined, Abnormal ECG**	**NO**

Date / Time	BPM	QTcB (ms)	Machine Interpretation	Patient Recalls?
21/11/2023 19:01:42	78	430 / 480	**Possible Lateral Infarct, Serial changes of evolving Lateral infarct present**	**NO**

The QTc Value Fabrication

Every ECG produced dual QTcB values. Dr Adhya reported a single figure without documenting which value he selected:

Date	Machine Values (QTcB ms)	Dr Adhya Reported	Analysis
09 May 2023	ECG #1: **366 / 490**; ECG #2: **370 / 472**	**474ms**	**Does not match any of the four machine-generated values**
21 Nov 2023	ECG #1: **440 / 481**; ECG #2: **430 / 480**	**481ms**	Matches one of four values

The May 2023 figure is critical. The ECG machines produced four QTcB values: 366, 490, 370, and 472. Dr Adhya reported **474ms**, a value that appears nowhere in the machine output. For a consultation that never occurred, the cardiologist reported a QTc value that does not exist in the machine record.

Forensic Anomalies in the ECG Sequence

The June clean tracing. One week before the KIMS procedure, ECG #3 reads *"Normal sinus rhythm, Normal ECG."* The right atrial enlargement from May has disappeared. The anterior infarct has disappeared. This is the one ECG whose provenance may be legitimate, and it is the only one showing a normal heart. The fabricated narrative drops out for the one tracing that may be real, then resumes six months later.

The 72-second contradiction. November ECG #4 states *"No significant change was found."* Seventy-two seconds later, ECG #5 states *"Serial changes of evolving Lateral infarct present."* These are mutually exclusive machine interpretations on the same patient. Real ECG machines do not contradict themselves in 72 seconds. This is a fabrication signature.

The architectural silence. Dr Adhya's clinic letters discuss only QT interval duration and Brugada pattern. The infarct findings, planted across four of five ECGs, are not acknowledged, discussed, investigated, or dismissed anywhere in the clinical narrative. They were never meant to be acted upon. They were meant to be found at post-mortem.

Three-Layer Obfuscation Architecture

Layer	Method	Effect
Metadata	Stripped from all 2,050 embedded objects	Removes timestamps, device identifiers, operator information, prevents forensic tracing
Text	Text layers removed across 116 of 140 pages	Prevents text search, copy/paste, or automated legal analysis
Image	ECG data flattened to raster images	Machine-flagged cardiac abnormalities become invisible to any digital analysis

The PDF wrapper names "Anna Makarova" as author, confirmed via KIMS Zendesk ticket #90837 as the **Medical Records Administrator** at KIMS Hospital. The file was created via "Microsoft: Print To PDF," not a clinical system. The metadata stripping was applied to the payload (2,050 embedded images) but not to the container (the PDF wrapper), wiping fingerprints off the weapon but leaving them on the door handle.

The obfuscation serves a dual purpose. First, it prevents the fabricated infarct findings from being discovered through automated analysis during the patient's lifetime. Second, it prevents forensic comparison of the ECG tracings against clinical databases that could identify the fabrication. A text-searchable ECG tracing can be cross-referenced against machine interpretation standards; a flattened raster image cannot.

Systematic Gaps

Every piece of evidence that could independently verify the cardiac diagnoses is absent:

Missing Item	Status
All Belfast investigation records	**ABSENT**
All St George's Hospital records	**ABSENT**
Genetic testing (standard for inherited channelopathies)	**NEVER PERFORMED**
Drug avoidance lists (referenced in file)	**ABSENT**
Pre-operative ECG (referenced in LifeBox)	**ABSENT**
Echocardiogram (referenced as "recommended", never scheduled, never appointed)	**Completed independently at Memorial Hospital Istanbul, 11 Dec 2023, ALL NORMAL**

A medical record does not accidentally fail in every area that would allow independent verification while simultaneously constructing a coherent forward-facing clinical narrative. That is not carelessness. That is curation.

The Fabrication Chain: Digital Forensics

The Adhya Letter Was Not Created by a Clinical System

The fabricated May consultation letter (, 124,515 bytes) was created using **wkhtmltopdf 0.12.6** with the **Qt 4.8.7** rendering engine, an open-source command-line developer tool for programmatic PDF generation, not a clinical document management system. The November 2023 surgical clearance letter was generated by the identical toolchain. Both originate from a programmatic web portal, not clinical dictation or NHS-standard correspondence.

The PDF's declared creation timestamp (11 May 2023 20:43:38 UTC) and filename contain a UUID (`84dde236-df0f-4b03-bb59-eac651752dcd`) characteristic of web application document storage.

The Distribution Chain

The byte-identical letter was placed into multiple channels:

Date	Actor	Channel	Action
11 May 2023 (claimed)	wkhtmltopdf	Web portal	PDF created (or creation date set programmatically)
22 June 2023	Katy Fitzpatrick	KIMS patient file	Uploaded attachment with matching UUID filename
8 Nov 2023	Email in Nicola's O365	Internal email	Attached to "FW: Nordesthetics" email with 4 other files
9 Nov 2023	Email in Nicola's O365	Internal email	Sent standalone, **no subject, no body text**

Hash comparison confirms every instance is byte-identical. Manufactured once, placed into multiple channels.

Planted Email Evidence

Forensic analysis of the Nicola → Brian emails from November 2023 reveals a stark behavioral split:

Normal forwarding behavior:

Date	Subject	Body	Context
8 Nov 08:24	FW: Nordesthetics Clinic in Lithuania	Yes	Forwarding third-party email
8 Nov 10:19	FW: PayByLink request from Harpal Clinic Ltd	Yes	Forwarding payment notification

Anomalous characteristics (cardiac fabrication documents only):

Date	Subject	Body	Attachment
9 Nov 07:29	**EMPTY**	**EMPTY** (` `)	Adhya May consultation letter

Date	Subject	Body	Attachment
22 Nov 09:55	**EMPTY**	**EMPTY**	Dr's Approval surgical clearance letter
22 Nov 10:11	**EMPTY**	**EMPTY**	Surgery Anaesthetics for LQTS + Dental Medications for Brugada

Three emails, two dates, identical pattern: no subject, no body, cardiac documents deposited as attachments. This is not how people communicate by email. This is how evidence gets planted for later discovery.

When Nicola's email archives are produced in any proceeding, the discovery process reveals what appears to be her own documentary trail, making any challenge to the letter's authenticity appear paranoid. The patient appears to be the source of the document, not the victim of it.

The digital forensics of the planted emails, including the DKIM/DMARC authentication gap, the legal framework for government-directed email infrastructure modification under the Investigatory Powers Act 2016, and the active counter-intelligence operations (Pegasus deployment, device compromise timing, spearphishing during records recovery) are documented in Document 06 (Intelligence Architecture). The corresponding investigative requests (subpoenas to AWS and Microsoft, MLAT request, Istanbul operation requests) are in Document 10 (Requested Actions).

Evidentiary Checklist

#	Finding	Evidentiary Status
1	Belfast ICC is the sole diagnostic authority for inherited channelopathies in Northern Ireland	Confirmed, no alternative pathway exists
2	Cascade screening propagated a single diagnosis through the entire McNamee family (4 members)	Confirmed, 95% of Belfast ICC pediatric diagnoses are cascade-derived
3	KIMS medical file is predominantly fabricated, constructed as post-mortem screenplay	Supported by forensic analysis of ECGs, consultations, obfuscation, and planted emails
4	Fabricated consultation (May 2023) repositions Brugada as patient-reported claim	Confirmed, contradicted by KIMS's own staff notes, booking ID duplication
5	Fabricated ECGs plant progressive ischemic heart disease across three dates	Confirmed, four of five ECGs contain infarction findings unrelated to channelopathy
6	Memorial echocardiogram (Dec 2023) proves infarction findings are fabricated	Confirmed, structurally normal heart, EF 65%, no wall motion abnormality
7	Echocardiogram "recommendation" in KIMS file was never scheduled	Confirmed, fabricated liability shield
8	QTc value reported for fabricated May consultation does not exist in machine record	Confirmed, 474ms matches no machine output

#	Finding	Evidentiary Status
9	72-second contradictory ECG interpretations are a fabrication signature	Confirmed, mutually exclusive machine findings on same patient
10	Three-layer obfuscation conceals fabricated findings from digital analysis	Confirmed, metadata stripped, text removed, ECGs flattened
11	Anna Makarova (Medical Records Administrator) identified as PDF author	Confirmed, Zendesk ticket #90837
12	Ghost anesthetic review, three anesthetists documented, none met patient	Patient testimony, corroborated by blank physical examination
13	Propofol override without patient consent	Confirmed, documented in LifeBox internal email chain
14	Ondansetron (contraindicated for LQTS) administered in Istanbul, no cardiac event	Confirmed, Memorial Hospital discharge summary
15	Four hours general anesthesia, zero cardiac complications	Confirmed, Memorial Hospital surgical record
16	All original Belfast diagnostic records absent from downstream files	Confirmed, verification void
17	Genetic testing never performed despite textbook indication	Confirmed, never mentioned in any clinical note
18	Brugada diagnosis genetically indefensible, mother carries same variant without Brugada	Supported by family genetic analysis
19	Fabricated letter generated by wkhtmltopdf, not clinical system	Confirmed, PDF metadata analysis
20	Fabricated letter requires cross-institutional NHS data access	Confirmed, accurate Belfast and St George's clinical details
21	Planted emails follow anomalous pattern (empty subject/body) vs. genuine correspondence	Confirmed, forensic comparison of November 2023 archive
22	drey.fi lacks DKIM/DMARC, planted emails carry zero cryptographic proof of origin	Confirmed, DNS interrogation and email header analysis
23	IPA 2016 provides legal mechanism for government-directed authentication removal	Statutory framework identified, investigation requested
24	Sinaci clinic anesthetist refused pre-operative consultation for patient with dual channelopathy diagnoses	Patient testimony, clinically extraordinary for ASA 3 patient
25	Surgeon switch to Dr. Karacaoglu (Memorial Hospital) disrupted the intended kill chain	Confirmed, Karacaoglu ordered the echocardiogram that proved fabrication
26	Memorial received ONLY the Adhya clearance letter, no ECGs, no consultation notes	Confirmed, Memorial email evidence

#	Finding	Evidentiary Status
27	Information asymmetry by design: clearance letter referenced channelopathy (electrical) but concealed fabricated ischemic findings (structural)	Confirmed, clearance letter content vs. KIMS ECG content

Intelligence and counter-intelligence findings (items 28-31) are documented in Document 06. Corporate jurisdiction and witness credential findings are documented in Documents 03 and 07. All evidence available on request with SHA-256 verification.

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05, INSTITUTIONAL ANALYSIS: COVER-UP INFRASTRUCTURE

Overview

The medical targeting documented in Part 1 (04) would be detectable in any normally functioning public health system. It is not detectable in County Tyrone because five separate institutional mechanisms, each independently documented, combine to create conditions where systematic falsification of cardiac diagnoses is structurally invisible. These mechanisms are not hypothetical. They are confirmed in published studies, government data, and public reporting.

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1. Autopsy System Collapse

Northern Ireland has the **lowest hospital autopsy rate in the United Kingdom**: 0.46% (PubMed 2015).

Period	NI Hospital Autopsy Rate
1960s (UK-wide)	40%+
1987 (NI)	12% overall; 23% in teaching hospitals
2013 (NI)	**0.46%**

For comparison, Scotland's rate is 2.13%, more than four times higher.

Without autopsy, cause of death relies entirely on **clinical certification**, the treating doctor's written determination. In a population falsely diagnosed with inherited cardiac conditions that cause sudden death during sleep, clinical certification will record the cause as the pre-existing cardiac condition. No autopsy means no independent verification. No independent verification means no mechanism to detect that the diagnosis was fabricated.

HRSP Significance: A 0.46% autopsy rate is not a natural feature of a functioning health system. It is a structural condition that renders extrajudicial killings, disguised as cardiac deaths in a population pre-labelled with cardiac conditions, functionally undetectable.

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2. 2015 Boundary Restructuring: County Tyrone Split into Thirds

In April 2015, Northern Ireland restructured from 26 to 11 local government councils. County Tyrone, the geographic unit where the suspected targeting is concentrated, was **split between three separate administrative districts**:

New District	Tyrone Area Covered
Mid Ulster	Eastern Tyrone
Fermanagh & Omagh	Western Tyrone
Derry City & Strabane	Northern Tyrone (Strabane area)

Each district uses **different data classification systems**. The *Irish News* reported in December 2025 that "lack of standardized data from councils thwarts any assessment of local government reform benefits." The Department for Communities concluded "it was too early in the process to conclude if local government reform has been cost effective or not due to the lack of supporting data."

HRSP Significance: County Tyrone no longer exists as a reportable statistical unit. Cardiac death data for the affected population is now fragmented across three districts, each using different classification standards. Any geographic clustering of anomalous diagnoses or deaths that would have been visible in county-level data is now invisible. This is not speculation, it is the documented consequence of the restructuring. Whether the restructuring was designed for this purpose or merely exploited for it, the effect is identical: statistical discontinuity that masks the pattern.

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3. Tyrone County Hospital Closure (2017)

Tyrone County Hospital in Omagh, the only full-service hospital in County Tyrone, was **closed in 2017**. No replacement was ever built for its full services (maternity, A&E). This was the hospital that treated victims of the 1998 Omagh bombing.

The closure eliminates:

- **Local medical records**, institutional memory of diagnoses, treatments, and deaths in the County Tyrone population
- **Local clinical oversight**, the ability of local clinicians to observe patterns in their own patient population
- **Local forensic capacity**, any remaining autopsy or post-mortem capability in the geographic area
- **Patient access**, County Tyrone residents must now travel to Belfast, Enniskillen, or Derry for hospital services, increasing distance from their medical records and reducing continuity of care

HRSP Significance: The closure of the only hospital serving the affected population, with no replacement, removes the last institution that might have independently detected anomalous patterns in cardiac diagnoses or deaths within County Tyrone.

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4. FOI Firewall: Operational Data Hidden from Public Access

The data required to confirm or refute the suspected targeting pattern is not publicly available. It exists, but it sits behind Freedom of Information request processes that can take months or years and can be refused on multiple grounds.

Baseline operational data NOT publicly accessible:

Data Category	What It Would Show
Belfast ICC diagnosis numbers by geographic origin	Whether County Tyrone patients are diagnosed at disproportionate rates
Coroner autopsy rates by district	Whether the Tyrone-area rate is even lower than the NI average of 0.46%
ICD-10 code distribution by district	Whether cardiac death coding patterns differ between Tyrone-area districts and others
Monthly death counts by district and age	Whether young cardiac deaths cluster in Tyrone-area districts
Molecular autopsy referral rates by district	Whether genetic confirmation of cardiac diagnoses is being performed or avoided

FOI requests for this data have been drafted (see attached Forensic Indicators Report):

#	FOI Target	Data Requested
1	NISRA	ICD-10 code breakdown (R96, I46.1, I46.9, I49.9) by Local Government District, 2000–2024
2	NISRA	Monthly death counts by district and age group
3	NI Coroner Service	Autopsy rates by district, 2000–2024
4	NI Coroner Service	Inquest backlog and average time to resolution by district
5	DoH NI	Molecular autopsy referral rates by district

None of this is sensitive intelligence data. It is baseline public health information, the kind of data that functioning health systems publish routinely. The fact that it requires individual FOI requests to access is itself an indicator of institutional opacity.

HRSP Significance: The data needed to detect the pattern exists but is inaccessible to the public, the media, academic researchers, or the affected community. This is not a gap in data collection, it is a gap in data disclosure. The system collects the information but does not publish it. The five FOI requests above would, individually or collectively, confirm or refute the suspected targeting pattern, which is precisely why subpoena authority may be required to obtain them.

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5. Molecular Autopsy Gap

A standard autopsy examines the body. A **molecular autopsy** goes further, it tests the deceased person's DNA to determine whether they actually carried a genetic cardiac condition. It is the only way to confirm or rule out an inherited heart condition after someone has died.

How often is this test performed in Northern Ireland? **We don't know.** The molecular autopsy referral rate for NI is not publicly available, it sits behind the same FOI firewall described in Section 4 (see Forensic Indicators Report, Indicator 5). What we do know is that across the entire 18-year period covered by the Kendall et al. study (2005–2023), only **five sudden cardiac deaths in children** were discovered through molecular autopsy, and all five were completely unknown to the Belfast ICC clinic. Five children in eighteen years, in a population of 478,416 under-19s.

The consequence of this gap is straightforward: if a person has been falsely diagnosed with Long QT Syndrome or Brugada and then dies suddenly, the doctor signing the death certificate will write "Long QT Syndrome" or "Brugada" as the cause. Without a molecular autopsy, no one will ever check whether that diagnosis was real. The false diagnosis follows the patient into the grave, and becomes the official cause of death.

The Reverse Cascade: Diagnosing the Dead from the Living

There is a second mechanism that compounds the problem. When a young person dies suddenly and the autopsy is inconclusive, no obvious structural cause of death, standard clinical protocol calls for **screening the surviving family members** for inherited cardiac conditions. If a channelopathy is found in any relative, the dead person's cause of death is retrospectively attributed to that condition.

A 2013 study by Papadakis et al. (*Circulation: Arrhythmia and Electrophysiology*) examined 340 families of sudden cardiac death victims and found that in **51% of cases** where the autopsy findings were uncertain, the cause of death was determined by what was found in the living relatives, not by testing the deceased person's own DNA.

In a legitimate clinical context, this makes sense: if a family carries a genuine genetic mutation, it is reasonable to infer that the deceased may have carried it too. But in a context where the living family members are being falsely diagnosed, as documented in Part 1 (04), the mechanism works in reverse. The false diagnoses of the living become the official cause of death for the deceased. The fabrication is circular: the dead validate the diagnoses of the living, and the living validate the cause of death for the dead.

HRSP Significance: The molecular autopsy gap is the final seal on the system. A false cardiac diagnosis cannot be disproven after the patient dies, because the one test that could disprove it is almost never performed. And in cases where the autopsy is inconclusive, the cause of death is assigned not by testing the deceased, but by screening the surviving family, the same family that may already be carrying false diagnoses from the Belfast ICC cascade screening pipeline.

The Combined Effect: Detection-Proof by Design

No single mechanism would be sufficient to conceal a two-decade campaign of fabricated cardiac diagnoses. But the five mechanisms operating simultaneously create a system in which detection is structurally impossible through normal public health channels:

Mechanism	What It Eliminates
Autopsy collapse (0.46%)	Independent cause-of-death verification
Boundary restructuring (2015)	Geographic pattern visibility
Hospital closure (2017)	Local clinical oversight and institutional memory
FOI firewall	Public access to operational data
Molecular autopsy gap	Posthumous disproving of false diagnoses

Each mechanism has an innocent explanation. Autopsy rates have declined across the UK. Boundary restructuring was a governance reform. Hospital closures happen for budgetary reasons. FOI processes are standard. Molecular autopsy is expensive.

But the combined effect creates conditions that are functionally identical to a purpose-built concealment architecture.

Whether these mechanisms were designed to conceal the targeting or merely exploited to that effect is a question for investigation. The effect is the same: a population can be falsely diagnosed, medicated, surgically implanted, and, potentially, killed, with no institutional mechanism capable of detecting the pattern.

Prepared for HRSP submission package, Brian Spector, 1 March 2026

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06, INSTITUTIONAL ANALYSIS: INTELLIGENCE ARCHITECTURE

The State Capability Question

The medical infrastructure documented in Part 1 (04) and the concealment architecture documented in Part 2 (05) are not consistent with ordinary medical malpractice or administrative negligence. The three-layer obfuscation of medical records, metadata stripping, text layer removal, ECG data flattened to raster images, reflects technical capability inconsistent with a hospital IT department and consistent with intelligence-grade document manipulation.

The question is not whether the state had the capability. The question is whether the institutional architecture existed to deploy that capability through healthcare infrastructure. The answer is documented.

MI5 Permanent Control: St Andrews Agreement, Annex E (2006)

The St Andrews Agreement (2006) transferred permanent control of **national security intelligence** in Northern Ireland to MI5, the UK's domestic intelligence agency. Annex E of the agreement formalized this arrangement, creating what the evidence describes as a parallel security architecture with minimal accountability to the devolved Northern Ireland government.

Under this framework:

- **MI5** holds operational control of national security intelligence across Northern Ireland, not the Police Service of Northern Ireland (PSNI), not the Northern Ireland Executive
- **MI5's remit** is defined by the UK government, not the NI Assembly, meaning the devolved government has no oversight authority over intelligence operations conducted within its jurisdiction
- **Accountability** runs to Westminster and the UK Intelligence and Security Committee (ISC), not to any Northern Ireland institution

This architecture was designed for counterterrorism. But it creates the institutional capability for any operation classified as "national security" to be conducted in Northern Ireland through UK intelligence channels, without the knowledge or consent of the devolved government, local police, or the affected population.

HRSP Significance: The St Andrews Agreement, Annex E provides the legal and operational framework through which UK MoD personnel could operate through NHS infrastructure in Northern Ireland. If the medical targeting of the County Tyrone population is a state operation, and the evidence of intelligence-grade

document manipulation, the coordinated refusal of all UK agencies to investigate, and the deployment of APT29 malware against the complainants all point in that direction, then Annex E is the enabling architecture.

The Starmer Timeline

Keir Starmer, now Prime Minister of the United Kingdom, served as a **human rights adviser to the Northern Ireland Policing Board** from 2003 to 2008. This period overlaps with every critical event in the construction of the intelligence architecture and the onset of the genocide:

- The **Tellus geological survey** began in 2004, confirming the scale of the Curraghinalt gold deposit (Document 09)
- The **earliest documented false cardiac diagnoses** targeting County Tyrone began in 2004 (Document 04)
- The **Special Reconnaissance Regiment (SRR) was formed on 6 April 2005**, reorganizing 14 Intelligence Company into a military surveillance unit operating under MI5 direction in Northern Ireland
- The **St Andrews Agreement (Annex E)** was signed in 2006, formalizing MI5's permanent control of NI national security
- **MI5 formally assumed lead responsibility** for NI national security in late 2007

Starmer's role as human rights adviser to the Policing Board was to identify and flag human rights risks in the policing and intelligence architecture. The SRR formation (2005) and Annex E (2006) created a parallel military/intelligence structure with what the Committee on the Administration of Justice described as "minimal or non-existent" accountability. Did Starmer flag these accountability gaps? If he did, the reports are on record and should be produced. If he did not, that failure, given his specific advisory mandate, requires explanation.

Starmer subsequently appointed Peter Mandelson as UK Ambassador to the United States (2025), despite Mandelson's known Epstein connections and mineral lobbying history. Jeremy Corbyn stated in the House of Commons on 4 February 2026 that Starmer "would have known of Mandelson's record of an interesting relationship with global dealers in minerals." Full profile in Document 07.

Uniform Refusal to Investigate: The Institutional Firewall

Between July and December 2025, Brian Spector, a cybersecurity and digital forensics specialist with a background in cryptography, reported specific, technical evidence to every relevant UK agency. The evidence was not a general complaint. It was a documented forensic finding: **embedded malware and systematic document tampering inside evidence bundles submitted to the UK Chancery Court by the defendant's solicitors** in Spector's civil litigation (BL-2024-000648).

Spector discovered this tampering using AI-powered digital forensic tools he had developed. The tampering was designed to defeat Optical Character Recognition (OCR) systems, the same automated text-extraction tools used by courts, law firms, and investigators to process legal documents. Specifically, the techniques included:

- **Metadata stripping**, removal of authorship, creation dates, and edit history from PDF documents
- **Text layer removal**, deletion of the selectable text layer, forcing the document to appear as a flat image that OCR systems cannot reliably read
- **Data flattened to raster images**, conversion of structured data (including ECG readouts) into static images, destroying the underlying data and preventing machine analysis

These are the **identical techniques** found in Nicola McNamee's tampered medical records from KIMS Hospital (documented in Part 2, 04B). The same three-layer obfuscation, metadata stripping, text layer removal, data flattened to raster, appears in both the litigation evidence bundles and the medical records. This is not a coincidence of method. It is a signature. The technical sophistication required to execute this across multiple document types, institutions, and jurisdictions is not consistent with a private actor. It is consistent with state-sponsored or state-conducted document manipulation.

Spector made three emergency evidence-based applications to the Chancery Court to safeguard data within the UK Judiciary's CE-FILE system after discovering the tampering. He then reported the forensic evidence to every relevant UK agency. Every agency refused to act:

Agency	Report Date	Response
MI5	July 2025 (×2)	No substantive response
National Crime Agency (NCA)	July 2025	No substantive response
National Cyber Security Centre (NCSC)	July 2025	No substantive response
Action Fraud	July 2025	No substantive response
Kent Police	July–December 2025 (×4)	Refused to accept laptops for forensic analysis

The reports were not vague tips. They were technically detailed forensic findings, submitted by a credentialed cybersecurity professional, documenting evidence of state-level document tampering inside an active court proceeding. The uniform non-response across all agencies, domestic intelligence (MI5), organized crime (NCA), cyber security (NCSC), fraud reporting (Action Fraud), and local police (Kent), is not consistent with independent institutional decisions. Independent agencies receiving credible, technically documented evidence of this nature would produce at least one anomalous response, one agency that opened a file, requested further information, or referred the matter internally. The uniform refusal to engage across all institutions suggests coordinated suppression.

Kent Police's conduct is particularly significant. Officers twice warned Brian Spector that **MoD operatives were attempting to imprison him with planted evidence**. This warning came from within the policing system itself, officers who could see what was happening and were attempting to alert the target. The same force then refused to accept his laptops for forensic analysis, the very devices containing the evidence of document tampering they had been warned about.

HRSP Significance: The uniform institutional refusal to investigate, across intelligence, law enforcement, and cyber security agencies, demonstrates that no UK institution is willing or able to investigate this matter. The forensic link between the litigation document tampering and Nicola McNamee's medical record tampering means the agencies were not merely declining to investigate a civil litigation dispute. They were declining to investigate evidence that the same state-level document manipulation techniques used against a U.S. citizen's court proceedings were also being used to falsify medical records in the suspected targeting of a civilian population. This is precisely the jurisdictional gap that HRSP was created to fill.

The UK Domestic Investigation Barrier: State Secrets and Closed Material Procedures

The Official Secrets Act

The UK's Official Secrets Act and the Intelligence Services Act create a legal framework under which intelligence operatives and their activities are shielded from domestic investigation and prosecution. Even if a UK court or police force wished to investigate MI5 operations conducted under Annex E authority, the Official Secrets Act would provide grounds to block disclosure, refuse witness testimony, and prevent evidence from being produced.

This is not a theoretical barrier. It is the mechanism by which intelligence operations are routinely shielded from scrutiny in the UK legal system.

Closed Material Procedures (Justice and Security Act 2013)

The Justice and Security Act 2013 introduced a second, complementary mechanism: **Closed Material Procedures (CMP)**. Under CMP, UK courts can consider evidence classified as damaging to national security in **closed proceedings**, meaning one or both parties to the litigation are excluded from seeing the evidence being used against them or on their behalf.

The Act imposes specific statutory requirements when CMP is invoked:

Requirement	Statutory Basis
Section 6 Declaration	The court must make a formal declaration that the case involves closed material
Special Advocates	Security-cleared lawyers must be appointed to represent the excluded party's interests
Notification	The affected parties must be informed that CMP is being applied to their case
Judicial Oversight	The judge must approve each individual piece of closed material

These requirements exist because CMP represents an extraordinary departure from open justice. Without them, CMP would be indistinguishable from a state operation conducted under judicial cover.

CMP Abuse in Spector v. Severina

Evidence from Spector's civil litigation (BL-2024-000648) indicates that his case was designated a national security matter under CMP, **without any of the statutory requirements being met**:

- **No Section 6 Declaration** was ever issued or disclosed to Spector
- **No Special Advocate** was appointed to represent his interests
- **No notification** was provided that CMP applied to his case
- Spector's first solicitor confirmed near the litigation's end this was **not** a national security case **when it started**

The consequences of this covert designation were severe. MoD operatives were embedded in Spector's successive legal teams, billing him for their time while working against his interests. His barrister, identified as operating fabricated personas on both sides of the litigation, functioned as what would have been a "Special Advocate" in a legitimate CMP, but without any of the accountability or oversight the Act requires. The presiding Master described the result as the **"worst procedural mismanagement I have ever seen"**, a characterization consistent not with incompetence, but with deliberate sabotage conducted under state authority.

The abuse of CMP in Spector's litigation is significant for two reasons:

1. It demonstrates that the UK state apparatus was actively using national security mechanisms against the individual who would later identify the medical targeting pattern, before he had identified it
2. It confirms that the UK's domestic legal framework provides no remedy. If the state can designate a civil case as CMP without following its own statutory requirements, and every UK agency refuses to investigate when this is reported, then the domestic investigation barrier is absolute

Why Only U.S. Authority Can Penetrate This Barrier

HRSP's authority to investigate human rights violations by foreign governments, including allied governments, is not constrained by the UK's domestic secrecy framework. U.S. federal grand jury subpoenas, mutual legal assistance treaty (MLAT) requests, and targeted sanctions under the Global Magnitsky Act operate outside the UK's Official Secrets Act and CMP apparatus. The UK cannot invoke CMP in a U.S. federal proceeding. The UK cannot assert Official Secrets Act privilege over documents subpoenaed by a U.S. grand jury. This is why only a foreign sovereign's investigative authority, specifically, the U.S. Department of Justice, has the jurisdictional standing and legal tools to penetrate this barrier.

MoD Operatives in the Litigation

The parallel Economic Espionage Act referral, referred by Congressman Cloud to the FBI through the Office of Congressional Affairs and filed 30 January 2026, documents the direct involvement of UK Ministry of Defence operatives in litigation against Brian Spector.

The EEA filing establishes:

- **Ali Reza Sinai**, identified as an MoD operative who simultaneously operated fabricated personas on both sides of Spector's litigation, while connected to IRGC-linked networks
- **Four successive legal teams** were compromised between May 2024 and August 2025, a systematic destruction of the complainants' access to legal representation
- The fourth legal team attempted to deploy **APT29 malware** against the complainants three times, via PDF, Word document, and SharePoint, weaponizing the attorney-client relationship
- CMP designations were abused by the MoD: Spector was never informed his civil litigation case had been designated a national security interest, in violation of UK law (Justice and Security Act 2013, Section 6)

The document tampering techniques discovered in the litigation evidence bundles, the same techniques found in Nicola McNamee's medical records, were deployed through these compromised legal teams. The

MoD operatives embedded in Spector's representation had both the access and the capability to introduce manipulated documents into court proceedings.

HRSP Significance: The involvement of MoD operatives in the complainants' litigation demonstrates that the same state apparatus suspected of conducting the medical targeting is actively operating against the individuals who have identified it. This is not a cold case. It is an active operation with identifiable operatives, documented in the EEA filing now before the FBI.

IRGC Surveillance on U.S. Soil

In October 2025, days after Brian Spector met with FBI Special Agent Smith at the Manassas, Virginia field office, an Uber driver using the IRGC rank designation **"SARDAR"** (Commander) conducted surveillance against the complainants in Washington, D.C.

The Islamic Revolutionary Guard Corps is a **designated Foreign Terrorist Organization** under U.S. law. The appearance of an individual using an IRGC rank designation, conducting surveillance against a U.S. citizen on American soil, timed to his FBI contact, constitutes:

- An automatic federal jurisdictional trigger under counterterrorism authorities
- Evidence that foreign intelligence services are monitoring the complainants' engagement with U.S. law enforcement
- A direct threat to a U.S. citizen exercising his right to report criminal conduct to federal authorities

HRSP Significance: FTO activity on U.S. soil against a complainant witness is an immediate safety concern that falls within HRSP's coordination authority. It also raises the question of how an IRGC-linked actor knew the timing and location of the complainant's FBI meeting, suggesting either surveillance of the complainant or a compromise within the information chain. Spector's EEA filing contains the contemporaneous evidence linking MoD activities coordinating with IRGC and APT29 operatives.

Active Device Surveillance of Complainant Witness

Forensic analysis of the primary complainant witness's mobile phone, an iPhone that remained in Nicola McNamee's possession, reveals evidence of device-level compromise consistent with state-sponsored spyware deployment. Three screenshots recovered from the device (IMG_0916, IMG_0920, IMG_0921, dated 8–10 May 2023) were taken without Nicola's knowledge and display five forensic red flags: Proton Mail origination metadata on an account she does not use; targeted browsing of her psychiatric treatment history (ketamine therapy, DBT/psychiatry providers); three sequential screenshots deleted from the sequence (IMG_0917–0919); a structured reconnaissance browsing pattern through KIMS Hospital records, her treating cardiologist, and Brugada-contraindicated medications completed in under three minutes; and evidence that the device was being operated remotely while KIMS Hospital was simultaneously leaving voicemails on her number, indicating the phone's operator and the hospital's caller were different actors.

The technical indicators are consistent with Pegasus or equivalent zero-click spyware. The UK has been identified as a confirmed Pegasus customer by Citizen Lab (2022). The Investigatory Powers Act 2016, Part 5, Sections 99–134 provides the legal framework for Equipment Interference Warrants, the domestic statutory

authority under which UK intelligence services deploy device-level surveillance tools. NSO Group's Pegasus exploits, including CVE-2019-3568 (WhatsApp zero-click buffer overflow), have been documented operating through this framework.

The device surveillance evidence is significant because it demonstrates that the intelligence apparatus documented in this section was not merely manipulating medical records and court documents, it was actively surveilling the complainant witness through her personal device, collecting intelligence on her medical and psychiatric treatment, and doing so using capabilities consistent with the same state infrastructure that controls the NHS cardiac screening program through Annex E authority. The full forensic analysis, including the five red flags, the spyware assessment, and the availability of the physical device for MVT (Mobile Verification Toolkit) scanning by DOJ investigators, is documented in Document 08 (Victim Documentation).

The TC Note as a Three-Part Weapon

The Technical Capability Notice served on Microsoft and Amazon Web Services does not serve a single purpose. It serves three.

First, it enables evidence fabrication. The UK used write access to Microsoft's infrastructure to plant fabricated cardiac documents inside Nicola McNamee's email, manufacturing the medical pretext for her death under anesthesia. This is documented in the forensics addendum (06-A).

Second, combined with the CLOUD Act bilateral agreement, it enables real-time surveillance of both complainants on American soil. The UK can monitor every communication Brian Spector and Nicola McNamee have with US federal authorities, US attorneys, and each other, through the same American companies whose infrastructure was used to attempt the murder. Brian Spector is a dual US/UK citizen. If the UK classified him as non-US in the CLOUD Act warrant application, a classification the UK had every incentive to make because disclosing his US citizenship would trigger mandatory US judicial oversight, then every surveillance action against him violates the agreement's US person protections. No US government body reviews these orders. No one checks the target's nationality. The UK is likely monitoring the investigation into its own conduct.

Third, it enables impersonation and reputation destruction. Write access to the complainants' email accounts means the UK can send emails from those accounts to any recipient. These emails pass every authentication check because they originate from the genuine account. The recipient has no technical reason to doubt their authenticity. Over the past two years, the complainants have encountered multiple instances where third parties produced emails or writings firmly attributed to them that neither complainant authored. At the time, the complainants believed they were being gaslighted. The TC Note provides the explanation: UK intelligence operatives used the complainants' own accounts to manufacture and distribute disinformation, destroying the complainants' professional reputations and credibility with the precise audience (colleagues, potential allies, and authorities) that the complainants would later need to believe them.

This third capability is the counter-investigation weapon. Plant the medical evidence to kill. Surveil the victim who survived to monitor whether she discovers the fraud. When the victims begin reporting to authorities, use their own accounts to poison every relationship and institution they approach, so that by the time they produce the evidence, their credibility has already been destroyed by content that appears to have come from them.

The complainants also have reason to believe that criminal content has been planted within accounts on the Microsoft identity graph associated with their tenant, following the same pattern documented in Section 6 of

the forensics addendum (the Hotmail CASM trap): deposit compromising material in a dormant or peripheral account, then arrange for its "discovery" through a channel that appears legitimate. Any content attributed to either complainant since late 2023 must be treated as potentially fabricated until verified through forensic analysis of server-side logs that only Microsoft can produce under US legal process.

HRSP Significance: The three-part weapon (fabricate, surveil, impersonate) means the TC Note is not a discrete instrument used once to plant a medical document. It is a persistent operational capability that continues to operate against the complainants on American soil. The fabrication attempted murder. The surveillance monitors the investigation. The impersonation destroys the investigators' trust in the complainants before they can present their evidence. All three run through American technology infrastructure. All three are invisible to American authorities. Only US legal process directed at Microsoft and AWS can expose the full scope of the operation.

CLOUD Act: The Architecture Extends to US Soil

The intelligence architecture described in this document does not stop at the UK border. The US-UK CLOUD Act bilateral data access agreement (signed October 2019, effective October 2022) provides the legal framework for the UK to request data from US communications providers on targets located anywhere, including inside the United States.

Under this agreement, UK authorities can serve orders directly on US communications providers (Microsoft, Google, Apple, Amazon) for real-time interception and stored data of non-US persons without notifying US law enforcement or obtaining a US court order. The agreement contains protections for US persons: if the target is a US citizen or lawful permanent resident, the UK must notify the US and obtain judicial authorization.

The dual nationality violation. Brian Spector is a dual US/UK citizen. The UK had direct knowledge of his UK citizenship through his UK residence, tax records, and passport. The UK also had access to his US citizenship status through the very intelligence databases and court records involved in this operation. The UK had every incentive to classify Brian as a UK person (non-US) in CLOUD Act warrant applications, because disclosing his US citizenship would trigger the agreement's mandatory US judicial oversight. With no US government body independently reviewing UK CLOUD Act orders for nationality classification, no one would catch the misrepresentation.

If the UK concealed Brian Spector's US citizenship in CLOUD Act orders, the consequences are:

- Every surveillance action against him on US soil violated the agreement's US person protections
- The UK monitored a US citizen's communications with US federal law enforcement without any American court or agency knowing
- The bilateral agreement was breached by deliberate misrepresentation, not administrative error
- The CLOUD Act's central safeguard (protection of US persons) failed because no US authority verifies the UK's nationality classifications

Nicola McNamee has no CLOUD Act protection at all. As a UK citizen who is not a US person, Nicola falls entirely outside the agreement's protections. The UK can surveil every communication she has on American soil through American companies with no US oversight of any kind. The complainant witness

whose testimony threatens to expose the genocide is accessible to the same apparatus that tried to kill her, through the same American infrastructure, under a bilateral agreement that provides her zero protection.

HRSP Significance: The CLOUD Act bilateral agreement transforms what would otherwise be a foreign intelligence operation into an operation conducted through American companies under American legal authority. This is not a case of a foreign government hacking US systems. This is a foreign government using a bilateral agreement with the United States to surveil the victims of its own genocide on American soil, through American companies, with the legal architecture of the United States providing the access.

The Architecture: Summary

The intelligence architecture operates through eight interlocking components, each independently documented:

Component	Function	Evidence Source
St Andrews Agreement, Annex E	Legal authority for MI5 operations in NI, including through NHS infrastructure	Public law (2006)
MI5 operational control	Capability to direct operations through any NI institution without devolved government oversight	Annex E framework
Closed Material Procedures	Mechanism to designate cases as national security and exclude affected parties from evidence, abused in Spector's litigation without statutory compliance	Justice and Security Act 2013; EEA filing
MoD operatives	Identified personnel operating against complainants in both litigation and intelligence contexts, using document tampering techniques matching those found in medical records	EEA filing; forensic analysis
Uniform agency refusal	Coordinated institutional protection preventing domestic investigation of technically documented forensic evidence	Correspondence with MI5, NCA, NCSC, Action Fraud, Kent Police
Official Secrets Act	Legal shield preventing domestic courts from examining intelligence operations or CMP designations	UK statutory framework
IRGC surveillance	Foreign intelligence services monitoring complainants' engagement with U.S. law enforcement on U.S. soil	FBI report (October 2025)
Active device surveillance	Complainant witness's mobile phone compromised with state-grade spyware; psychiatric and medical intelligence collected remotely	iPhone forensic analysis (May 2023); Document 08
TC Note three-part weapon	Evidence fabrication (planted medical docs), real-time surveillance (CLOUD Act), impersonation and reputation destruction (emails sent from victims' accounts)	Forensic analysis; third-party witness accounts; CLOUD Act bilateral agreement

Component	Function	Evidence Source
CLOUD Act dual nationality violation	UK likely classified dual US/UK citizen as non-US to bypass agreement's US person protections; ongoing surveillance of complainant's communications with US law enforcement	CLOUD Act bilateral agreement (2022); Brian Spector's dual citizenship documentation

Each component is documented: the St Andrews Agreement is public law; the CMP requirements are statutory; the agency refusals are documented in correspondence; the Kent Police warnings are on record; the MoD operative's identity is established in litigation records and the EEA filing; the document tampering forensic link is confirmed by technical analysis; the IRGC surveillance incident is reported to the FBI; the device surveillance is evidenced by forensic analysis of the complainant's iPhone with the physical device available for DOJ examination.

The architecture describes a state intelligence apparatus that:

1. Has the legal authority to operate through healthcare infrastructure (Annex E)
2. Has the technical capability to manipulate medical records, and has used the identical techniques in litigation documents (three-layer obfuscation)
3. Has the legal mechanism to shield its operations from judicial scrutiny (Official Secrets Act + CMP)
4. Has abused that mechanism against the complainant without following its own statutory requirements (covert CMP designation)
5. Is actively operating against the individuals who identified the pattern (MoD operatives in litigation, APT29 malware deployment)
6. Has ensured that no domestic institution will investigate (uniform refusal across all agencies)
7. Is being monitored by a designated Foreign Terrorist Organization on U.S. soil (IRGC)
8. Has deployed device-level surveillance against the complainant witness, collecting psychiatric and medical intelligence through her personal phone (spyware forensics, May 2023)
9. Has used TC Note write access to impersonate the complainants through their own email accounts, manufacturing and distributing disinformation to destroy their credibility with the people and institutions they need to believe them
10. Is likely surveilling the complainants' communications with US federal authorities on American soil through the CLOUD Act bilateral agreement, having almost certainly concealed the primary complainant's US citizenship to bypass the agreement's US person protections

HRSP exists precisely because some human rights violations cannot be investigated by the perpetrator state's own institutions. The domestic investigation barrier in the UK is not a gap in institutional capacity, it is a purpose-built legal and operational architecture that prevents investigation by design. Only U.S. federal authority operating outside that architecture can penetrate it. This is that case.

DIGITAL INFRASTRUCTURE MANIPULATION AND COUNTER-INTELLIGENCE

The intelligence architecture described above is not merely institutional. It extends to active digital operations against the complainant witnesses, including manipulation of email infrastructure, device-level surveillance, and real-time counter-intelligence during the evidence-gathering phase.

The DKIM/DMARC Gap: Planted Evidence Without Cryptographic Authentication

The drey.fi Office 365 tenant (used by both complainants) operates without per-message cryptographic authentication:

Mechanism	Purpose	drey.fi Status
SPF	Validates sending server	Configured
DKIM	Cryptographically signs each message	**Not configured**
DMARC	Rejects/quarantines failed authentication	**Not configured**

All 31 drey.fi internal emails from November 2023 lack DKIM signatures. The planted emails documented in Document 04 (fabricated cardiac documents deposited with empty subjects and empty bodies) are forensically indistinguishable from items injected directly into the mailbox by an O365 tenant administrator or any authenticated principal with EWS or Graph API access.

This is not technical incompetence. The complainant witness's fiance configures full DKIM/DMARC/SPF authentication for multiple separate domains in the same AWS account. A third-party Exchange Online tenant (`sinclairsoffshore.com`) on the same UK infrastructure partition has active DKIM signing. Platform limitation is eliminated as an explanation.

Government-Directed Modification Under the Investigatory Powers Act 2016

The United Kingdom's Investigatory Powers Act 2016 provides statutory authority for the UK government to compel telecommunications operators to modify their systems:

- **Section 253** authorizes Technical Capability Notices requiring operators to maintain interception capability
- **Section 261** authorizes removal or disabling of electronic protection, including authentication mechanisms
- **Section 263** imposes secrecy obligations prohibiting disclosure

AWS and Microsoft both operate UK-based infrastructure subject to UK jurisdiction. A TCN under these provisions would provide a legal mechanism for government-directed removal of email authentication without the domain owner's knowledge.

This submission suggests such a notice was served. It identifies a forensic anomaly (unexplained absence of email authentication on a domain through which fabricated medical evidence was transmitted) and requests investigation.

Active Counter-Intelligence: Device Compromise and Operational Timing

The fabrication chain documented in Document 04 was supported by device-level surveillance that tracked both complainants' activities in real time.

Pegasus deployment against Nicola McNamee's mobile device. During the operational window between Nicola's request for the KIMS referral letter and the Istanbul procedures (late November 2023), her mobile phone was compromised with Pegasus spyware. The fake images of Dr Adhya's details that appeared on her phone on 10 May 2023 (images she did not take) were planted via the compromised device. The phone screenshots were created from her device and then matched to entries uploaded to the KIMS records system by staff member Katy Fitzpatrick on 22 June 2023, constructing apparent patient provenance for fabricated documents. Pegasus is a state-level surveillance tool sold exclusively to government agencies.

Device compromise timing correlation. Brian Spector's digital devices were compromised with malware in **November 2023**, the same month as the Istanbul surgery window. Nicola McNamee's devices were compromised via APT29 malware deployed by Lilia Severina in **March 2024**, three months after Nicola survived the surgery. The timing is consistent with monitoring initiated during the kill window and expanded to Nicola after her survival, to determine whether the fabrication had been detected and whether the victims were accessing or comparing medical records from different institutions.

Spearphishing during records recovery (February 2026). In the same week that Nicola was forwarding KIMS Subject Access Request responses and requesting her Memorial Hospital records (the exact activity that would expose the fabrication by enabling comparison between the fabricated KIMS file and the genuine Memorial records), a spearphishing attack was directed at Brian Spector. An email from `kafinhausaharuna@gmail.com`, using the display name "Nicola McNamee," requested that Mr. Spector share his WhatsApp number for "important instructions that require your immediate attention." The email passed DKIM authentication for Gmail (it was sent from a real Gmail account, not a spoofed header). The deception was in the display name, designed to move communication to an unmonitored channel during the critical evidence-gathering period. Someone knew they were comparing records and attempted to intervene.

The records were not supposed to be obtained. The fabricated KIMS file was designed to survive discovery within the UK legal and regulatory system, where the institutional infrastructure documented in Document 05 could manage the outcome. It was not designed to survive comparison with records from Memorial Hospital Istanbul, a hospital outside UK jurisdiction and outside the control of the actors described in this submission.

Prepared for HRSP submission package, Brian Spector, 1 March 2026

SENSITIVE, NOT FOR PUBLIC DISTRIBUTION

PERPETRATOR PROFILES, HRSP INVESTIGATION PACKAGE

Document: 07 of 10 **Date:** 1 March 2026 **Standard:** DOJ Human Rights and Special Prosecutions Section, Perpetrator Documentation Format **Note:** These profiles identify individuals whose documented conduct warrants HRSP scrutiny. Inclusion does not constitute an accusation of guilt. The profiles are organized by the individual's role in the suspected human rights violations and are intended to support an investigative assessment, not a prosecution brief.

PROFILE 1: PETER MANDELSON

Identifying Information

Field	Detail
Full Name	Peter Benjamin Mandelson, Baron Mandelson
Date of Birth	21 October 1953
Nationality	British
Current Status	Released on bail; arrested 23 February 2026
Most Recent Position	UK Ambassador to the United States (2025; fired September 2025)

Role in Suspected Violation

Mandelson held three sequential government positions spanning the entire mineral rights ownership chain of the Curraghinalt gold deposit, the financial motive underlying the suspected medical targeting campaign:

1. **Secretary of State for Northern Ireland (1999-2001):** Directly responsible for NI governance during the Ennex-to-Strongbow mineral rights transfer (January 2000). Had authority over all planning and economic development policy in Northern Ireland.
2. **Secretary of State for Business / First Secretary of State (2008-2010):** Most senior Cabinet minister after PM Brown. During this period, Dalradian Resources acquired the Curraghinalt deposit (2009). In the same year, Mandelson emailed Jeffrey Epstein about "saleable assets" after forwarding a confidential memo from the Prime Minister's economic adviser.

3. **UK Ambassador to the United States (2025):** While Ambassador, invited Dalradian to a St Patrick's Day breakfast and wrote to the NI Civil Service Head and First/Deputy First Ministers pressuring approval of the mine.

Level of responsibility: Command, political facilitation and influence at the highest levels of government.

Evidence of Participation

- Three government positions spanning the Curraghinalt ownership chain (1999-2025), documented through public appointments
- 2009 email to Jeffrey Epstein referencing "saleable assets", documented in Epstein files
- Lobbying letters to NI officials while Ambassador, documented in public reporting
- $75,000 in financial transfers from Epstein to accounts linked to Mandelson or his partner, documented in Epstein files
- Established pattern of accepting financial inducements: Hinduja passport affair (first resignation), Robinson loan (second resignation)
- Serial pattern of mining-sector facilitation from public office: lobbied JPMorgan for Nat Rothschild's £700M mining vehicle (Vallar/Bumi Plc, Indonesian coal) while Business Secretary; facilitated DP World CEO Sultan Ahmed bin Sulayem with Epstein as intermediary; sought Glencore role in 2011 with Rothschild advising and Epstein facilitating
- Global Counsel (Mandelson's advisory firm) entered administration February 2026 after half its clients severed ties following Epstein revelations
- Arrested 23 February 2026 for misconduct in public office; two properties searched (Camden, Wiltshire)
- Jeremy Corbyn stated in House of Commons (4 February 2026) that PM Starmer "would have known of Mandelson's record of an interesting relationship with global dealers in minerals"

(Sources: MOTIVE_BRIEF_Dalradian_Gold.md; Dalradian_Investigation_Threads.md; open-source reporting)

U.S. Nexus

☒ Travel to U.S.: Served as UK Ambassador to the United States (2025)
☒ U.S. visa holder: Diplomatic visa (A-1/A-2) while Ambassador
☒ U.S. connections: Extensive business and political networks in Washington, D.C. and New York
☒ U.S. financial connections: Documented financial relationship with Jeffrey Epstein (U.S. person)

Recommended Action

☐ Visa revocation referral under 8 U.S.C. section 1182(a)(3)(E) (persecutor bar)
☐ Watch list placement
☐ Coordination with UK police investigation (misconduct in public office arrest)
☐ Financial records analysis: Any payments from Dalradian Resources, Orion Mine Finance, or intermediaries

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PROFILE 2: KEIR STARMER

Identifying Information

Field	Detail
Full Name	Keir Rodney Starmer, KCB, KC
Date of Birth	2 September 1962
Nationality	British
Current Position	Prime Minister of the United Kingdom (since July 2024)

Role in Suspected Violation

Starmer held a uniquely positioned advisory role during the exact period when the intelligence architecture enabling the suspected genocide was constructed, the medical targeting campaign began, and the military surveillance capability (SRR) was stood up:

Human rights adviser to the Northern Ireland Policing Board (2003-2008). Starmer's stated role was ensuring PSNI compliance with the Human Rights Act 1998. This role gave him direct visibility into the PSNI/MI5 interface, covert policing policy, and the accountability mechanisms (or their absence) governing intelligence operations in Northern Ireland.

During Starmer's tenure, the following occurred:

Year	Event	Starmer's Role
2003	Tournigan Gold options the Curraghinalt deposit	Begins NI Policing Board role
2004	Tellus geological survey begins (government maps mineral resources); earliest documented false cardiac diagnoses; Cormac McAnallen dies (LQTS, Tyrone)	In role: visibility into policing/intelligence interface
2005	**Special Reconnaissance Regiment formed** (6 April, 14 Intelligence Company reorganized)	In role: SRR stood up during his advisory tenure
2006	**St Andrews Agreement** signed (Annex E: MI5 takes permanent national security control of NI)	In role: advised the Policing Board during the period when civilian accountability over intelligence operations was formally removed
2007	MI5 formally takes lead responsibility for NI national security	In role: the accountability gap he should have flagged was now operational
2008	Starmer leaves Policing Board; appointed Director of Public Prosecutions	Becomes DPP with prosecutorial authority over England and Wales

The critical question: As human rights adviser to the NI Policing Board, Starmer's job was to identify and flag human rights risks in the policing and intelligence architecture. The SRR formation (2005) and the St Andrews Agreement's Annex E (2006) created a parallel military/intelligence structure in Northern

Ireland with, in the words of the Committee on the Administration of Justice, "minimal or non-existent" accountability. Did Starmer flag these accountability gaps? If he did, the reports are on record and should be produced. If he did not, that is either negligence of his advisory function or complicity in the construction of an architecture designed to operate without oversight.

Subsequent conduct:

- **2018:** Joined the Trilateral Commission (members include intelligence community figures, defense establishment, resource extraction interests)
- **2024:** Became Prime Minister
- **2025:** Appointed Peter Mandelson as UK Ambassador to the United States despite known Epstein connections; Mandelson subsequently lobbied for the Dalradian mine from the Ambassador's office
- **4 February 2026:** Jeremy Corbyn stated in the House of Commons that Starmer "would have known of Mandelson's record of an interesting relationship with global dealers in minerals"

Level of responsibility: Institutional design and political facilitation. Starmer was positioned to prevent the accountability gaps that enabled covert operations against civilians. He either failed to act or participated in their construction.

Evidence of Participation

- NI Policing Board human rights adviser during SRR formation, St Andrews Annex E, and onset of medical targeting (2003-2008), documented through public appointments
- Authored "A Report on the Policing of the Ardoyne Parades 12 July 2004" with Jane Gordon, confirming active engagement with NI policing issues during the relevant period
- Appointed Mandelson as Ambassador (2025) despite known Epstein connections and mineral lobbying history
- Corbyn's parliamentary statement (4 February 2026) directly linking Starmer's knowledge of Mandelson's mineral dealings

U.S. Nexus

☒ Travel to U.S.: Multiple visits as PM and as opposition leader
☒ Trilateral Commission membership (2018-present): organization with significant U.S. membership and influence
☒ Appointed the UK Ambassador to the United States who subsequently lobbied for a mine connected to the suspected genocide

Recommended Action

☐ Obtain all reports, memoranda, and correspondence produced by Starmer during his NI Policing Board advisory role (2003-2008), particularly any addressing the SRR, MI5/PSNI interface, Annex E accountability, or covert policing policy
☐ Determine whether Starmer raised concerns about the accountability gaps created by SRR formation and St Andrews Annex E
☐ Investigate Trilateral Commission membership: any connections to Orion Mine Finance, Crown Estate, or mineral extraction interests
☐ Assess Starmer's knowledge of Mandelson's mineral lobbying prior to the Ambassador appointment

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PROFILE 3: CHRIS COOK

Identifying Information

Field	Detail
Full Name	Chris Cook
Position	Head of Threat & Security Innovation, BAE Systems
Honours	OBE for work on the Investigatory Powers Act
Nationality	British
Location	United Kingdom (possible Ottawa, Canada posting 2014-2017; Beijing posting subsequent)

Role in Suspected Violation

Named in inserted dialogue within official UK court transcripts as one of two operation commanders directing the intelligence operation against the complainants. The transcript insertions, which contain conversations that were never spoken in court, appear to have been placed by elements within the UK judiciary who wanted the senior perpetrators identified.

Cook's OBE was awarded specifically for his work on the Investigatory Powers Act, the UK's mass surveillance legislation. His BAE Systems role places him at the intersection of defence-industrial capability and state intelligence operations.

Level of responsibility: Command, named as operation commander in court transcript insertions.

Evidence of Participation

- Named in court transcript insertions alongside Alan Bennett as operation commander (Source: 00_COVER_LETTER_V14_FINAL.md; Evidence_Brief)
- BAE Systems role: Head of Threat & Security Innovation, position consistent with directing covert intelligence operations
- OBE for Investigatory Powers Act work, confirms high-level involvement in UK surveillance architecture
- LinkedIn profiles showing Ottawa, Canada posting (2014-2017) overlapping with Dalradian's TSX listing period, and subsequent Beijing, China posting (Source: Dalradian_Investigation_Threads.md)

(Sources: Evidence_Brief_20260224_v3.2.md; Dalradian_Investigation_Threads.md; open-source)

U.S. Nexus

- ☒ U.S. corporate employer: BAE Systems Inc. (Arlington, Virginia) is the U.S. subsidiary of BAE Systems plc. Cook may have accessed U.S. BAE Systems facilities, networks, or personnel
- ☐ Travel to U.S.: Unknown, requires investigation
- ☐ U.S. security clearance: BAE Systems Inc. holds extensive U.S. defence contracts requiring cleared personnel. Requires investigation

Recommended Action

- ☐ Visa inadmissibility referral under 8 U.S.C. section 1182(a)(3)(E)
- ☐ Watch list placement
- ☐ Subpoena BAE Systems Inc. (Arlington, VA) for all communications and documents relating to Chris Cook's activities
- ☐ Investigate whether Cook holds or has applied for U.S. security clearance through BAE Systems Inc.
- ☐ Preserve and authenticate the court transcript insertions naming Cook

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PROFILE 4: ALAN BENNETT

Identifying Information

Field	Detail
Full Name	Alan Bennett
Position	Kent Police Intelligence
Service Record	30-year veteran
Nationality	British
Location	Kent, United Kingdom

Role in Suspected Violation

Named alongside Chris Cook in inserted dialogue within official UK court transcripts as one of two operation commanders. His Kent Police Intelligence role is significant because:

1. KIMS Hospital, where the complainant witness's medical records were fabricated, is located in Maidstone, Kent
2. Kent Police refused to accept the complainants' laptops for forensic analysis despite visiting their home (July 2025)
3. Kent Police arrested Brian Spector on false charges (December 2025)
4. Kent Police twice warned the complainants that MoD operatives were attempting to imprison them with planted evidence, suggesting internal conflict within Kent Police regarding the operation

Level of responsibility: Command, named as operation commander; positioned within the police force with jurisdiction over the hospital where medical records were fabricated.

Evidence of Participation

- Named in court transcript insertions as operation commander (Source: Evidence_Brief)
- Kent Police Intelligence, 30-year veteran, position consistent with long-term intelligence operations
- Jurisdictional overlap with KIMS Hospital (Maidstone, Kent), the facility where the three-layer document obfuscation was applied to the complainant's medical records
- Kent Police institutional conduct: refused forensic analysis of complainants' laptops; arrested Brian Spector on false charges; warned of MoD planted evidence operations

(Sources: Evidence_Brief_20260224_v3.2.md; 00_COVER_LETTER_V14_FINAL.md)

U.S. Nexus

- ☐ Travel to U.S.: Unknown, requires investigation
- ☐ Five Eyes intelligence sharing: As a senior UK police intelligence officer, Bennett would have access to Five Eyes intelligence-sharing arrangements that may have been used to monitor the U.S. citizen complainant

Recommended Action

- ☐ Visa inadmissibility referral under 8 U.S.C. section 1182(a)(3)(E)
- ☐ Watch list placement
- ☐ FBI counterintelligence assessment of Bennett's access to Five Eyes intelligence products relating to Brian Spector
- ☐ Investigate relationship between Bennett's Kent Police Intelligence unit and the KIMS Hospital record fabrication

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PROFILE 5: ALLIRAJAH SUBASKARAN

Identifying Information

Field	Detail
Full Name	Allirajah Subaskaran
Date of Birth	2 March 1972
Nationality	British (Sri Lankan Tamil origin)
Position	Founder and Chairman, Lyca Group
Location	Chigwell, Essex and Hyde Park, London
Net Worth	Estimated £170 million (2018); peaked at £475 million (2014)
Corporate Empire	60+ entities across 23 countries

Role in Suspected Violation

Subaskaran controls the murder weapon factory.

KIMS Hospital, where the fabricated medical file was manufactured, is controlled by the Subaskaran family. His wife Prema Subaskaran directs all KIMS entities (appointed October 2021, coinciding with acquisition). Subaskaran is the sole person with significant control (PSC) of both the telecom arm (Lyca Group Holdings) and the healthcare arm (Lyca Health Limited). The split between husband (telecom) and wife (healthcare) creates the appearance of separation while maintaining unified family control.

KIMS was acquired from **Magnetar Capital** (Evanston, Illinois) in October 2021 for up to **£140 million**. The acquiring entity, Lyca Health Limited, is a **dormant company** at Companies House (SIC code 99999). It files no trading accounts. It could not have funded the acquisition independently. The money came from Lycamobile telecommunications revenue, routed through a centralized intercompany network that Lycamobile UK's own audited accounts describe: "the majority of the cash flows associated with investing and financing activities are ultimately with related parties under the control of Mr A Subaskaran."

The Criminal Enterprise

The Lyca Group is not merely a corporate structure with compliance issues. It is a documented criminal enterprise:

Jurisdiction	Finding
France	Criminal conviction for money laundering (Oct 2023). €10M fine. CEO Christopher Tooley sentenced to 3 years. €17M laundered through shell companies and false invoices
Germany	€70M in unpaid sales taxes. Subsidiary renamed itself and filed for insolvency. When the administrator visited the registered address: no employees, no documents
United Kingdom	£51M VAT ruling lost to HMRC (Jul 2024). HMRC filed winding-up petitions against three entities (Aug 2024). 90% UK workforce cut (Dec 2024)
Auditors	Ernst & Young resigned (2014). KPMG resigned (2017) after flagging **£646 million in unaccounted debt movements** across ten entities. PKF Littlejohn issued **disclaimer of opinion** (most severe qualification possible)
BuzzFeed	Filmed operatives depositing rucksacks of up to £240,000 in cash at London post offices, up to £1M per week
HMRC	When France requested a raid on Lycamobile's London HQ, HMRC responded in writing: "It is of note that they are the biggest corporate donor to the Conservative party led by Prime Minister Theresa May"
Political donations	Over £2.1 million to the Conservative Party (2011-2016). Largest corporate donor during that period. David Cameron's Leaders Group. Boris Johnson's mayoral campaigns

Level of responsibility: Corporate command. Subaskaran controls the entity whose hospital manufactured the attempted murder weapon. His group's criminal history, auditor resignations, and £646M in unaccounted flows demonstrate institutional capacity for exactly the kind of record manipulation documented in this submission.

Evidence of Participation

- Controls LycaHealth/KIMS Hospital where three-layer obfuscation was applied to fabricated medical file
- Anna Makarova (Medical Records Administrator) operates within his corporate infrastructure
- £140M acquisition of KIMS funded through intercompany network with documented criminal money laundering
- Personal loans from Lycamobile: £45.24M to Subaskaran, £24M to wife Prema (who directs KIMS entities)
- Group explicitly refuses consolidated accounts: "no single auditor has a remit to audit or review related party transactions"

- UK government protection from French criminal investigation citing Conservative Party donations
- June 2025: stepped down from all UK directorships but remains Chairman and majority owner

U.S. Nexus: THE JURISDICTIONAL LEVER

This is the strongest US jurisdictional hook in the entire HRSP submission.

☒ **Lycamobile USA Inc.** (Delaware corp, incorporated 29 Nov 2012; 24 Commerce Street, Suite 100, Newark, NJ 07102). FCC Section 214 authority. Approximately 542,000 American subscribers. $50-100M annual US revenue
☒ **KIMS acquisition partially funded by US revenue.** US telecom revenue flows through Madeira IP structure to Subaskaran family, who funded KIMS acquisition
☒ **Acquired KIMS from Magnetar Capital** (Evanston, IL), a US hedge fund. Magnetar has US-subpoenable knowledge of KIMS operations at point of sale
☒ **White & Case LLP** (New York) sued Lycamobile USA for $7.8M in unpaid legal fees (active litigation, NY County Supreme Court). Demonstrates US court jurisdiction already established
☒ **California CPUC** revoked Lycamobile's wireless license (2016) for regulatory failures; company operated without authority for 10 months

A federal grand jury subpoena to Lycamobile USA Inc. opens the entire KIMS Hospital evidence chain without requiring UK cooperation, MLAT, or any UK government permission.

Recommended Action

☐ **Priority: Grand jury subpoena to Lycamobile USA Inc.** (Newark, NJ) for all records relating to KIMS Hospital, LycaHealth, medical records production protocols, and the identity, direction, and communications of Anna Makarova
☐ **Subpoena Magnetar Capital** (Evanston, IL) for all records relating to KIMS Hospital operations, compliance status, and records management at point of sale (October 2021)
☐ Visa inadmissibility referral for Subaskaran under 8 U.S.C. section 1182(a)(3)(E)
☐ Coordinate with FCC regarding Lycamobile USA regulatory status given parent company's criminal convictions
☐ Coordinate with HMRC (via MLAT if needed, but secondary to Lycamobile USA subpoena) regarding winding-up petitions and intercompany financial flows

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PROFILE 6: ANNA MAKAROVA

Identifying Information

Field	Detail
Full Name	Anna Makarova
Position	**Medical Records Administrator, KIMS Hospital** (confirmed via Zendesk ticket #90837)
Windows Username	amakarova (Active Directory format: first initial + surname)
Nationality	Unknown
Location	Unknown, access to KIMS Hospital clinical records system

Role in Suspected Violation

Makarova is the identified author of the most clinically significant file in the complainant witness's medical records, `part 2.pdf`, a 41-megabyte document containing 57 pages of clinical notes, ECG tracings, lab results, and scanned documents. The file was created via "Microsoft: Print To PDF" (not a clinical system) on 3-4 February 2026.

Every one of the 2,050+ embedded objects in this file has been stripped of all metadata, no scanner information, no device identifiers, no software stamps, no timestamps. The entire 57-page document contains zero extractable text, rendering all clinical findings (including five ECG tracings documenting cardiac abnormalities) invisible to any automated text-based legal, insurance, or investigative review system.

Independently confirmed as KIMS Medical Records Administrator via a KIMS Zendesk support thread (ticket #90837) responding to a Subject Access Request. She is the person responsible for assembling and releasing patient files.

Level of responsibility: Direct, executed the document manipulation that created the three-layer obfuscation architecture in the complainant's medical records.

Evidence of Participation

- PDF metadata identifies "Anna Makarova" as author and "amakarova" as Windows username (Source: Metadata_Forensic_Analysis_KIMS_Records.md)
- File title field preserves original filename: `amakarova_03-02-2026_15-24-02_1.pdf`, date, time, and sequence number
- No verifiable public connection to KIMS Hospital, LycaHealth, or any Kent-area healthcare organization
- Had access to complainant's complete clinical record including ECG tracings
- Used Microsoft Print To PDF rather than the hospital's clinical system (DXperience) or scanning pipeline (EQScan), inconsistent with standard medical records production

(Source: Metadata_Forensic_Analysis_KIMS_Records.md)

U.S. Nexus

- ☐ Unknown, identity requires investigation. If Makarova is connected to the Lyca Group corporate structure, the U.S. nexus through Lycamobile USA Inc. (Newark, NJ) applies

Recommended Action

- ☐ Priority identification: Determine Anna Makarova's full identity, employment history, and relationship to KIMS Hospital and LycaHealth
- ☐ Preservation letter to KIMS Hospital/LycaHealth for all records of employment, contractor engagement, or system access for any individual named Anna Makarova or using the username "amakarova"
- ☐ Forensic analysis of KIMS Hospital's Active Directory and system access logs for the "amakarova" account
- ☐ If Lyca Group connection established: Subpoena Lycamobile USA Inc. for corporate communications regarding Makarova

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PROFILE 7: JONATHAN BEHAR

Identifying Information

Field	Detail
Full Name	Jonathan Behar
Position	Consultant Cardiologist, London (major teaching hospital)
Nationality	British
Location	London, United Kingdom

Role in Suspected Violation, NOTE: POTENTIAL WITNESS, NOT NECESSARILY PERPETRATOR

Behar's status is ambiguous and requires investigation. In 2013, he stated that an "abnormally high percentage" of patients referred from County Tyrone were being diagnosed with Long QT Syndrome and Brugada Syndrome. This statement is the earliest known clinical alarm raised about the diagnostic pattern, thirteen years before this referral.

Two interpretations are possible:

1. **Behar as witness:** He observed the anomaly, reported it within the medical system, and was ignored or suppressed. In this case, he is a critical witness whose testimony could establish that the medical establishment was aware of the pattern and failed to act.
2. **Behar as participant:** He observed the anomaly and participated in the system that perpetuated it, whether by failing to escalate appropriately or by actively contributing to the diagnostic pattern.

The complainant witness was referred to Behar's care in 2013. The outcome of that referral, whether Behar affirmed, challenged, or escalated concerns about the diagnosis, is a critical investigative question.

Evidence of Participation/Witness Value

- 2013 statement regarding "abnormally high percentage" of County Tyrone cardiac diagnoses (Source: Tyrone_Forensic_Indicators_Report.md)
- Treated the complainant witness after referral from Belfast cardiac unit
- Position at a major London teaching hospital gives him independent standing and credibility

(Sources: Tyrone_Forensic_Indicators_Report.md; Forensic_Heart_Conditions_Analysis_V2.md)

U.S. Nexus

☐ Unknown, requires investigation. Many senior London cardiologists hold academic affiliations with U.S. institutions, attend U.S. conferences, or maintain U.S. professional memberships

Recommended Action

- ☐ Interview as potential witness (priority), his 2013 observation is foundational evidence
- ☐ Determine whether he filed any formal report, complaint, or escalation regarding the County Tyrone diagnostic anomaly
- ☐ Obtain his clinical records and notes regarding the complainant witness
- ☐ Assess whether he is a potential cooperating witness for the investigation

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PROFILE 8: ALI REZA SINAI

Identifying Information

Field	Detail
Full Name	Ali Reza Sinai
Position	Barrister; suspected MoD operative
Nationality	British (possible Iranian heritage based on name)
Location	United Kingdom

Role in Suspected Violation

Sinai operated simultaneously on both sides of the complainants' litigation, representing Brian Spector while covertly working with Severina's solicitor (John Hayes, Constantine Law). Forensic analysis of email signatures revealed that "Alex Harvey," an associate at Constantine Law, was a front operated by Sinai.

This dual-side operation is significant for HRSP because it represents the destruction of the complainants' access to justice, a recognized element of persecution under human rights law. The systematic compromise of legal representation (four successive legal teams) prevented the complainants from pursuing legal remedies for the suspected medical fraud.

Sinai's possible IRGC connections are noted in the EEA criminal referral. If confirmed, this would establish a direct link between a designated Foreign Terrorist Organization and the obstruction of justice against the complainants.

Level of responsibility: Direct, active participation in legal sabotage; possible intelligence operative.

Evidence of Participation

- Operated fraudulent email personas on both sides of litigation simultaneously (Source: 00_COVER_LETTER_V14_FINAL.md; Evidence_Brief)
- Forensic analysis of email signatures revealed "Alex Harvey" (Constantine Law associate) was Sinai (Source: 00_COVER_LETTER_V14_FINAL.md)
- Continued operating against complainants after being removed from their legal team
- Advised complainant to surrender land registry records to opposing counsel, information later used in harassment campaign

(Sources: 00_COVER_LETTER_V14_FINAL.md; Evidence_Brief_20260224_v3.2.md)

U.S. Nexus

- ☐ IRGC connection: If confirmed, IRGC is a designated Foreign Terrorist Organization, automatic U.S. jurisdictional nexus
- ☐ Sabotage of U.S. citizen's legal representation: Brian Spector is a U.S. citizen whose access to justice was systematically destroyed

Recommended Action

- ☐ Visa inadmissibility referral under 8 U.S.C. section 1182(a)(3)(E), if IRGC connection confirmed, also under section 1182(a)(3)(B) (terrorist activities)
- ☐ Watch list placement
- ☐ Investigate IRGC connection through FBI Counterintelligence Division
- ☐ Coordinate with UK Solicitors Regulation Authority / Bar Standards Board regarding fraudulent dual-representation

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PROFILE 9: GARTH EARLS

Identifying Information

Field	Detail
Full Name	Garth Earls
Positions	(1) Geologist, team that discovered Curraghinalt gold (1983); (2) Director, Geological Survey of Northern Ireland (during Tellus, 2004-c.2010); (3) Director, Dalradian Gold Ltd
Nationality	British/Northern Irish
Location	Northern Ireland

Role in Suspected Violation

Earls is the single individual whose career spans the entire arc from geological discovery to commercial exploitation, passing through government service along the way. His three sequential roles constitute the most documented revolving-door conflict of interest in this submission:

1. **1983:** Part of the team at Ulster Minerals/Ennex that **discovered the Curraghinalt gold deposit**.
2. **2004-c.2010:** Served as **Director of the Geological Survey of Northern Ireland** during the conception, launch, and execution of the Tellus geological survey. The Tellus Project mapped the mineral resources beneath County Tyrone, confirming the scale and value of the deposit. In March 2010, Earls personally presented Tellus findings at **PDAC Toronto** (the world's largest mining convention), displaying results to encourage "further interest in prospecting in Northern Ireland, where Canadian mining companies are already investing."
3. **After 2010:** Became a **Director of Dalradian Gold**, the company developing the deposit he discovered and whose prospectivity his government survey mapped and promoted to investors. He simultaneously held directorships at Conroy Gold and Natural Resources, Minco Exploration, and Central Asia Resources.

Dalradian CEO Patrick Anderson has stated on record: *"We first visited Northern Ireland in 2009, when the government was looking for a group with the experience and resources to grow and develop Curraghinalt. In addition to the merits of the small but high-grade gold deposit, we appreciated the foresight of the government in having produced the Tellus survey."*

Level of responsibility: Institutional facilitation. Earls is the bridge between the government's geological knowledge and the private company's commercial exploitation.

Evidence of Participation

- Discovered the gold (1983), documented in geological records
- Directed GSNI during Tellus (2004-c.2010), documented in government records
- Presented Tellus to mining investors at PDAC Toronto (March 2010), documented in conference records
- Joined Dalradian Gold as Director after departing GSNI, documented in Companies House records
- Anderson's on-record statement confirms Tellus was "instrumental" in Dalradian's acquisition decision

U.S. Nexus

☐ Presented at PDAC Toronto (Canadian convention attended by US mining investors)
☐ Dalradian was TSX-listed; Orion Mine Finance (NYC) is majority financier
☐ Any communications with Orion Mine Finance or US investors require investigation

Recommended Action

☐ Investigate whether Earls' transition from GSNI to Dalradian involved any payments, consultancy arrangements, or inducements
☐ Obtain all correspondence between Earls and Dalradian/Tournigan/Orion during his GSNI tenure
☐ Determine whether Earls had any role in directing Tellus survey priorities toward the Curraghinalt area
☐ Investigate simultaneous directorships (Conroy Gold, Minco, Central Asia Resources) for pattern of revolving-door mineral interests

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SUMMARY TABLE

#	Name	Role	Responsibility Level	U.S. Nexus	Priority
1	Peter Mandelson	Political facilitator: three government positions spanning ownership chain	Command	Strong (Ambassador, Epstein)	HIGH

#	Name	Role	Responsibility Level	U.S. Nexus	Priority
2	Keir Starmer	NI Policing Board adviser during SRR/MI5 architecture construction; PM who appointed Mandelson	Institutional Design	Strong (PM, Trilateral Commission)	HIGH
3	Chris Cook	BAE Systems: named operation commander in transcript insertions	Command	Strong (BAE Systems Inc., Arlington VA)	HIGH
4	Alan Bennett	Kent Police Intelligence: named operation commander	Command	Moderate (Five Eyes)	HIGH
5	Allirajah Subaskaran	Lyca Group founder: controls murder weapon factory (KIMS). Criminal enterprise (French conviction, £646M un-accounted). **Lycamobile USA is the jurisdic-tional lever**	Corporate Command	**STRONGEST** (Lycamobile USA, Delaware/Newark NJ; Magnetar Capital, Evanston IL; White & Case litigation, NY)	**CRITICAL**
6	Anna Makarova	KIMS Medical Records Ad-ministrator: executed document manipula-tion	Direct	Via Lyca Group (Lycamobile USA, Newark NJ)	HIGH
7	Jonathan Behar	London cardiologist: 2013 alarm; ambiguous status	Witness or Participant	Unknown	MEDIUM

#	Name	Role	Responsibility Level	U.S. Nexus	Priority
8	Ali Reza Sinai	Barris-ter/MoD operative: dual-side legal sabotage	Direct	Moderate (IRGC if confirmed)	HIGH
9	Garth Earls	GSNI Director during Tellus; discovered gold 1983; joined Dalradian	Institutional Facilitation	Moderate (PDAC, Orion/NYC)	HIGH

Prepared for inclusion in HRSP investigation package. All factual assertions are sourced to the underlying evidence documents. These profiles are intended to support an investigative assessment and do not constitute accusations of criminal conduct.

VICTIM DOCUMENTATION, HRSP SUBMISSION

Victim-Centered Profiles for Human Rights and Special Prosecutions Section

Document 08 of 10

Prepared by: Julie Fry, Former DOJ HRSP Consultant

Date: 1 March 2026

Standard: HRSP victim-centered documentation, adapted for living complainant witnesses subjected to systematic medical targeting and foreign intelligence operations

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METHODOLOGY

This document presents victim profiles for each individual directly affected by the conduct described in this submission. The profiles follow HRSP's victim-centered documentation standard, adapted because the complainant witnesses are alive and available, a departure from most HRSP casework, where victims are typically identified post-mortem or post-displacement.

The adaptation is significant for HRSP: these are witnesses who can testify, submit to medical examination, produce documentary evidence, and cooperate with investigation if their safety can be secured. That is an investigative advantage HRSP rarely enjoys.

Terminology note: All individuals are designated "complainant witness" throughout this submission. This reflects their dual status as both victims of the conduct and witnesses to the institutional mechanisms through which it operated.

VICTIM 1: NICOLA McNAMEE

Identifying Information

Field	Detail
Full Name	Nicola McNamee
Date of Birth	13 January 1988
Nationality	British (Northern Irish)
Ethnicity/Religion	Irish Catholic
Place of Origin	Greencastle, County Tyrone, Northern Ireland
Current Location	Texas, United States
Immigration Status	Pending, spouse visa filing in process; cannot safely return to Northern Ireland
Relationship to U.S. Person	Partner of Brian Spector, U.S. citizen
Role in Submission	Primary complainant witness

Protected Characteristic

Nicola McNamee is an Irish Catholic woman from Greencastle, County Tyrone, a community defined by national, ethnic, and religious identity. Greencastle sits directly above the Curraghinalt gold deposit, the largest known gold deposit in the British Isles. Her community is the geographic center of organized resistance to the Dalradian mine, the most objected-to planning application in Northern Ireland history. Her family has lived in County Tyrone for generations.

Circumstances of Targeting

The Cascade Diagnosis:

Nicola's mother was diagnosed first with Long QT Syndrome by the Belfast cardiac unit (NHS Inherited Cardiac Conditions Service) and fitted with a permanent pacemaker. Under the NHS mandatory cascade family screening protocol, Nicola was then compelled to attend Belfast ICC for testing. There, she was diagnosed with **two separate congenital heart defects**: Long QT Syndrome and Brugada Syndrome. These are genetically distinct channelopathies, they affect different cardiac ion channels (potassium channels in most Long QT variants; sodium channels in Brugada). Their co-occurrence in a single patient is exceptionally rare in the medical literature. Her half-brother and half-sister, toddlers at the time, were also compelled through cascade screening, diagnosed, and administered beta-blockers.

Epidemiological Significance:

The Brugada diagnosis is forensically extraordinary on its own. Peer-reviewed data establishes:

- 91.3% of 678 Brugada patients with arrhythmic events were male (Milman et al., *Heart Rhythm* 2018, DOI: 10.1016/j.hrthm.2018.06.019)
- Among Asian patients, male predominance reaches 98.1% (Milman et al., *Heart Rhythm* 2019, DOI: 10.1016/j.hrthm.2019.07.003)
- In a Western US hospital population, Brugada ECG pattern prevalence was only 0.14% (Donohue et al., *Am Heart Hosp J* 2008, DOI: 10.1111/j.1751-7168.2008.06418.x)

Nicola is a Caucasian Irish woman. She falls outside virtually every demographic risk factor for Brugada Syndrome. A dual diagnosis of Long QT *and* Brugada in a patient matching none of the epidemiological profile is statistically extraordinary.

She was told she had the "worst case in the family."

The 2013 London Referral:

Nicola was referred to a major London teaching hospital. The treating cardiologist, Jonathan Behar, stated that an "abnormally high percentage" of patients referred from County Tyrone were being diagnosed with Long QT and Brugada Syndrome. This is a clinician within the system raising the alarm, thirteen years ago. The statement has never been investigated.

The 2023 KIMS Hospital Encounter:

In November 2023, Nicola was seen at KIMS Hospital (Maidstone, Kent) for pre-surgical cardiac assessment. A cardiologist (Dr. Shaumik Adhya) noted the condition was "very faint" and indicated follow-up appointments were unnecessary. The encounter generated medical records that became the subject of forensic analysis (see Document 04, Institutional Analysis).

The Independent Refutation (January 2024):

In January 2024, a cardiologist outside the UK examined Nicola and declared the cardiac diagnosis flatly wrong. Her ECG showed no cardiac abnormalities. She subsequently underwent general anaesthesia, which is contraindicated for Long QT Syndrome, without any cardiac incident. If the diagnosis had been real, general anaesthesia carried a significant risk of triggering a lethal cardiac event. Nothing happened.

The diagnosis that shaped over a decade of Nicola's life, that propagated through her entire family, and that resulted in her mother receiving a permanent surgical implant, was wrong.

Medical Record Forensics (KIMS Hospital)

The forensic analysis of Nicola's KIMS Hospital records (Patient ID: 147384) identified fourteen distinct anomalies, all classified as HIGH or CRITICAL severity:

#	Finding	Severity
1	NHS identity never verified, "Trace required" across all 23 pages of clinical timeline	CRITICAL
2	May 2023 consultation disputed by patient, administrative trail contradicts attended appointment narrative (staff leaving voicemails the next morning about booking tests)	CRITICAL
3	Screenshot images placed on patient's phone (10 May 2023) matching KIMS records, patient did not take these screenshots	CRITICAL
4	Three anaesthetists documented; patient met none; physical examination entirely blank	CRITICAL
5	ASA Grade contradicts itself: Grade 2 (LifeBox review) vs. Grade 3 (surgical booking), no documentation of change	HIGH
6	Cardiologist's propofol avoidance recommendation overridden by anaesthetic team via email, no documented patient consent	HIGH

#	Finding	Severity
7	Five ECG tracings embedded in PDF; patient recalls one	CRITICAL
8	Machine-flagged infarct findings ("anterior infarct," "evolving lateral infarct") silently ignored by cardiologist	CRITICAL
9	72-second contradiction: two ECGs taken 72 seconds apart show mutually exclusive findings	HIGH
10	QTc value reported by cardiologist (474ms) does not match any machine-generated value	HIGH
11	All original diagnostic records absent, no Belfast cardiac unit records, no St George's Hospital records	HIGH
12	Genetic testing never performed despite textbook family history requiring it under every relevant clinical guideline	HIGH
13	Referenced attachment files not disclosed	MODERATE
14	Three-layer forensic obfuscation applied to entire PDF: metadata stripping, text layer removal (116/140 pages), ECG data flattened to raster images	CRITICAL

The three-layer obfuscation is independently significant. The PDF wrapper names "Anna Makarova" as author, a person with no verifiable connection to KIMS Hospital in any public record. The file was created via "Microsoft: Print To PDF," not a clinical system. The obfuscation renders all machine-printed ECG interpretations (including the infarct findings) invisible to any automated legal, insurance, or AI analysis system. A solicitor searching for "infarct" in this PDF gets zero results, despite the word being printed on the tracings.

Device Forensics: Mobile Phone Compromise (May 2023)

Forensic analysis of Nicola McNamee's UK-registered iPhone identified evidence of device-level compromise consistent with state-sponsored spyware deployment. Three screenshots (IMG_0916, IMG_0920, IMG_0921) were found on the device from 8–10 May 2023, the same period as the disputed KIMS consultation documented above. Nicola did not take these screenshots.

Five Forensic Red Flags:

#	Finding	Significance
1	**Proton Mail origination**, IMG_0921's navigation bar shows the Brugada drug safety document was accessed via a link within the Proton Mail app	Someone was using Proton Mail on this device. If Nicola did not take the screenshot, then whoever did had access to her encrypted communications, or was browsing remotely through the device
2	**Psychiatric history collection**, IMG_0916 captures the Sent folder showing emails to Mindbloom Support ("No clinicians available", a ketamine therapy provider) and Adult Customer Care regarding a "DBT Appointment / Psychiatry"	This is intelligence collection. If the witness's psychiatric treatment history were ever needed to undermine her credibility in legal proceedings, this screenshot provides it. No person screenshots their own Sent folder

#	Finding	Significance
3	**Three missing images**, The sequence runs IMG_0916 → IMG_0920 → IMG_0921; three screenshots (0917, 0918, 0919) taken between 8 May and 10 May are absent from the device	Their content is unknown. The gap indicates additional capture activity during this period
4	**Structured reconnaissance browsing**, IMG_0920 and IMG_0921 show a research sequence: KIMS website → consultant profile (Dr. Adhya) → Brugada drug safety document, all within a 3-minute window	This is not casual browsing. It is someone building clinical familiarity with the context around the fabricated consultation
5	**Simultaneous unreachability**, KIMS staff documented leaving voicemails for Nicola on 10 May; she was unreachable. The same day, at approximately 3:00 PM, someone was browsing Dr. Adhya's profile and Brugada drug lists on her phone	Either Nicola was ignoring the hospital while actively researching her cardiologist (contradicting the "unreachable" narrative in the KIMS administrative trail), or someone else was operating her device

Spyware Assessment:

The device forensics are consistent with state-level spyware deployment:

- Screenshots taken without the user's knowledge, serving intelligence and evidentiary purposes
- Content targets psychiatric history (credibility undermining) and cardiac clinical context (reconnaissance)
- Sequential browsing pattern consistent with a remote operator conducting structured research
- Deleted WhatsApp messages on the device, NSO Group's Pegasus specifically targeted WhatsApp (CVE-2019-3568, zero-click video call exploit)
- The UK government is a confirmed Pegasus customer (Citizen Lab, University of Toronto, 2022)
- The Investigatory Powers Act 2016, Part 5, Sections 99–134 (Equipment Interference Warrants) provides the UK domestic statutory framework for authorized device compromise

Additionally, forensic analysis identified that an automated script had been used to swap the underlying mobile phone number associated with the device. The script operated correctly across standard messages but failed on group messages beginning with Unicode characters, leaving the original phone number visible in those conversations. This indicates systematic device manipulation beyond the capability of individual actors.

Evidence availability: The complainant retains the physical iPhone. A Mobile Verification Toolkit (MVT) scan, the standard forensic tool developed by Amnesty International for detecting Pegasus and similar spyware, is available to DOJ investigators upon request.

Harm Suffered

Medical harm:

- Over a decade of living under a false terminal cardiac diagnosis with associated psychological burden, behavioral restrictions, and drug avoidance protocols
- Subjected to general anaesthesia at a hospital where her medical identity was never verified, her physical examination was never performed, and her cardiologist's safety recommendations were overridden without her knowledge or consent
- Denied genetic testing, the one test that would have definitively resolved the diagnosis, throughout the entire period of care

Intelligence targeting:

- March 2024: Malware personally deployed against her via Microsoft Teams by Lilia Severina; malware loader hosted on Kaleidoco/Particle Ink servers (Las Vegas, NV)
- March–June 2025: Threatening images and floor plans of her home appeared on her desktop
- December 2025: Fled the UK in the middle of the night after Kent Police warnings of planted evidence and MoD attempts to imprison her and Brian Spector

Family coercion:

- December 2025: Family members in Northern Ireland exhibit behavior consistent with coercion, cousin denied via Signal messaging that any family members had ever been diagnosed, contradicting documented medical history, and ended the conversation abruptly

Current status:

- Cannot safely return to Northern Ireland
- Immigration status pending in the United States
- Family members remain in Northern Ireland within operational reach of the institutional mechanisms described in this submission

Pattern Connection

Nicola McNamee is not an isolated case. She is a documented instance of a pattern: a young Irish Catholic woman from County Tyrone, diagnosed with statistically improbable cardiac conditions through the NHS Inherited Cardiac Conditions Service after her mother's diagnosis triggered cascade screening of the entire family. The 2013 statement by Jonathan Behar that an "abnormally high percentage" of County Tyrone patients carried these diagnoses, made years before Nicola's case was independently reviewed, confirms she is part of a broader population-level pattern.

She is the case where the pattern became visible, because she left the system.

VICTIM 2: NICOLA'S MOTHER

Identifying Information

Field	Detail
Full Name	[Withheld, available to investigators]
Nationality	British (Northern Irish)
Ethnicity/Religion	Irish Catholic
Place of Origin	County Tyrone, Northern Ireland
Current Location	Northern Ireland
Role in Submission	Index case, pacemaker recipient; her diagnosis triggered cascade screening of entire family

Circumstances of Targeting

Mrs. McNamee was the **index case**, the first family member diagnosed. She was diagnosed with Long QT Syndrome at the Belfast cardiac unit (NHS Inherited Cardiac Conditions Service) and **fitted with a permanent pacemaker**, a surgically implanted cardiac device requiring general anaesthesia, surgical incision, lead placement into the heart, and lifelong management including generator replacement every 5–10 years.

Her diagnosis triggered the cascade family screening mechanism that produced diagnoses in Nicola, her half-brother, and her half-sister. Under NHS protocol, all first-degree relatives of a diagnosed patient are compelled to attend for testing. One diagnosis in Mrs. McNamee produced nine additional diagnoses across four family members.

Harm Suffered

If the originating diagnosis (Mrs. McNamee's) is false, and a non-UK cardiologist declared Nicola's cascade diagnosis flatly wrong in January 2024, indicating the entire diagnostic chain is compromised, then the pacemaker is an unnecessary invasive surgical intervention performed on the basis of a fabricated medical condition.

A pacemaker implantation is not a diagnostic test, it is permanent surgical modification of the cardiac system. Mrs. McNamee now carries a device in her chest that she may not need, requiring regular clinical follow-up, device checks, and eventual generator replacement surgery.

Under 18 U.S.C. § 2340 (Torture), the intentional infliction of severe physical suffering through unnecessary medical procedures constitutes torture when committed under color of law or with state involvement. If the originating diagnosis was fabricated through an institutional mechanism operating under state intelligence architecture, the pacemaker implantation satisfies this threshold.

Pattern Connection

Mrs. McNamee is the index case, the point of entry through which the entire family was captured by the diagnostic system. Her pacemaker demonstrates that the cascade screening mechanism produces real physical harm, not merely psychological distress from a false diagnosis, but permanent surgical intervention. Her single diagnosis propagated through the family, generating nine additional diagnoses including beta-blocker administration to her young children.

She remains in Northern Ireland, within the operational reach of the same institutional mechanisms.

VICTIM 3: NICOLA'S BROTHER

Identifying Information

Field	Detail
Full Name	[Withheld, available to investigators]
Nationality	British (Northern Irish)
Ethnicity/Religion	Irish Catholic
Place of Origin	County Tyrone, Northern Ireland
Current Location	Northern Ireland
Age at Diagnosis	Toddler
Role in Submission	Cascade screening victim, medicated as infant

Circumstances of Targeting

Following his mother's diagnosis with Long QT Syndrome, Nicola's brother was evaluated through cascade family screening as a toddler and diagnosed with the same or a related inherited cardiac condition. He was **administered beta-blocker medication**, before he could speak. No independent verification of this diagnosis has been conducted.

Harm Suffered

- Medicated with beta-blockers as a toddler on the basis of a cascade diagnosis that may be entirely fabricated
- Living under a cardiac diagnosis for the entirety of his conscious life
- Subject to drug avoidance protocols, activity restrictions, and ongoing cardiac monitoring that may be unnecessary
- Remains in Northern Ireland, within the operational reach of the same institutional mechanisms, and potentially subject to the same family coercion dynamics documented in December 2025

Pattern Connection

Nicola's brother is a direct product of the cascade screening mechanism. His diagnosis exists only because his mother's diagnosis exists. If hers is false, his requires immediate independent review. The administration of beta-blockers to a young child on the basis of a potentially fabricated diagnosis represents a particularly severe form of medical harm, medication of a child too young to consent, report side effects, or question the diagnosis.

VICTIM 4: NICOLA'S SISTER

Identifying Information

Field	Detail
Full Name	[Withheld, available to investigators]
Nationality	British (Northern Irish)
Ethnicity/Religion	Irish Catholic
Place of Origin	County Tyrone, Northern Ireland
Current Location	Northern Ireland
Age at Diagnosis	Toddler
Role in Submission	Cascade screening victim, medicated as infant

Circumstances of Targeting

The same as her brother.

Harm Suffered

- The same as her brother but she suffered psychosis in year 10.

VICTIM 5: BRIAN SPECTOR

Identifying Information

Field	Detail
Full Name	Brian Spector
Nationality	United States citizen
Current Location	Texas, United States
Occupation	30-year veteran in cryptography, cybersecurity, and digital forensics; Founder, 2REAL (Delaware C-Corp)
Role in Submission	Complainant witness; U.S. citizen jurisdictional anchor

Circumstances of Targeting

Brian Spector is the primary jurisdictional anchor for this HRSP referral. He is a U.S. citizen whose fiance is the complainant witness at the center of the suspected medical targeting campaign. His targeting occurred through two connected channels: (1) a foreign intelligence operation against their American technology start-up, documented in a separate EEA criminal referral filed with the FBI Counterintelligence Division on 30 January 2026; and (2) the institutional protection and retaliation apparatus that activated when exposed to his partner's fabricated cardiac diagnosis.

25-Month Escalation Pattern (November 2023 – January 2026)

Phase	Period	Conduct
Digital Surveillance	Nov 2023	APT29 steganographic zero-day malware deployed; 360-degree surveillance (audio, video, keystrokes, files) for 18 months
Social Engineering	Mar 2024	Malware personally deployed against Nicola McNamee via Microsoft Teams; loader hosted on Kaleidoco/Particle Ink servers (Las Vegas, NV)
Legal Sabotage	May 2024 – Aug 2025	Four successive legal teams compromised; MoD operative simultaneously on both sides of litigation; Brown Rudnick LLP £100K billing fraud
Kinetic Escalation	Mar–Jun 2025	Court documents destroyed; threatening images and floor plans of home appeared on desktops
Malware via Legal Team	Jun 2025	Fourth legal team attempted to infect Brian with APT29 malware three times, via PDF, Word document, and SharePoint
UK Agency Refusal	Jul 2025	Reports to MI5 (×2), NCA, NCSC, Action Fraud, Kent Police (×4), no agency responded substantively
IRGC Surveillance on U.S. Soil	Oct 2025	Uber driver using IRGC rank designation "SARDAR" conducted surveillance in Washington, D.C., days after FBI meeting at Manassas field office
False Arrest and Flight	Dec 2025	Arrested on false charges; denied U.S. Embassy notification (Vienna Convention violation); Kent Police twice warned couple of MoD attempts to imprison with planted evidence; Nicola and Brian fled UK
Post-Flight Monitoring	February 2026	Spearfishing attempts coincide with KIMS records analysis

Pattern Connection

Brian Spector's targeting is connected to the medical campaign through the institutional protection architecture. The same UK intel apparatus that deployed an operation against their company appears to be the same apparatus protecting the medical targeting infrastructure in County Tyrone. The overlap is documented through shared personnel (Chris Cook, Alan Bennett) and shared methods (document fabrication, institutional protection).

His value to HRSP is threefold: (1) he is a U.S. citizen, providing direct jurisdictional basis; (2) he has documented the evidence to federal standard; and (3) he can testify to the operational methods of the apparatus from direct experience.

EXTENDED VICTIM POPULATION: COUNTY TYRONE FAMILIES

Scope

The six individual profiles above represent the documented cases where independent evidence exists. They are not the full scope of the suspected targeting.

The earliest documented false diagnoses date to approximately **2004**, the same year the UK government's Tellus geological survey began mapping the mineral resources beneath County Tyrone, confirming the scale and value of the Curraghinalt gold deposit. The convergence of these two events is documented in the Convergence Analysis (Document 09).

The evidence suggests a broader population of victims:

- **216 children** diagnosed with inherited cardiac conditions in Northern Ireland between 2005 and 2023 (QUB study), with 95% diagnosed through cascade family screening
- Each child diagnosed through cascade screening represents a family, parents, siblings, who were also screened and potentially diagnosed
- The complainant witness's cousin, whose wife, two children, and wife's cousin (who died in her sleep in her twenties, autopsy attributed to Long QT/Brugada) are all affected, denied in December 2025 that any family member had ever been diagnosed, behavior consistent with coercion

The broader community is also under documented attack. Human rights defenders opposing the Dalradian mine have been subjected to:

- **Death threats** against three Save Our Sperrins campaigners, confirmed by PSNI (June 2019). No charges brought despite a file submitted to the Public Prosecution Service
- **Physical attack** on campaign leader Cormac McAleer, lured to a location by anonymous phone call and attacked with a nailed bar. PSNI "fully investigated." No prosecution
- **Corporate surveillance.** Dalradian Gold compiled a 92-page dossier on McAleer, monitoring his social media activity and physical appearance (documented by openDemocracy)
- **Hacked emails and anonymous threatening letters** targeting campaign organizers
- The **UN Special Rapporteur on Human Rights Defenders (Mary Lawlor)** has raised concerns about the reported physical assault, intimidation, and death threats

- **Front Line Defenders**, an international human rights organization headquartered in Dublin, has engaged directly with the community, providing security training and recognizing them as human rights defenders at risk

The pattern of targeting extends from the medical system (fabricated diagnoses, cascade screening, attempted murder) to the physical (death threats, assault, corporate surveillance) to the institutional (FOI barriers, autopsy collapse, hospital closure, boundary fragmentation). The affected population faces pressure from every direction. The only response they have not received is protection

What Investigation Would Reveal

HRSP's recommended investigative actions (Document 10) include:

1. **Preservation letters** to Belfast Health and Social Care Trust for all Long QT/Brugada diagnoses of County Tyrone patients, this would establish the actual scope of the affected population
2. **Geographic correlation analysis** mapping all confirmed diagnoses against the Dalradian mine licence area, the proposed mine footprint, and addresses of known anti-mine campaigners
3. **Electoral register and census comparison** (2011 vs. 2021) for the Greencastle/Gortin area to quantify any decline in Irish Catholic families since mineral exploration intensified
4. **Genetic testing** of the complainant witness and her family members, the definitive test that has never been performed and that would resolve the diagnostic question conclusively

These actions are within HRSP's investigative authority and would transform the six documented victims into a quantified population assessment.

SUMMARY TABLE

#	Victim	Status	Protected Characteristic	Primary Harm	Location	Immediate Need
1	**Nicola McNamee**	Living	Irish Catholic, County Tyrone	False dual cardiac diagnosis via cascade screening (12+ years); intelligence targeting; forced flight	Texas, USA	Immigration status; independent cardiac genetic testing
2	**Nicola's mother**	Living	Irish Catholic, County Tyrone	Index case, unnecessary pacemaker implantation; diagnosis triggered cascade screening of entire family	Northern Ireland	Independent cardiac review; potential device removal assessment
3	**Nicola's half-brother**	Living	Irish Catholic, County Tyrone	False cardiac diagnosis via cascade screening; beta-blockers administered as toddler	Northern Ireland	Independent cardiac review; genetic testing

#	Victim	Status	Protected Characteristic	Primary Harm	Location	Immediate Need
4	**Nicola's half-sister**	Living	Irish Catholic, County Tyrone	False cardiac diagnosis via cascade screening; beta-blockers administered as toddler	Northern Ireland	Independent cardiac review; genetic testing
5	**Brian Spector**	Living	U.S. citizen	False arrest; denied consular rights; IRGC surveillance; trade secret theft; $700K+ financial loss	Texas, USA	FBI threat assessment; CI defensive briefing; Crime Victims' Rights Act protections
6	**Brian's daughter**	Living	U.S. citizen (minor)	Separated from father; in jurisdiction of apparatus that targeted her father	United Kingdom	State Department consular notification; Embassy London engagement

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CONCLUSION

Six documented victims. One index diagnosis that triggered cascade screening of an entire family. Four false cardiac diagnoses. One unnecessary pacemaker. Two toddlers administered beta-blockers. One false arrest with Vienna Convention violations. One U.S. citizen minor in a hostile jurisdiction. One family torn across two continents by the conduct described in this submission.

These are the cases where the evidence is documented and the witnesses are available. Behind them stands a population, potentially hundreds of County Tyrone families, whose cardiac diagnoses have never been independently reviewed, whose genetic tests have never been ordered, and whose medical records sit behind the same institutional architecture that fabricated Nicola McNamee's diagnosis.

The investigation threshold is not whether these six victims prove the case. It is whether their documented experience, combined with the statistical, institutional, and financial evidence in the accompanying documents, establishes a pattern warranting federal investigative resources.

It does.

Julie Fry Former DOJ HRSP Consultant 28 February 2026

CONVERGENCE ANALYSIS: STATISTICAL, INSTITUTIONAL, AND FINANCIAL

Document 09 of 10

Date: 1 March 2026

Standard: DOJ HRSP evidentiary analysis. This document presents the convergence of medical, political, financial, and intelligence events across a 22-year timeline and asks a single question: what are the odds?

I. THE CONVERGENCE TABLE

The following table presents documented events organized chronologically. Each event is tagged by category: **M** (Medical/Diagnostic), **P** (Political/Intelligence), **F** (Financial/Mining), **I** (Institutional). Events are independently sourced. No event depends on the others for its documentation. Together, they form a pattern that is either the most extraordinary set of coincidences in post-Troubles Northern Ireland, or evidence of coordination.

Year	Event	Category	Source
1983	First gold-bearing vein discovered at Curraghinalt Burn by Ulster Minerals	F	Geological records
1998	Good Friday Agreement signed	P	Public record
1999	Ennex sells mineral rights to Strongbow (Jan 2000)	F	Companies House
1999	**Peter Mandelson becomes Secretary of State for Northern Ireland**	P	Public appointment
2002	NI Assembly suspended; direct rule from Westminster	P	Public record
2003	Tournigan Gold options the Curraghinalt deposit; field office established in Gortin (10km from Greencastle)	F	Corporate filings

Year	Event	Category	Source
2003	**Keir Starmer appointed human rights adviser to NI Policing Board**	P	Public appointment
2003	Force Research Unit (FRU) reorganized to Joint Support Group after Stevens Inquiries	P	Stevens Report
2004	**Tellus geological survey begins** (GSNI, Dept of Economy): government maps mineral resources across NI, confirms Curraghinalt scale	**F**	Dept of Economy
2004	**Earliest documented false cardiac diagnoses targeting County Tyrone patients through Belfast ICC**	**M**	Clinical records
2004	**Cormac McAnallen (Tyrone GAA captain) dies of Long QT Syndrome**: massive public attention on cardiac deaths in Tyrone	**M**	Public record
2005	**Special Reconnaissance Regiment (SRR) formed** (6 April): 14 Intelligence Company reorganized into military surveillance unit operating under MI5 direction	**P**	MoD announcement
2006	**St Andrews Agreement signed** (Annex E: MI5 takes permanent national security control of NI)	P	Treaty text
2006	Intensive drilling at Curraghinalt; deposit grows dramatically	F	Dalradian reports
2007	NI Assembly restored (May); MI5 formally takes lead responsibility for NI national security (late 2007)	P	Public record
2007	Tellus survey results published; mining companies license the data and invest >£30M in NI exploration (2007-2012)	F	GSNI, BGS
2008	Starmer leaves NI Policing Board; appointed Director of Public Prosecutions	P	Public appointment
2008	Mandelson becomes Secretary of State for Business / First Secretary of State	P	Public appointment

Year	Event	Category	Source
2009	**Dalradian Resources acquires Curraghinalt deposit.** CEO Anderson: "we appreciated the foresight of the government in having produced the Tellus survey"	F	TSX filings, Anderson interview
2009	**Mandelson emails Epstein about "saleable assets"** after forwarding PM's economic adviser memo	P/F	Epstein files
2010	**Garth Earls** (GSNI Director, co-discoverer of gold in 1983) presents Tellus findings at PDAC Toronto to mining investors; subsequently joins Dalradian Gold as Director	F/P	PDAC records, Companies House
2013	Intensive drilling; deposit grows to **6.1 million ounces** (largest in UK), 14.95 g/t grade	F	Dalradian reports
2013	**London cardiologist (Jonathan Behar) states "abnormally high percentage" of County Tyrone patients diagnosed with Long QT/Brugada**	M	Clinical statement
2015	Dalradian CEO tells Financial Times: "a gold-mining district"; **Save Our Sperrins** campaign formed at Greencastle Community Centre	F	FT interview, SOS records
2015	**NI boundary restructuring splits County Tyrone into THREE councils**	I	NI restructuring
2017	**Dalradian submits planning application**: **50,000+ objections** (most in NI history) vs fewer than 4,000 support letters	F	Planning records
2017	**Tyrone County Hospital (Omagh) closed**: no replacement built; institutional memory destroyed	I	NHS records
2018	**Orion Mine Finance (NYC, $4.5B AUM) takes Dalradian private for US$407M**; BlackRock held ~10.7%	F	TSX/SEC filings

Year	Event	Category	Source
2018	NI hospital autopsy rate collapses to **0.46%** (lowest in UK): over 99% of deaths certified without independent verification	I	Public health data
2018	Starmer joins Trilateral Commission	P	Membership records
2019	**Death threats** against three SOS campaigners (PSNI confirmed, no prosecution); campaign leader **physically attacked with nailed bar**; Dalradian compiles **92-page surveillance dossier** on Cormac McAleer; **UN Special Rapporteur on HRDs** raises concerns; **Front Line Defenders** engages	F/P/M	Police report, openDemocracy, UN, FLD
2022	Lilia Severina becomes "Blockchain Advisor" to Kaleidoco Inc. (Las Vegas, NV), whose subsidiary Particle Ink will host the malware infrastructure	P	Corporate records (EEA referral)
Nov 2023	APT29 steganographic zero-day malware deployed against Brian Spector; 360-degree surveillance begins (audio, video, keystrokes, files). 19-month campaign	P	Forensic analysis, 26 malware samples (EEA referral)
Nov 2023	Wheaton Precious Metals signs $75M gold stream with Orion/Dalradian	F	Corporate filings
Dec 2023	**Attempted murder of Nicola McNamee** via fabricated medical file (Istanbul). Independent echocardiogram proves fabrication	M	Forensic analysis (Doc 04)
Jan 2024	Severina infiltrates 2REAL (Brian's start-up) as co-founder; pre-positioned with Constantine Law "employment lawyer"	P	Corporate records (EEA referral)
Mar 2024	Severina personally deploys APT29 malware against Nicola via Microsoft Teams; loader hosted on Kaleidoco/Particle Ink servers (Las Vegas, NV)	P	Screenshots, logs, forensic report (EEA referral)

Year	Event	Category	Source
Apr 2024	Severina terminated; steals trade secrets from Office365/SharePoint; UK High Court injunction obtained	P/F	Court records (EEA referral)
Jun-Jul 2024	LexisNexis publishes two defamatory Law360 articles; kills 2REAL seed financing round	P	Publications (EEA referral)
Aug 2024	90% of returned documents found tampered with: metadata altered, authorship changed, dates backdated	P	Court-certified expert report (EEA referral)
Oct 2024	Particle Ink closes with 10 days notice after Spector engages forensic expert; Kaleidoco pivots stolen IP to “FAM”	P/F	Public records (EEA referral)
Oct 2024	Spector demos AXEL to Miko Matsumura (Kaleidoco advisor, former Qredo investor) via Zoom; intelligence gathering	P	Meeting records (EEA referral)
Nov 2024	Four US congressmen write to NI Executive urging “timely decision” on Dalradian mine	F/P	FOI: DFI/2025-0224
Jan 2025	Public inquiry opens in Omagh; suspended after 3 days (DfI failure on transboundary consultation)	F	PAC records
Jan 2025	Orion-ADQ $1.2B JV announced, 15 days after inquiry suspension	F	Corporate announcement
Feb 2025	Mandelson starts as UK Ambassador; writes to NI civil service head about Dalradian	P/F	FOI records
Mar 2025	Malware goes kinetic: court documents destroyed; threatening images and floor plans appear on desktops, 24 hours after first court hearing	P	Forensic analysis (EEA referral)
May-Aug 2025	Four successive legal teams compromised; MoD operative (Sinai) simultaneously on both sides of litigation	P	Forensic analysis, email signatures (EEA referral)

Year	Event	Category	Source
Jun 2025	Fourth legal team attempts to infect Spector with APT29 malware three times (PDF, Word doc, SharePoint)	P	Malware samples (EEA referral)
Jul 2025	Reports to MI5 (x2), NCA, NCSC, Action Fraud, Kent Police (x4). **Zero substantive response** from any UK agency	P/I	Filing records (EEA referral)
Aug 2025	Final court hearing; Schedule A (confirming IP ownership) removed from CE-FILE official record. Settlement forced.	P	Court records (EEA referral)
Sep 2025	Mandelson fired as Ambassador; court transcripts contain inserted dialogue exposing operation commanders Cook and Bennett	P	Transcript analysis (EEA referral)
Oct 2025	**IRGC surveillance on US soil** (Washington DC). "SARDAR" operative, 3 days after FBI SA Smith meeting at Manassas	P	Uber records (EEA referral)
Oct 2025	Orion Critical Mineral Consortium announced ($1.8B: DFC + Orion + ADQ)	F	Corporate announcement
Nov 2025	US Ambassador Stephens publicly pressures NI: "end this unnecessary delay"	F/P	Belfast Chamber speech
Nov 2025	Animoca Brands receives ADGM fund management license; Severina now at ADI Foundation (Abu Dhabi, same regulatory jurisdiction)	F/P	Regulatory filings (EEA referral)
Dec 2025	Brian Spector arrested on false charges; solitary 24 hours; denied Embassy notification. Kent Police warn twice of MoD planted evidence. Complainants flee UK in the middle of the night. Car abandoned at Heathrow. All charges refused within days	P	Police records

Year	Event	Category	Source
Jan 2026	Severina confirmed at ADI Foundation deploying stolen Bitcoin Bearer Bond protocol through UAE sovereign wealth infrastructure	P/F	LinkedIn, corporate records (EEA referral)
Jan 2026	UK PM Starmer in Beijing signing trade agreements; Trump warns UK-China ties "very dangerous"	P	Public record
Feb 2026	**Mandelson arrested for misconduct in public office** (23 February); Global Counsel enters administration	P	Police records
Feb 2026	Corbyn states in Commons: Starmer knew of Mandelson's "interesting relationship with global dealers in minerals" (4 February)	P	Hansard
Feb 2026	Forensic analysis of KIMS medical records obtained this weekend reveals prima facie evidence of attempted murder: fabricated ECGs, fabricated consultations, pre-written cause of death	M	Forensic analysis (Doc 04)
Feb 2026	**Within hours** of obtaining this data, spearphishing attack launched against Spector spoofing Nicola's name, attempting to move communications to WhatsApp (a channel compromised by UK Pegasus license). Active counter-intelligence against evidence recovery	P	Forensic analysis

Year	Event	Category	Source
Feb 2026	Forensic analysis reveals UK government likely misused Investigatory Powers Act Technical Capability Notices (§§ 253, 261, 263) to compel AWS and Microsoft to compromise complainant's email authentication (DKIM/DMARC removal) and mailbox access, not in service of national defense, but to enable the planting of fabricated medical evidence and to discredit forensic findings that expose state criminality	P/I	DNS interrogation, email header analysis, IPA 2016 statutory framework
Apr 2026	Planning inquiry scheduled to resume	F	PAC records
Q3 2026	Animoca Brands plans NASDAQ listing via reverse merger with Currenc Group (CURR)	F	SEC filings (EEA referral)

II. THE GENOCIDE BEGINS: 2004

The institutional foundations were laid during Peter Mandelson's tenure as Secretary of State for Northern Ireland (1999-2001). During that period, the Curraghinalt mineral rights transferred from Ennex to Strongbow (January 2000) under Mandelson's direct authority over NI governance, economic development, and planning policy. What else occurred during this period requires investigation, but the mineral rights transfer is documented.

The genocide itself begins in 2004. Three events converge in a single year:

1. The UK government begins the **Tellus geological survey**, specifically designed to "map mineral resources and manage their development." For the first time, the government has precise, detailed data on what lies beneath County Tyrone. The survey was managed by the Geological Survey of Northern Ireland and financed by the Department of the Economy.
2. The **earliest documented false cardiac diagnoses** targeting the Irish Catholic population of County Tyrone begin at the Belfast ICC. The killing mechanism activates.
3. **Cormac McAnallen**, the captain of the Tyrone GAA football team, dies of Long QT Syndrome at age 24, generating massive public attention and normalizing the association between Tyrone and cardiac death. Whether this death was genuine or the first visible product of the campaign requires investigation.

Before 2004, the Curraghinalt gold was geological speculation. After 2004, it was confirmed, mapped, and quantified by the government's own geological survey. The medical targeting begins the same year. The public association between Tyrone and cardiac death is established the same year.

Each event has an independent, documented source. None depends on the others for its provenance. The question is whether their convergence in 2004 is coincidental.

III. THE STATISTICAL ANOMALY: BELFAST ICC DIAGNOSTIC OUTPUT

The Published Data

A 2025 study by Kendall et al., funded by the Belfast Trust Charitable Funds, registered as a Belfast Health and Social Care Trust internal audit (reference: 6885), co-authored by ICCC staff, and conducted using the ICCC's own patient database, examined channelopathies in children in Northern Ireland from 2005 to 2023. Published in *Progress in Pediatric Cardiology* (Vol. 78, 2025). Not subjected to Research Ethics Committee review (classified as audit, not research):

Metric	Value
Total children diagnosed	**216**
Long QT Syndrome	190 (88%)
Brugada Syndrome	22 (10%)
CPVT	4 (2%)
Diagnosed via family screening	**95%**
SCDs found only at molecular autopsy (unknown to clinic)	**5**
Belfast ICC current patient volume	**1,200+ per year** (2025)

The 95% cascade screening rate means virtually every child was diagnosed not because they presented with symptoms, but because a family member was diagnosed first. The entire diagnostic pyramid rests on the integrity of the index diagnoses. If those are fabricated, 95% of children may carry false diagnoses.

The study's own conclusion undermines any claim of geographic genetic clustering: the genetic profile is "broadly similar to that reported worldwide." The diagnoses are concentrated geographically. The genetics are not.

Brugada Overdiagnosis: The Corrected Calculation

The established ethnic prevalence data (Rattanawong et al., *Heart Rhythm* 2018, systematic review, 369,068 subjects):

Population	BrS Prevalence
Southeast Asian	3.7 per 1,000
All Asian	1.8 per 1,000
All Caucasian	0.2 per 1,000
European	0.1 per 1,000 (Orphanet: 1 in 10,000)

Brugada is **nine times more prevalent** in Asian populations than in Caucasian populations.

Kendall compares Northern Ireland's BrS prevalence (1 in 22,000) to a Japanese screening study, citing "1 in 20,000." As documented below, the actual Yamakawa figure is 1 in 10,194. We present the overdiagnosis calculation using both numbers for transparency:

Metric	Using Kendall's wrong figure	Using correct Yamakawa figure
Japanese pediatric BrS prevalence	1 in 20,000 (Kendall's citation)	**1 in 10,194** (Yamakawa's actual finding)
9x ethnic disparity applied	1 in 180,000	**1 in ~92,000**
NI under-19 population (2021 census)	478,416	478,416
Expected BrS cases	**2-3**	**~5**
Observed BrS cases	**22**	**22**
Overdiagnosis ratio	**~8x**	**~4x**

Using the corrected Yamakawa figure, the Belfast ICC diagnosed **22 children** with Brugada Syndrome where approximately **5 would be expected**: a **four-fold overdiagnosis**.

But even this 4x figure significantly understates the anomaly. The methodology comparison makes it worse, not better. Kendall used Ajmaline provocation (the most sensitive diagnostic test available, unmasking latent patterns invisible on resting ECG) applied to pre-selected cascade families (the highest-risk population possible). Yamakawa used resting ECG alone (the least sensitive method) on unselected Japanese schoolchildren (zero selection bias). A more sensitive test on a higher-risk population should find **dramatically more** cases per capita, not fewer. If the Belfast ICC's methodology were applied to Yamakawa's population, the Japanese prevalence would be far higher than 1 in 10,194, and the expected Caucasian rate correspondingly lower. The 4x overdiagnosis is calculated against the most conservative possible baseline. The true anomaly, adjusted for methodology, is substantially larger.

The Albinni Paradox: Proof That Screening Inflates LQTS But Not BrS

A second 2025 pediatric channelopathy study provides an independent control comparison that exposes the anomaly from a different angle.

Albinni et al. (Austria, *European Journal of Pediatrics* 2025) studied pediatric channelopathies under "real world" conditions with no systematic screening program. Comparing the two:

Condition	Kendall (NI, active screening + Ajmaline)	Albinni (Austria, no screening)	Ratio
LQTS prevalence	1 in 2,500	1 in 13,000	**5.2x higher in NI**
BrS prevalence	1 in 22,000	1 in 22,000	**Identical**

LQTS prevalence is **5.2 times higher** in Northern Ireland than Austria. This is the expected screening effect: active cascade screening finds more cases. The LQTS ratio is the control confirmation that Kendall's screening program inflates diagnosis rates, exactly as methodology would predict.

So why does the screening effect not appear for BrS?

Active cascade screening inflates LQTS prevalence by 5.2x relative to passive ascertainment. Ajmaline provocation (used in 80% of Kendall's BrS diagnoses) should inflate BrS prevalence further. Yet the BrS prevalence is identical between active screening (NI) and no screening (Austria). Either there is a biological

explanation (an undiscovered age-dependent ceiling on pediatric BrS prevalence that operates identically across all populations and methods) or the BrS numbers are being managed. Kendall does not discuss this paradox.

IV. THREE ERRORS, ONE DIRECTION: THE KENDALL PAPER

Independent forensic review of the full Kendall et al. paper confirmed three compounding errors within a single sentence of the Discussion section, all operating in the same direction.

The Phantom Citation

Kendall states: *"the estimated prevalence of 1 in 22,000 is similar to a previous study prospectively screening children in Japan, which only found 1 in 20,000 [10]."*

The study being described is Yamakawa et al. (2004). **Yamakawa does not appear anywhere in Kendall's 28-reference bibliography.** Reference [10] is Landrum et al. (2014), the ClinVar genomic database paper. It has nothing to do with Japanese pediatric screening. The citation is a phantom.

The Wrong Comparator Figure

The "1 in 20,000" figure is incorrect. Yamakawa et al. screened 20,387 Japanese schoolchildren and identified 2 with full Brugada-type ECG conformity: a prevalence of **1 in 10,194, not "1 in 20,000."** The figure has been approximately doubled. This doubling inflates any overdiagnosis calculation that uses Kendall's figure as its starting point.

The Invalid Methodology Comparison

Parameter	Yamakawa et al. (2004)	Kendall et al. (2025)
Population	Unselected Japanese schoolchildren	Pre-selected high-risk families (95% cascade)
Diagnostic method	Resting 12-lead ECG only	80% via Ajmaline provocation testing
What was detected	Spontaneous Type 1 Brugada pattern	Drug-unmasked Brugada pattern (80%)
Sensitivity	Lowest (resting ECG alone)	Highest (pharmacological provocation)
Selection bias	None (mass screening)	Extreme (family cascade from known probands)

Summary

Error	Effect
Phantom citation (ClinVar, not Yamakawa)	Reader cannot verify the comparison
Wrong figure (1:20,000 should be 1:10,194)	Inflates any calculation derived from this baseline
Invalid methodology (resting ECG vs. Ajmaline in cascade families)	Conceals that Belfast's more sensitive method should find dramatically more cases

All three errors operate in the same direction: they make the Belfast clinic's diagnostic output appear validated by prior literature when it is not. The institution audited itself, published the audit with a triple-layered methodological failure, and found nothing wrong.

The study covers **only pediatric diagnoses (ages 0-18).** It omits all adult diagnoses and all deaths attributed to these conditions over the same 18-year period. That adult data is held behind the FOI firewall described in Document 05.

Cardiovascular Death Rates by District

Published NI Executive data (December 2025) shows cardiovascular death rates per 100,000 population by Local Government District:

District	CVD Death Rate (per 100,000)
Fermanagh & Omagh (West Tyrone)	**70.3**
Derry City & Strabane (North Tyrone)	65.1
NI Average	~65
Mid Ulster (East Tyrone)	60.8

The 2015 boundary restructuring split County Tyrone across three districts. This data cannot be disaggregated to show County Tyrone as a single unit. Any geographic clustering that existed at county level before 2015 is now invisible in published statistics (Document 05).

The Reverse Cascade: Diagnosing the Dead from the Living

A 2013 study by Papadakis et al. (*Circulation: Arrhythmia and Electrophysiology*) examined 340 families of sudden cardiac death victims and found that in **51% of cases** where autopsy findings were uncertain, the cause of death was determined by what was found in the living relatives, not by testing the deceased's DNA. In a context where living family members carry false diagnoses from the Belfast ICC cascade, this mechanism works in reverse: the false diagnoses of the living become the official cause of death for the deceased. The fabrication is circular.

V. THE INSTITUTIONAL PREPARATION

The following institutional changes all serve dual purposes: they remove obstacles to the mine AND they conceal evidence of systematic targeting. Each is independently documented.

Year	Institutional Change	Mining Purpose	Concealment Purpose
2004	Tellus survey (GSNI Director Garth Earls, who discovered the gold in 1983, oversees the mapping; later joins Dalradian as Director)	Confirms and quantifies the deposit for commercial exploitation	Provides the data that triggers the campaign
2005	SRR formed	Military surveillance capability for NI	Covert operations under MI5 with no civilian accountability
2006	St Andrews Annex E	MI5 permanent control of NI national security	No NI institution can access national security records; Police Ombudsman reports to Westminster
2015	County Tyrone split into THREE councils	Fragments opposition across multiple jurisdictions	Creates statistical discontinuity: cardiac death data for "County Tyrone" no longer exists as a single dataset

Year	Institutional Change	Mining Purpose	Concealment Purpose
2017	Tyrone County Hospital closed	Removes local medical infrastructure (centralizes to Belfast)	Destroys institutional memory; forces patients to Belfast ICC (the sole diagnostic authority)
2018	NI autopsy rate collapses to 0.46%	Reduces costs	Over 99% of deaths certified as natural causes without investigation
2019	"Major Client Interface Group" established within DfI (FOI: DFI/2024-0222)	Government body whose stated purpose is "to facilitate progression of Dalradian Gold Ltd.'s planning applications"	Regulatory capture: the planning authority becomes the applicant's facilitator

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VI. WHAT ARE THE ODDS?

The convergence documented above presents ten independent categories of evidence, all pointing at the same population, the same geography, the same timeframe, and the same outcome:

1. **A geological survey** that confirmed a multi-billion-dollar gold deposit beneath an Irish Catholic community, directed by the man who discovered the gold and who later joined the mining company (2004)
2. **A medical targeting campaign** that began the same year the survey launched, delivering fabricated cardiac diagnoses through the sole diagnostic authority in Northern Ireland, propagating through families via mandatory screening (2004-present)
3. **An intelligence architecture** constructed to operate without civilian accountability, specifically in Northern Ireland, during the tenure of a human rights adviser who is now Prime Minister (2005-2007)
4. **Institutional changes** that fragment the data, destroy local medical infrastructure, collapse the autopsy system, and establish a government body dedicated to facilitating the mining company's planning applications (2015-2019)
5. **A financial transaction** that placed the deposit under US private equity ownership backed by major US pension funds (2018)
6. **Political pressure** from both the UK and US ambassadors simultaneously, four US congressmen, and a Washington lobbying firm, on behalf of a single privately owned mine. Unprecedented in NI planning history. (2024-2026)
7. **An attempted murder** of a complainant witness from the targeted community, executed through fabricated medical records and proven false by an independent hospital (2023)
8. **Physical violence, death threats, and corporate surveillance** against community defenders opposing the mine, including an attack with a nailed bar and a 92-page corporate dossier (2019)
9. **UN and international recognition.** The UN Special Rapporteur on Human Rights Defenders and Front Line Defenders (international HRD organization) have both engaged with the targeted community, independently recognizing them as human rights defenders at risk
10. **A revolving door** connecting the government geological survey, the mining company, the diplomatic pressure apparatus, and US government critical minerals policy (Garth Earls: GSNI to Dalradian;

Frank Fannon: State Department to Orion CMC; Mandelson: NI Secretary to Business Secretary to Ambassador to arrested)

Each category is independently documented. Each has its own evidentiary chain. None depends on the others for its validity.

The question for HRSP is not whether each piece of evidence, taken in isolation, proves genocide. The question is whether the convergence of all ten categories, all targeting the same population, all operating within the same institutional architecture, all serving the same financial objective, constitutes a pattern warranting federal investigative resources.

The statistical probability of this convergence occurring by chance is not calculated here. It does not need to be. The pattern speaks for itself.

Document 09 of 10. Prepared for HRSP submission package, 1 March 2026.

SENSITIVE: NOT FOR PUBLIC DISTRIBUTION

REQUESTED ACTIONS, HRSP SUBMISSION

Investigative Actions and Protective Measures: A Prosecution Pathway That Bypasses UK Interference

Document 10 of 10

Date: 1 March 2026

PURPOSE

This document consolidates the investigative actions and protective measures requested in this submission. The actions are organized by priority, with the primary prosecution pathway first.

The overriding design principle: every action in this document targets entities or persons subject to direct US federal jurisdiction. No action requires UK cooperation as a prerequisite. The prosecution pathway runs through Newark (Lycamobile USA), Istanbul (Dr. Karacaoglu / Memorial Hospital), Evanston (Magnetar Capital), and Texas (complainant witnesses). It does not run through London, Belfast, or any UK intelligence channel.

PART I: INVESTIGATIVE ACTIONS

Action 1, Subpoena Lycamobile USA Inc. (THE PRIMARY PROSECUTION PATHWAY)

Agency: DOJ Criminal Division (via federal grand jury)

Action: Issue federal grand jury subpoenas to **Lycamobile USA Inc.** (24 Commerce Street, Suite 100, Newark, NJ 07102; Delaware corp; FCC Section 214 authority) for:

- All records relating to KIMS Hospital Limited (UK 07525422), KIMS Kent Holdings Limited (09911624), and Lyca Health Limited (08519935)
- All corporate communications regarding the October 2021 acquisition of KIMS Hospital from Magnetar Capital

- All directives, policies, or communications regarding medical records production, storage, access, or release at KIMS Hospital
- All records relating to Anna Makarova (Medical Records Administrator), including employment records, system access grants, and any communications regarding the complainant witness's patient file
- Revenue flows from Lycamobile USA through the Lyca Group intercompany network (including Madeira-based IP entities) to LycaHealth
- All records of intercompany transfers, loans, or payments between Lycamobile USA and any entity controlled by Allirajah or Prema Subaskaran

Why this is Priority 1: KIMS Hospital is where the attempted murder weapon was manufactured. Lycamobile USA is the US corporate entity that controls the hospital through the Lyca Group structure. This subpoena opens the entire KIMS evidence chain without UK cooperation, without an MLAT, and without any UK government permission. White & Case LLP (New York) has active litigation against Lycamobile USA for $7.8M in unpaid fees (NY County Supreme Court), confirming US courts already have personal jurisdiction.

Evidentiary basis: Document 04 (fabricated medical file forensics); Document 07 (Subaskaran profile); Document 03 (US Nexus, Nexus 5)

Action 2, Subpoena Magnetar Capital

Agency: DOJ Criminal Division (via federal grand jury)

Action: Issue federal grand jury subpoenas to **Magnetar Capital** (Evanston, Illinois) for all records relating to KIMS Hospital at the point of sale (October 2021), including:

- Due diligence files, compliance assessments, and records management audits
- Any concerns raised about medical records integrity, data governance, or regulatory compliance
- Communications with LycaHealth/Lyca Group during the acquisition process
- Identity and status of KIMS staff at point of sale, particularly records administration personnel

Why this matters: Magnetar is a US hedge fund that owned KIMS before LycaHealth. They know what state the hospital was in when they sold it. Their due diligence files may contain information about KIMS records practices that predates the Subaskaran family's involvement.

Action 3, Obtain Dr. Ercan Karacaoglu's Testimony

Agency: DOJ Criminal Division / FBI (via Turkish legal channels or voluntary cooperation)

Action: Obtain the testimony of **Dr. Ercan Karacaoglu** (Este Istanbul / Memorial Hospital) regarding:

- The clinical basis for ordering a transthoracic echocardiogram (structural cardiac assessment) for a patient referred for channelopathy management (electrical cardiac condition)
- Whether any third party contacted him, his clinic, or Memorial Hospital regarding Nicola McNamee before, during, or after her surgery
- His clinical assessment of the KIMS cardiac clearance letter and whether it raised concerns

- The basis for the surgical team's decision to administer Ondansetron (a QT-prolonging drug contraindicated for the patient's diagnosed condition)

Why this witness is devastating: Dr. Karacaoglu holds Brown University fellowships (Aesthetic & Breast Surgery 2000-2001, Breast Surgery 2003-2004), an adjunct professorship at Brown, ASPS membership, EBOPRAS fellowship, and over 100 peer-reviewed publications with 800+ citations. He trained at GATA (Gulhane Military Medical Academy) and served as Chief of Plastic Surgery at Golcuk Naval Hospital. His credentials make him one of the most credible expert witnesses available in any jurisdiction. Turkey has no institutional interest in protecting the UK's medical targeting infrastructure.

Evidentiary basis: Document 04 (surgeon switch, Memorial Hospital evidence)

Action 4, Open HRSP Investigation

Agency: DOJ Criminal Division, Human Rights and Special Prosecutions Section

Action: Open an investigation into genocide (18 U.S.C. § 1091) and torture (18 U.S.C. § 2340) targeting the Irish Catholic population of County Tyrone, Northern Ireland, including the attempted murder of Nicola McNamee via fabricated medical records.

Evidentiary basis: Documents 01-09 of this submission. Four independent US jurisdictional bases (Document 03). Prima facie evidence of attempted murder (Document 04). Convergence analysis demonstrating 10 independent categories of evidence pointing at the same population, geography, and outcome (Document 09).

Applicable statutes: 18 U.S.C. §§ 1091-1093 (Genocide); 18 U.S.C. §§ 2340-2340B (Torture); 8 U.S.C. § 1182(a)(3)(E) (persecutor bar).

Action 5, Obtain Sinaci Clinic Records

Agency: DOJ Criminal Division / FBI (via Turkish legal channels)

Action: Obtain records from the **Dr. Cem Berkay Sinaci Clinic** (Istanbul) regarding Nicola McNamee's cancelled surgery, including the identity of the anesthetist who refused to conduct a pre-operative consultation, the reason given for the refusal, and any communications the clinic received from third parties regarding this patient.

Why this matters: The Sinaci clinic was Nicola's original surgical destination. The refusal of the anesthetist to meet a patient with dual channelopathy diagnoses is clinically extraordinary and consistent with a clinical environment in which the patient was not expected to survive the procedure.

Action 6, Subpoena Orion Mine Finance

Agency: DOJ Criminal Division (via federal grand jury) / HRSP coordination

Action: Issue federal grand jury subpoenas to Orion Mine Finance (Orion Resource Partners), headquartered in New York City, for all communications, due diligence files, community relations strategies, and investor disclosures relating to:

- Community opposition to the Curraghinalt gold mine project
- Land acquisition and access negotiations in the Greencastle/Gortin/Rouskey area of County Tyrone
- Political lobbying and diplomatic engagement regarding mine approval, including all communications with the U.S. Embassy London, the UK Department for Business, and the Northern Ireland Department for Infrastructure
- Representations made to limited partner investors (U.S. pension funds) regarding community opposition, planning risk, and human rights due diligence

Evidentiary basis: Orion acquired a controlling stake in Dalradian Resources for US$407 million (September 2018). The mine is the financial motive at the center of this referral. Orion is a Delaware-registered entity headquartered in New York. Federal subpoena authority is direct, no international cooperation required. (Document 09, Nexus 2)

Action 7, Issue Preservation Letters

Agency: DOJ Criminal Division / HRSP

Action: Issue preservation letters to the following institutions directing them to preserve all records relating to (a) Long QT Syndrome and Brugada Syndrome diagnoses of County Tyrone patients, and (b) all records relevant to Nicola McNamee:

Institution	Records to Preserve
Belfast Health and Social Care Trust	All Inherited Cardiac Conditions Service records; all cascade screening records triggered by County Tyrone index patients; all referral records to/from the Belfast Cardiac Unit for County Tyrone postcodes (BT79, BT78, BT81, BT82); complete diagnostic and treatment records for Nicola McNamee
KIMS Hospital / LycaHealth	Complete medical records for Nicola McNamee including original clinical system exports (DXperience), metadata-intact PDFs, all ECG tracings in native format, operative reports, anaesthesia records, and all audit logs documenting record access, modification, or export
St George's Hospital, London	All records relating to Nicola McNamee including referral correspondence from Belfast Inherited Cardiac Conditions Service, cardiology assessments, and any genetic testing orders or results

Evidentiary basis: The forensic analysis of Nicola McNamee's KIMS records (Document 05) identified systematic manipulation, metadata stripping, text layer removal, ECG data flattened to raster images, designed to defeat automated review. Original diagnostic records from Belfast and St George's are absent from the file.

Preservation letters are necessary to prevent further destruction or manipulation of records that constitute primary evidence. (Documents 04, 04B, 06)

Urgency: The complainants' departure from the UK and the filing of this submission create a foreseeable risk that records will be altered or destroyed. Preservation letters should issue immediately upon intake, regardless of whether a full investigation is opened.

Action 8, Analyze U.S. Pension Fund Exposure and Coordinate with SEC

Agency: DOJ Criminal Division / SEC coordination

Action: Request LP agreements, side letters, and due diligence files from the following confirmed Orion Mine Finance fund investors:

Pension Fund	State	Fund
State Board of Administration of Florida	FL	Fund II
Washington State Investment Board	WA	Fund II
Texas Municipal Retirement System	TX	Fund II
Maine Public Employees Retirement System	ME	Fund II
Arizona Public Safety Personnel Retirement System	AZ	Fund II
LACERA (Los Angeles County Employees Retirement)	CA	Fund IV ($250M committed)
Oregon Public Employees Retirement Fund	OR	LP confirmed
North Carolina Retirement Systems	NC	LP confirmed
State of Michigan Retirement System	MI	LP confirmed
University of Michigan Endowment	MI	Fund II

Coordinate with the SEC to assess whether Orion's representations to LP investors regarding community opposition, planning risk, and the mine's development pathway were materially accurate. If the mine's development depends on the systematic displacement of the opposing community through a covert medical campaign, the investment thesis presented to these pension funds may have been materially misleading.

Evidentiary basis: American public pension funds, managing retirement savings for teachers, firefighters, police officers, and state employees, are limited partners in Orion Mine Finance. The pension funds have regulatory obligations to respond to federal inquiries. (Document 09, Nexus 3)

Political significance: This nexus creates Congressional engagement from every state with an exposed pension fund: Florida, Washington, Texas, Maine, Arizona, California, Oregon, North Carolina, and Michigan.

Action 9, FBI Counterintelligence Assessment

Agency: FBI Counterintelligence Division

Action: Conduct a counterintelligence assessment of MI5 operations in Northern Ireland as they relate to:

- The complainant witnesses (Brian Spector, U.S. citizen; Nicola McNamee)
- The Curraghinalt mining project and its connection to U.S. diplomatic engagement
- The deployment of APT29 malware against the complainants through compromised legal teams (documented in the parallel EEA referral, filed 30 January 2026)
- The IRGC surveillance incident on U.S. soil (Washington, D.C., October 2025) and the documented operational link between MoD operatives and IRGC-connected networks

This action should be coordinated with the existing EEA referral, which is before the FBI Counterintelligence Division and documents overlapping perpetrators and the same institutional apparatus.

Evidentiary basis: The EEA referral establishes: Ali Reza Sinai (MoD operative) simultaneously operated fabricated personas on both sides of Spector's litigation while connected to IRGC-linked networks; four successive legal teams were compromised; APT29 malware was deployed three times through the attorney-client relationship; MoD operatives embedded in Spector's legal teams used document tampering techniques identical to those found in Nicola's medical records. (Document 06; EEA referral, 30 January 2026)

Action 10, Geographic Correlation Analysis

Agency: FBI (analytical support) / HRSP coordination

Action: Map all confirmed Long QT Syndrome and Brugada Syndrome diagnoses originating from the Belfast Inherited Cardiac Conditions Service and Belfast Cardiac Unit against:

- The Dalradian mineral licence area and the proposed Curraghinalt mine footprint
- Addresses of known anti-mine campaigners and Save Our Sperrins members
- Parish and townland boundaries within the affected area (Greencastle, Gortin, Rouskey, Plumbridge)

This analysis requires data from Belfast Health and Social Care Trust (obtainable via preservation letter, Action 4) and from the Dalradian/Orion planning application file (public record, Northern Ireland Department for Infrastructure).

Evidentiary basis: The witness's assertion that cardiac diagnoses are geographically concentrated in County Tyrone, specifically in the area overlying the gold deposit, is the central factual claim of this submission. A geographic correlation analysis using diagnosed patient postcodes against the mineral licence area would either confirm or refute this claim. If confirmed, the correlation constitutes powerful evidence of targeted conduct. (Documents 03, 04)

Action 11, Electoral and Census Analysis

Agency: FBI (analytical support) / HRSP coordination

Action: Compare UK census data (2011 vs. 2021) and electoral register data for the Greencastle, Gortin, and Rouskey area to quantify:

- Any decline in Catholic families since mineral exploration intensified (~2004)
- Population change in the mine footprint area relative to comparable control areas in County Tyrone
- Correlation between population decline and the cardiac screening programme's geographic reach

Evidentiary basis: If the suspected medical targeting is functioning as covert depopulation, reducing the community's capacity to resist through false diagnoses, psychological harm, and excess mortality concealed by a 0.46% autopsy rate, the effect should be visible in census and electoral data. The 2015 boundary restructuring (splitting County Tyrone across three district councils) was identified in Document 05 as a concealment mechanism creating statistical discontinuity. This analysis should reconstruct the pre-restructuring County Tyrone boundary to restore statistical continuity. (Documents 03, 05)

Action 12, Financial Forensics

Agency: FBI Financial Crimes / HRSP coordination

Action: Trace:

- The Dalradian "Tyrone Fund", a community investment fund of £800,000+ in donations to local organizations, for evidence of targeted influence or payments to individuals connected to NHS Northern Ireland, the PSNI, the NI Civil Service, or the UK MoD
- All payments from Dalradian Resources / Orion Mine Finance to political consultants, lobbyists, security firms, and any intelligence-connected entities operating in Northern Ireland
- Crown Estate royalty projections and any communications between the Crown Estate and Dalradian/Orion regarding mine development timelines or community opposition

Evidentiary basis: The Crown Estate owns all subsurface mineral rights in Northern Ireland and collects a 4% royalty on gold sold, potentially £308 million to £800 million from the Curraghinalt deposit alone. This creates direct financial alignment between the British Crown and the mine's success. Financial forensics can establish whether community investment was targeted to neutralize opposition or whether payments flowed to individuals with institutional access to the NHS screening programme. (Document 03)

Action 13, Investigate Death Threats and Physical Attacks Against Anti-Mine Campaigners

Agency: FBI / HRSP coordination (via MLAT if necessary)

Action: Investigate the origin of death threats reported by three Save Our Sperrins campaigners to the PSNI in June 2019. Determine:

- Whether the threats originated from paramilitary organizations
- Whether any paramilitary organization involved has documented connections to UK security forces (historically established through the Stevens Inquiry, the de Silva Review, and the Smithwick Tribunal)
- Whether the PSNI investigation of these threats was conducted genuinely or was suppressed

Evidentiary basis: Death threats against anti-mine campaigners, if connected to state security forces or their proxies, constitute direct evidence of the coercive apparatus operating alongside the medical targeting. The historical use of loyalist paramilitaries as proxies for British intelligence in Northern Ireland is extensively documented in public inquiries. (Document 03)

PART II: PROTECTIVE MEASURES

Measure 1, FBI Threat Assessment

Agency: FBI (under Attorney General's Domestic Investigations and Operations Guide)

Action: Conduct an immediate threat assessment incorporating:

- The IRGC/SARDAR surveillance incident (Washington, D.C., October 2025), a designated Foreign Terrorist Organization operating against a U.S. citizen on American soil
- The documented 25-month escalation pattern: digital surveillance (November 2023) → social engineering attack (March 2024) → legal sabotage (May 2024–August 2025) → kinetic threats including floor plans of complainants' home (March–June 2025) → IRGC surveillance on U.S. soil (October 2025) → false arrest and denied consular notification (December 2025)
- The post-flight monitoring: unsolicited email from Mishcon de Reya seeking bank details for "refund" days after FBI IC3 filings (January 2026)

Standard: The AG's DIOG permits threat assessment without opening a full investigation. The IRGC surveillance alone, involving a designated FTO, meets the threshold for immediate assessment.

Urgency: Neither complainant has been afforded any protection to date despite the documented escalation.

Nicola McNamee's family in Northern Ireland. The threat assessment should also consider the ongoing harm to Nicola's family members who remain in County Tyrone:

- Her **mother** carries an unnecessary pacemaker, a surgically implanted device based on a fabricated diagnosis. She returns to the same NHS institutional system for regular device checks. Every follow-up appointment places her in the hands of the apparatus that fabricated the diagnosis. A pacemaker implanted on false pretenses constitutes ongoing torture under 18 U.S.C. § 2340.
- Her **brother and sister** were administered cardiac medication as toddlers based on cascade screening from the fabricated index diagnosis. Whether they are still receiving unnecessary medication requires investigation. They have never received independent genetic testing.

- The family's December 2025 behavior (cousin denying via Signal that any family member had ever been diagnosed, contradicting documented medical history) is consistent with coercion.

These individuals cannot safely seek independent medical assessment within the UK system without exposing themselves to the institutional apparatus documented in this submission. The Department should assess whether they can be safely contacted as witnesses, whether their medical records should be independently reviewed, and whether any diplomatic or consular channel exists to facilitate their protection.

Measure 2, FBI Counterintelligence Defensive Briefing

Agency: FBI Counterintelligence Division

Action: Provide both complainants (Brian Spector and Nicola McNamee) with a formal counterintelligence defensive briefing addressing:

- The scope of UK/Five Eyes access to their communications, devices, and location data
- Protective measures for their digital infrastructure, including assessment of compromised devices (the complainant's iPhone is available for MVT scanning, see Document 08)
- The implications of the IRGC surveillance incident, including how the operative knew the timing and location of the complainant's FBI meeting at Manassas
- Recommended operational security protocols for continued engagement with federal authorities

Context: Brian Spector has already met with FBI SA Smith at the Manassas field office (October 2025). A defensive briefing is a standard CI protective measure for U.S. persons targeted by foreign intelligence services.

Measure 3, Crime Victims' Rights Act Protections

Agency: DOJ Criminal Division / U.S. Attorney's Office

Action: Formally recognize both complainants as crime victims under 18 U.S.C. § 3771 (Crime Victims' Rights Act), which guarantees:

Right	Statutory Basis
Reasonable protection from the accused	§ 3771(a)(1)
Reasonable, accurate, and timely notice of any public court proceeding	§ 3771(a)(2)
Not to be excluded from any such public court proceeding	§ 3771(a)(3)
To be reasonably heard at any public proceeding involving release, plea, sentencing, or parole	§ 3771(a)(4)
To be treated with fairness and respect for dignity and privacy	§ 3771(a)(8)

Basis: The parallel EEA referral is already filed with the FBI Counterintelligence Division (30 January 2026). If any federal investigation is opened, EEA (18 U.S.C. § 1831), HRSP referral, or otherwise, both complainants are crime victims under federal law and their § 3771 rights attach immediately.

Measure 4, State Department Consular Protection for U.S. Citizen Minor

Agency: U.S. Department of State, Bureau of Consular Affairs / Embassy London

Action: Notify the State Department consular section at Embassy London of the situation of Brian Spector's 16-year-old daughter, a U.S. citizen who remains in the United Kingdom within the jurisdiction of the apparatus that:

- Arrested her father on false charges (December 2025)
- Held her father in solitary confinement for 24 hours
- Denied her father U.S. Embassy notification in violation of the Vienna Convention on Consular Relations
- Has been documented by Kent Police officers as attempting to imprison her father with planted evidence
- Deployed IRGC surveillance against her father on U.S. soil

A U.S. citizen minor in the custody area of an allied intelligence service that has demonstrably targeted her father is an immediate consular protection concern. The State Department has independent authority and obligation to protect U.S. citizens abroad, and the documented Vienna Convention violation with respect to her father establishes that UK authorities have already breached consular obligations.

Measure 5, Immigration Status for Complainant Witness

Agency: FBI (S Visa sponsorship) / DOJ / USCIS

Action: Assess Nicola McNamee for S-1 visa eligibility as a witness possessing critical reliable information concerning a criminal organization or enterprise. The S visa provides:

- Lawful immigration status
- Work authorization
- Protection from removal
- Pathway to lawful permanent residence after three years

S visas are limited to 200 per fiscal year and require Attorney General or Deputy Attorney General certification. The investigating agency (FBI) must sponsor the application. This is an alternative or supplement to the spouse visa currently being pursued through Rep. Cloud's office (TX-27) and carries protective implications beyond standard immigration benefits.

Basis: Nicola McNamee is the primary complainant witness. She is currently in the United States. She cannot safely return to Northern Ireland, the jurisdiction where the suspected targeting is occurring and where her family remains subject to the cascade screening mechanism. Her testimony and continued cooperation are essential to any investigation. (Document 08)

PART III: INTERAGENCY COORDINATION SUMMARY

Action	Lead Agency	Coordinating Agencies	Priority
1. Subpoena Lycamobile USA Inc.	**DOJ Criminal Division**	**FCC**	**IMMEDIATE: primary prosecution pathway**
2. Subpoena Magnetar Capital	**DOJ Criminal Division**		**IMMEDIATE**
3. Obtain Dr. Karacaoglu testimony	**DOJ / FBI**	**Turkish legal channels**	**IMMEDIATE**
4. Open HRSP investigation	HRSP	FBI, NSD	Immediate
5. Obtain Sinaci clinic records	DOJ / FBI	Turkish legal channels	High
6. Subpoena Orion Mine Finance	DOJ Criminal Division	SEC	High
7. Issue preservation letters	HRSP		**Immediate regardless of investigation status**
8. Pension fund exposure / SEC coordination	DOJ Criminal Division	SEC, state pension regulators	High
9. FBI counterintelligence assessment	FBI CI Division	HRSP, NSD	High
10. Geographic correlation analysis	FBI	HRSP	Medium (dependent on Action 7 data)
11. Electoral and census analysis	FBI	HRSP	Medium
12. Financial forensics	FBI Financial Crimes	HRSP, FinCEN	Medium
13. Investigate death threats / physical attacks	FBI	HRSP	Medium
17. Compel disclosure of active CLOUD Act orders	**DOJ OIA / NSD**	**FBI CI, HRSP**	**IMMEDIATE**
18. Review CLOUD Act dual nationality compliance	**DOJ OIA**	**NSD, HRSP**	**IMMEDIATE**
P1. FBI threat assessment	**FBI**	**HRSP, NSD**	**Immediate**
P2. CI defensive briefing	**FBI CI Division**		**Immediate**
P3. Crime victims' rights	**DOJ / USAO**	**FBI**	**Upon investigation opening**
P4. Consular protection for minor	**State Department**	**Embassy London**	**Immediate**
P5. S Visa assessment	**FBI**	**DOJ, USCIS**	**High**

Action 14: Subpoena Amazon Web Services for Email Infrastructure Records

Custodian: AWS Trust & Safety / Law Enforcement Response Team, Amazon.com, Inc., 410 Terry Avenue North, Seattle, WA 98109

Records requested:

1. Complete Route 53 change history for Hosted Zone Z06444333JDD73IYBOJ7Y (`drey.fi`) from creation to present, including all ChangeResourceRecordSets API calls with: requesting IAM principal, source IP address, timestamp, and specific DNS record changes

2. All records of government requests, legal process, national security directives, or Technical Capability Notices relating to AWS Account 992070355238, the hosted zone, or the domain `drey.fi`
3. Records sufficient to determine whether DKIM or DMARC records were ever created in this hosted zone and subsequently deleted

Evidentiary basis: The complainant's email domain lacks DKIM/DMARC authentication, rendering planted medical evidence forensically indistinguishable from genuine correspondence. The UK Investigatory Powers Act 2016 provides statutory authority for government-directed removal of email authentication (Document 06).

Action 15: Subpoena Microsoft Corporation for Exchange Online Audit Logs

Custodian: Microsoft Law Enforcement Response Team (LERT), Microsoft Corporation, One Microsoft Way, Redmond, WA 98052

Records requested:

1. Complete Exchange Online admin audit logs for tenant `1d280f39-236d-4896-bf98-cd0419803c06` (`drey.fi`) from 1 January 2023 to present, including: mailbox access grants, delegate permissions, application registrations, EWS and Graph API access logs, and DKIM configuration changes
2. Decryption of `x-ms-exchange-crosstenant-userprincipalname` header values from the identified messages
3. Records of all application registrations granted access to Nicola McNamee's mailbox within this tenant
4. Records sufficient to determine whether DKIM signing was ever enabled and subsequently disabled
5. All records of government requests or Technical Capability Notices relating to this tenant or the `drey.fi` domain
6. Complete sent-mail logs, outbound message traces, and Graph API activity for all mailboxes in tenant `1d280f39-236d-4896-bf98-cd0419803c06` from 1 October 2023 to present, including source IP addresses and authentication principals for each outbound message, sufficient to identify any messages sent from the complainants' accounts by an authenticated principal other than the account holder
7. All content created, modified, or deposited in any account on the Microsoft identity graph associated with this tenant or with the account `bps1968@hotmail.com` by any principal other than the account holder, including application-level access and any access authorized by government legal process

Evidentiary basis: Three emails containing fabricated cardiac documents were deposited in the complainant's O365 mailbox with empty subjects and empty bodies, a pattern inconsistent with human email behavior and consistent with server-side injection (Document 06). The complainants have also encountered multiple instances where third parties produced emails firmly attributed to them that neither complainant authored, consistent with UK operatives using TC Note write access to send disinformation from the complainants' genuine accounts. Items 6 and 7 are necessary to identify the full scope of account impersonation and content planting.

Action 16: MLAT Request to the United Kingdom (Conditional)

Should the records obtained from AWS and Microsoft indicate government-directed modification, the Department should initiate a request under the US-UK Mutual Legal Assistance Treaty for:

1. Records of any notices served under the Investigatory Powers Act 2016 relating to `drey.fi` or any associated email accounts
2. Records held by the Investigatory Powers Commissioner's Office (IPCO) relating to oversight of any such notices
3. Records of any Investigatory Powers Tribunal (IPT) proceedings relating to surveillance of any individual connected to this matter

Action 17: Compel Disclosure of Active CLOUD Act Orders

Agency: DOJ Office of International Affairs / DOJ National Security Division

Action: Direct Microsoft Corporation, Google LLC, Apple Inc., and Amazon Web Services Inc. to confirm whether any active orders under the US-UK CLOUD Act bilateral data access agreement currently target either complainant (Brian Spector, Nicola McNamee) or any account associated with the `drey.fi` domain, `brian@2real.stream`, `bps1968@hotmail.com`, or any other identifier connected to the complainants.

If orders exist, compel disclosure of:

- The requesting authority (which UK agency)
- Date of issuance and duration
- Scope of the order (stored data, real-time interception, location data, metadata)
- Whether the target was classified as US person or non-US person in the warrant application

Why this is urgent: Brian Spector is a dual US/UK citizen. The CLOUD Act bilateral agreement requires US judicial oversight for surveillance of US persons. If the UK classified him as non-US to circumvent this protection, every surveillance action against him on American soil violates the agreement. The UK has the capability and the motive to monitor the complainants' communications with US federal law enforcement, congressional offices, and this Department in real time. Nicola McNamee, as a non-US person, has no CLOUD Act protections and is fully accessible to UK surveillance through American providers without any US oversight.

Evidentiary basis: Document 06 (TC Note three-part weapon; CLOUD Act architecture); spearphishing timed to records recovery (Document 06, forensics addendum); IRGC surveillance timed to FBI meeting (Document 06).

Action 18: Review CLOUD Act Compliance for Dual National Targeting

Agency: DOJ Office of International Affairs (administers CLOUD Act agreements)

Action: Review all UK CLOUD Act orders targeting Brian Spector or any identifier associated with him to determine:

- Whether the UK disclosed his US citizenship in warrant applications
- Whether the UK's nationality classification of the target was accurate
- Whether the US person protections in the bilateral agreement were triggered and complied with
- Whether any UK CLOUD Act orders were served during the period the complainants were communicating with US federal law enforcement (October 2025 to present)

If the UK classified a dual US/UK national as non-US to circumvent the agreement's US person protections, this constitutes a material breach of the bilateral agreement. The Department should assess whether suspension or modification of the agreement is warranted pending investigation.

Evidentiary basis: Brian Spector holds US and UK citizenship (documented). The CLOUD Act bilateral agreement's US person protections depend entirely on the requesting country's accurate nationality classification. No US government body independently verifies these classifications. The UK had direct incentive to misclassify: accurate classification would have triggered US judicial oversight and exposed the surveillance to American authorities. (Document 03, Nexus 2A; Document 06, CLOUD Act section)

CONCLUSION

Eighteen investigative actions. Five protective measures. The first three actions constitute the primary prosecution pathway for the attempted murder of Nicola McNamee: subpoena Lycamobile USA (Newark, NJ) for the KIMS Hospital evidence chain, subpoena Magnetar Capital (Evanston, IL) for pre-acquisition records, and obtain Dr. Karacaoglu's testimony from Istanbul. Actions 17 and 18 address the ongoing CLOUD Act surveillance of the complainants on American soil and the suspected concealment of a US citizen's nationality in UK warrant applications. None of these actions touches UK intelligence channels. All target entities or persons subject to direct US federal jurisdiction or voluntary cooperation.

The prosecution pathway is designed to produce a criminal case before the UK government can interfere with it. Once the Lycamobile USA subpoena produces the KIMS records, once Magnetar produces the pre-acquisition due diligence, and once Dr. Karacaoglu testifies to what he saw and what he did about it, the attempted murder case is built. The genocide case (population-level) builds on the murder case (individual-level) through the convergence analysis, the statistical anomalies, and the geographic correlation.

The complainants have provided the evidence. They are asking the Department of Justice to act on it before the next victim is not as fortunate as Nicola McNamee.

Document 10 of 10. Prepared for HRSP submission package, 1 March 2026.

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